



# Evidence of Coverage 2024

**UHC Dual Complete NE-V001 (HMO-POS D-SNP)**



**Toll-free 1-844-368-7149, TTY 711**  
8 a.m.-8 p.m.: 7 Days Oct-Mar; M-F Apr-Sept



**[myuhc.com/communityplan](https://myuhc.com/communityplan)**

**United  
Healthcare®**

January 1 – December 31, 2024

# Evidence of Coverage

## Your Medicare Health Benefits and Services and Prescription Drug Coverage as a Member of our plan

This document gives you the details about your Medicare health care and prescription drug coverage from January 1 – December 31, 2024.



**This is an important legal document. Please keep it in a safe place.**

**For questions about this document, please contact Customer Service at 1-844-368-7149. (TTY users should call 711). Hours are 8 a.m.-8 p.m.: 7 Days Oct-Mar; M-F Apr-Sept.**

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This plan, UHC Dual Complete NE-V001 (HMO-POS D-SNP), is insured through UnitedHealthcare Insurance Company or one of its affiliates. (When this **Evidence of Coverage** says “we,” “us,” or “our,” it means UnitedHealthcare. When it says “plan” or “our plan,” it means UHC Dual Complete NE-V001 (HMO-POS D-SNP).)

UnitedHealthcare does not discriminate on the basis of race, color, national origin, sex, age, or disability in health programs and activities.

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Benefits, premium, deductible, and/or copayments/coinsurance may change on January 1, 2025. The formulary, pharmacy network, and provider network may change at any time. You will receive notice when necessary. We will notify affected enrollees about changes at least 30 days in advance.

This document explains your benefits and rights. Use this document to understand about:

- ☐ Your plan premium and cost-sharing;
- ☐ Your medical and prescription drug benefits;
- ☐ How to file a complaint if you are not satisfied with a service or treatment;
- ☐ How to contact us if you need further assistance; and,
- ☐ Other protections required by Medicare law.

OMB Approval 0938-1051 (Expires: August 31, 2026)

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**Questions?** Call Customer Service at **1-844-368-7149**, TTY **711**, 8 a.m.-8 p.m.: 7 Days Oct-Mar; M-F Apr-Sept

# **Chapter 1**

Getting started as a member

## Section 1 Introduction

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### Section 1.1 You are enrolled in UHC Dual Complete NE-V001 (HMO-POS D-SNP), which is a specialized Medicare Advantage Plan (Special Needs Plan)

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You are covered by both Medicare and Medicaid:

- ☐ **Medicare** is the Federal health insurance program for people 65 years of age or older, some people under age 65 with certain disabilities, and people with end-stage renal disease (kidney failure).
- ☐ **Medicaid** is a joint Federal and state government program that helps with medical costs for certain people with limited incomes and resources. Medicaid coverage varies depending on the state and the type of Medicaid you have. Some people with Medicaid get help paying for their Medicare premiums and other costs. Other people also get coverage for additional services and drugs that are not covered by Medicare.

You have chosen to get your Medicare health care and your prescription drug coverage through our plan, UHC Dual Complete NE-V001 (HMO-POS D-SNP). We are required to cover all Part A and Part B services. However, cost-sharing and provider access in this plan differ from Original Medicare.

UHC Dual Complete NE-V001 (HMO-POS D-SNP) is a specialized Medicare Advantage Plan (a Medicare “Special Needs Plan”), which means its benefits are designed for people with special health care needs. UHC Dual Complete NE-V001 (HMO-POS D-SNP) is designed for people who have Medicare and who are also entitled to assistance from Medicaid.

Because you get assistance from Medicaid with your Medicare Part A and B cost sharing (deductibles, copayments, and coinsurance) you may pay nothing for your Medicare health care services. Medicaid may also provide other benefits to you by covering health care services and prescription drugs that are not usually covered under Medicare. You will also receive “Extra Help” from Medicare to pay for the costs of your Medicare prescription drugs. UHC Dual Complete NE-V001 (HMO-POS D-SNP) will help manage all of these benefits for you, so that you get the health care services and payment assistance that you are entitled to.

Our plan is a Medicare Advantage HMO Plan (HMO stands for Health Maintenance Organization) with a Point-of-Service (POS) option approved by Medicare and run by a private company. “Point-of-Service” means you can use providers outside the plan’s network for an additional cost. (See Chapter 3, Section 2.3 for information about using the Point-of-Service option.) Like all Medicare Advantage plans, this Medicare Special Needs Plan is approved by Medicare. The plan also has a contract with the Nebraska Medicaid program to coordinate your Medicaid benefits. We are pleased to be providing your Medicare health care coverage, including your prescription drug coverage.

**Coverage under this Plan qualifies as Qualifying Health Coverage (QHC)** and satisfies the Patient Protection and Affordable Care Act’s (ACA) individual shared responsibility requirement.



Please visit the Internal Revenue Service (IRS) website at: [irs.gov/Affordable-Care-Act/individuals-and-families](https://irs.gov/Affordable-Care-Act/individuals-and-families) for more information.

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## Section 1.2 What is the Evidence of Coverage document about?

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This **Evidence of Coverage** document tells you how to get your Medicare and Medicaid medical care and prescription drugs. It explains your rights and responsibilities, what is covered, what you pay as a member of the plan, and how to file a complaint if you are not satisfied with a decision or treatment.

The words “coverage” and “covered services” refer to the medical care, services and prescription drugs available to you as a member of the plan.

It’s important for you to learn what the plan’s rules are and what services are available to you. We encourage you to set aside some time to look through this **Evidence of Coverage** document.

If you are confused, concerned or just have a question, please contact Customer Service.

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## Section 1.3 Legal information about the Evidence of Coverage

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This **Evidence of Coverage** is part of our contract with you about how the plan covers your care. Other parts of this contract include your enrollment form, the **List of Covered Drugs (Formulary)**, and any notices you receive from us about changes to your coverage or conditions that affect your coverage. These notices are sometimes called “riders” or “amendments.”

The contract is in effect for months in which you are enrolled in the plan between January 1, 2024 and December 31, 2024.

Each calendar year, Medicare allows us to make changes to the plans that we offer. This means we can change the costs and benefits of the plan after December 31, 2024. We can also choose to stop offering the plan, or to offer it in a different service area, after December 31, 2024.

Medicare (the Centers for Medicare & Medicaid Services) must approve our plan each year. You can continue each year to get Medicare coverage as a member of our plan as long as we choose to continue to offer the plan and Medicare renews its approval of the plan.

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## Section 2 What makes you eligible to be a plan member?

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### Section 2.1 Your eligibility requirements

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**You are eligible for membership in our plan as long as:**

- ☐ You have both Medicare Part A and Medicare Part B
- ☐ — **and** — you live in our geographic service area (Section 2.3 below describes our service area). Incarcerated individuals are not considered living in the geographic service area even if they are physically located in it.
- ☐ — **and** — you are a United States citizen or are lawfully present in the United States

☐— **and** — you meet the special eligibility requirements described below.

### Special eligibility requirements for our plan

Our plan is designed to meet the needs of people who receive certain Medicaid benefits. (Medicaid is a joint Federal and state government program that helps with medical costs for certain people with limited incomes and resources.) To be eligible for our plan you must be eligible for both Medicare and Medicaid.

Please note: If you lose your Medicaid eligibility but can reasonably be expected to regain eligibility within 6 month(s), then you are still eligible for membership in our plan (Chapter 4, Section 2.1 tells you about coverage and cost sharing during a period of deemed continued eligibility).

## Section 2.2      What is Medicaid?

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Medicaid is a joint Federal and state government program that helps with medical costs for certain people who have limited incomes and resources. Each state decides what counts as income and resources, who is eligible, what services are covered, and the cost for services. States also can decide how to run their program as long as they follow the Federal guidelines.

In addition, there are programs offered through Medicaid that help people with Medicare pay their Medicare costs, such as their Medicare premiums. These “Medicare Savings Programs” help people with limited income and resources save money each year.

You can enroll in this plan if you are in one of these Medicaid categories:

- **Qualified Medicare Beneficiary Plus (QMB+):** You get Medicaid coverage of Medicare cost-share and are also eligible for full Medicaid benefits. Medicaid pays your Medicare Part A and Part B premiums, deductibles, coinsurance and copayment amounts for Medicare covered services. You pay nothing, except for Part D prescription drug copays (if applicable).
- **Qualified Medicare Beneficiary (QMB):** You get Medicaid coverage of Medicare cost-share but are not eligible for full Medicaid benefits. Medicaid pays your Medicare Part A and Part B premiums, deductibles, coinsurance and copayment amounts only for Medicare covered services. You pay nothing, except for Part D prescription drug copays (if applicable).
- **Qualified Disabled and Working Individual (QDWI):** Medicaid pays your Part A premium only. The State Medicaid Office does not pay your cost-share. You do not have full Medicaid benefits. There may be some services that do not have a member cost share amount.
- **Qualifying Individual (QI):** Medicaid pays your part B premium only. The State Medicaid Office does not pay your cost-share. You do not have full Medicaid benefits. You pay the cost share amounts listed in the chart below. There may be some services that do not have a member cost share amount. The State Medicaid Office does not pay your cost-share. You do not have full Medicaid benefits. There may be some services that do not have a member cost share amount.
- **Specified Low-Income Medicare Beneficiary (SLMB+):** Medicaid pays your Part B premium and provides full Medicaid benefits. You are eligible for full Medicaid benefits. At times you may

also be eligible for limited assistance from your state Medicaid agency in paying your Medicare cost share amounts. Generally your cost share is 0% when the service is covered by both Medicare and Medicaid. There may be cases where you have to pay cost sharing when a service or benefit is not covered by Medicaid.

- **Specified Low-Income Medicare Beneficiary (SLMB):** Medicaid pays your Part B premium only. The State Medicaid Office does not pay your cost-share. You do not have full Medicaid benefits. There may be some services that do not have a member cost share amount.
- **Full Benefits Dual Eligible (FBDE):** Medicaid may provide limited assistance with Medicare cost-sharing. Medicaid also provides full Medicaid benefits. You are eligible for full Medicaid benefits. At times you may also be eligible for limited assistance from the State Medicaid Office in paying your Medicare cost share amounts. Generally your cost share is 0% when the service is covered by both Medicare and Medicaid. There may be cases where you have to pay cost sharing when a service or benefit is not covered by Medicaid.

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## **Section 2.3                    Here is the plan service area for UHC Dual Complete NE-V001 (HMO-POS D-SNP)**

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Our plan is available only to individuals who live in our plan service area. To remain a member of our plan, you must continue to reside in the plan service area. The service area is described below.

Our service area includes these counties in Nebraska: Adams, Antelope, Boone, Buffalo, Burt, Butler, Cass, Cedar, Clay, Colfax, Cuming, Custer, Dakota, Dixon, Dodge, Douglas, Fillmore, Gage, Hall, Hamilton, Howard, Jefferson, Johnson, Kearney, Knox, Lancaster, Madison, Merrick, Nuckolls, Otoe, Pawnee, Pierce, Platte, Polk, Saline, Sarpy, Saunders, Seward, Sherman, Stanton, Thayer, Thurston, Washington, Wayne, Webster, York.

If you plan to move out of the service area, you cannot remain a member of this plan. Please contact Customer Service to see if we have a plan in your new area.

When you move, you will have a Special Enrollment Period that will allow you to switch to Original Medicare or enroll in a Medicare health or drug plan that is available in your new location.

It is also important that you call Social Security if you move or change your mailing address. You can find phone numbers and contact information for Social Security in Chapter 2, Section 5.

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## **Section 2.4                    U.S. Citizen or Lawful Presence**

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A member of a Medicare health plan must be a U.S. citizen or lawfully present in the United States. Medicare (the Centers for Medicare & Medicaid Services) will notify UHC Dual Complete NE-V001 (HMO-POS D-SNP) if you are not eligible to remain a member on this basis. UHC Dual Complete NE-V001 (HMO-POS D-SNP) must disenroll you if you do not meet this requirement.

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## **Section 3                    Important membership materials you will receive**

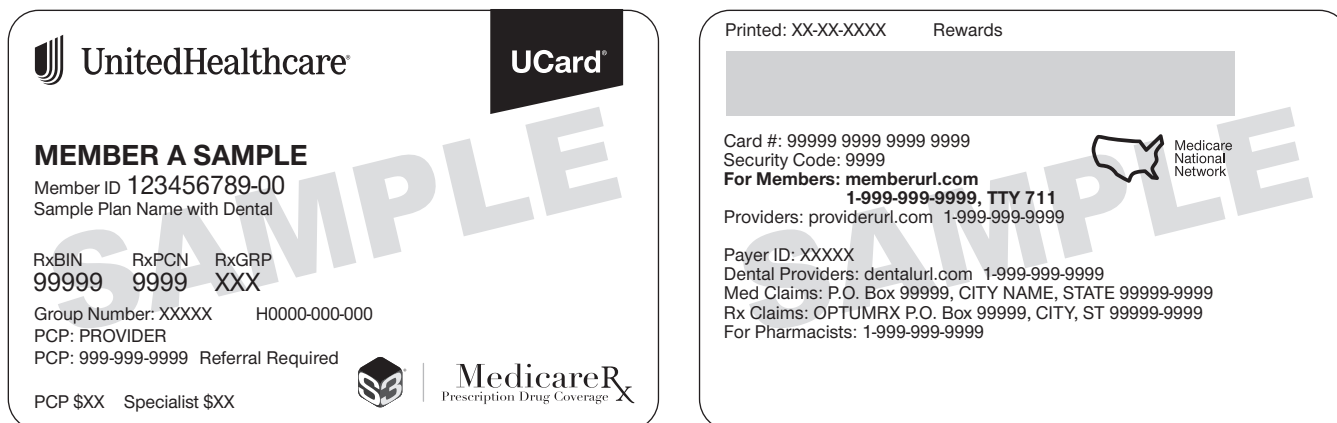
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### **Section 3.1                    Your UnitedHealthcare member ID card**

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While you are a member of our plan, you must use your UnitedHealthcare member ID card whenever you get services covered by this plan and for prescription drugs you get at network pharmacies. **IMPORTANT** – If you have Medicare and Nebraska Department of Health and Human Services (Medicaid), make sure to show your UnitedHealthcare member ID card and your state Medicaid ID card whenever you access services. This will help your provider bill correctly. Here's a sample UnitedHealthcare member ID card to show you what yours will look like:



Do NOT use your red, white, and blue Medicare card for covered medical services while you are a member of this plan. If you use your Medicare card instead of your UnitedHealthcare member ID card, you may have to pay the full cost of medical services yourself. Keep your Medicare card in a safe place. You may be asked to show it if you need hospital services, hospice services, or participate in Medicare approved clinical research studies also called clinical trials.

If your UnitedHealthcare member ID card is damaged, lost, or stolen, call Customer Service right away and we will send you a new card.

## Section 3.2 Provider Directory

The **Provider Directory** lists our current network providers and durable medical equipment suppliers. **Network providers** are the doctors and other health care professionals, medical groups, durable medical equipment suppliers, hospitals, and other health care facilities that have an agreement with us to accept our payment and any plan cost-sharing as payment in full.

You must use network providers to get your medical care and services, except for covered routine dental services. If you go elsewhere without proper authorization you will have to pay in full. The only exceptions are emergencies, urgently needed services when the network is not available (that is, in situations when it is unreasonable or not possible to obtain services in-network), out-of-area dialysis services, and cases in which our plan authorizes use of out-of-network providers.

Members of this plan may use their Point of Service (POS) benefits to see non-network providers for covered routine dental services only. Please refer to Chapter 3 (Using the plan's coverage for your medical services) for more specific information about POS.

The most recent list of providers and suppliers is available on our website at [myuhc.com/communityplan](https://myuhc.com/communityplan).

If you don't have your copy of the **Provider Directory**, you can request a copy (electronically or in hardcopy form) from Customer Service. Requests for hard copy Provider Directories will be mailed to you within three business days.

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### **Section 3.3      Pharmacy Directory**

The pharmacy directory lists our network pharmacies. **Network pharmacies** are all of the pharmacies that have agreed to fill covered prescriptions for our plan members. You can use the Pharmacy Directory to find the network pharmacy you want to use. See Chapter 5, Section 2.5 for information on when you can use pharmacies that are not in the plan's network.

If you don't have the **Pharmacy Directory**, you can get a copy from Customer Service. You can also find this information on our website at [myuhc.com/communityplan](https://myuhc.com/communityplan).

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### **Section 3.4      The plan's List of Covered Drugs (Formulary)**

The plan has a **List of Covered Drugs (Formulary)**. We call it the "Drug List" for short. It tells which Part D prescription drugs are covered under the Part D benefit included in our plan. The drugs on this list are selected by the plan with the help of a team of doctors and pharmacists. The list must meet requirements set by Medicare. Medicare has approved the plan's Drug List.

In addition to the drugs covered by Part D, some prescription drugs are covered for you under your Medicaid benefits. You can learn more about prescription drug coverage under your Medicaid benefits by contacting your Medicaid health plan or Nebraska Department of Health and Human Services (Medicaid) listed in Chapter 2 of this booklet. Your Medicaid health plan or Nebraska Department of Health and Human Services (Medicaid) may also be able to provide a Medicaid Drug List that tells you how to find out which drugs are covered under Medicaid.

The Drug List also tells you if there are any rules that restrict coverage for your drugs.

We will provide you a copy of the Drug List. To get the most complete and current information about which drugs are covered, you can visit the plan's website ([myuhc.com/communityplan](https://myuhc.com/communityplan)) or call Customer Service.

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## **Section 4      Your monthly costs for the plan**

Your costs may include the following:

- ☐ Plan Premium (Section 4.1)
- ☐ Monthly Medicare Part B Premium (Section 4.2)
- ☐ Part D Late Enrollment Penalty (Section 4.3)
- ☐ Income Related Monthly Adjusted Amount (Section 4.4)

**In some situations, your plan premium could be less**

The “Extra Help” program helps people with limited resources pay for their drugs. Chapter 2, Section 7 tells more about this program. If you qualify, enrolling in the program might lower your monthly plan premium.

If you are **already enrolled** and getting help from this program, **the information about premiums in this Evidence of Coverage may not apply to you.**

Medicare Part B and Part D premiums differ for people with different incomes. If you have questions about these premiums review your copy of Medicare & You 2024 handbook, the section called “2024 Medicare Costs.” If you need a copy you can download it from the Medicare website (medicare.gov). Or, you can order a printed copy by phone at 1-800-MEDICARE (1-800-633-4227), 24 hours a day, 7 days a week. TTY users call 1-877-486-2048.

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## **Section 4.1      Plan premium**

As a member of our plan, you pay a monthly plan premium unless you qualify for “Extra Help” with your prescription drug costs. You may not pay a monthly Plan premium (prescription drug plan premium) if you qualify for “Extra Help”. People with Medicare and Medicaid automatically qualify for “Extra Help”. For 2024, the monthly premium for our plan is \$42.20.

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## **Section 4.2      Monthly Medicare Part B Premium**

### **Many members are required to pay other Medicare premiums**

In addition to paying the monthly plan premium, some members are required to pay other Medicare premiums. As explained in Section 2 above, in order to be eligible for our plan, you must maintain your eligibility for Nebraska Department of Health and Human Services (Medicaid) as well as have both Medicare Part A and Medicare Part B. For most UHC Dual Complete NE-V001 (HMO-POS D-SNP) members, Nebraska Department of Health and Human Services (Medicaid) pays for your Part A premium (if you don’t qualify for it automatically) and for your Part B premium.

**If Medicaid is not paying your Medicare premiums for you, you must continue to pay your Medicare premiums to remain a member of the plan.** This includes your premium for Part B. It may also include a premium for Part A which affects members who aren’t eligible for premium free Part A.

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## **Section 4.3      Part D Late Enrollment Penalty**

Because you are dually-eligible, the LEP doesn’t apply to you as long as you maintain your dually-eligible status, but if you lose your dually-eligible status, you may incur an LEP. The Part D late enrollment penalty is an additional premium that must be paid for Part D coverage if at any time after your initial enrollment period is over, there is a period of 63 days or more in a row when you did not have Part D or other creditable prescription drug coverage. “Creditable prescription drug coverage” is coverage that meets Medicare’s minimum standards since it is expected to pay, on average, at least as much as Medicare’s standard prescription drug coverage. The cost of the late



enrollment penalty depends on how long you went without Part D or other creditable prescription drug coverage. You will have to pay this penalty for as long as you have Part D coverage.

The Part D late enrollment penalty is added to your monthly premium. When you first enroll in our plan, we let you know the amount of the penalty.

You will not have to pay it if:

- ☐ You receive “Extra Help” from Medicare to pay for your prescription drugs.
- ☐ You have gone less than 63 days in a row without creditable coverage.
- ☐ You have had creditable drug coverage through another source such as a former employer, union, TRICARE, or Department of Veterans Affairs. Your insurer or your human resources department will tell you each year if your drug coverage is creditable coverage. This information may be sent to you in a letter or included in a newsletter from the plan. Keep this information, because you may need it if you join a Medicare drug plan later.
  - ☐ **Note:** Any notice must state that you had “creditable” prescription drug coverage that is expected to pay as much as Medicare’s standard prescription drug plan pays.
  - ☐ **Note:** The following are **not** creditable prescription drug coverage: prescription drug discount cards, free clinics, and drug discount websites.

**Medicare determines the amount of the penalty.** Here is how it works:

- ☐ If you went 63 days or more without Part D or other creditable prescription drug coverage after you were first eligible to enroll in Part D, the plan will count the number of full months that you did not have coverage. The penalty is 1% for every month that you did not have creditable coverage. For example, if you go 14 months without coverage, the penalty will be 14%.
- ☐ Then Medicare determines the amount of the average monthly premium for Medicare drug plans in the nation from the previous year. For 2024, this average premium amount is \$34.70.
- ☐ To calculate your monthly penalty, you multiply the penalty percentage and the average monthly premium and then round it to the nearest 10 cents. In the example here it would be 14% times \$34.70, which equals \$4.86. This rounds to \$4.90. This amount would be added **to the monthly premium for someone with a Part D late enrollment penalty.**

There are three important things to note about this monthly Part D late enrollment penalty:

- ☐ First, **the penalty may change each year**, because the average monthly premium can change each year.
- ☐ Second, **you will continue to pay a penalty** every month for as long as you are enrolled in a plan that has Medicare Part D drug benefits, even if you change plans.
- ☐ Third, if you are under 65 and currently receiving Medicare benefits, the Part D late enrollment penalty will reset when you turn 65. After age 65, your Part D late enrollment penalty will be based only on the months that you don’t have coverage after your initial enrollment period for aging into Medicare.

If you disagree about your Part D late enrollment penalty, you or your representative can ask for a review. Generally, you must request this review **within 60 days** from the date on the first letter you

receive stating you have to pay a late enrollment penalty. However, if you were paying a penalty before joining our plan, you may not have another chance to request a review of that late enrollment penalty.

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## **Section 4.4      Income Related Monthly Adjustment Amount**

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Some members may be required to pay an extra charge, known as the Part D Income Related Monthly Adjustment Amount, also known as IRMAA. The extra charge is figured out using your modified adjusted gross income as reported on your IRS tax return from 2 years ago. If this amount is above a certain amount, you'll pay the standard premium amount and the additional IRMAA. For more information on the extra amount you may have to pay based on your income, visit [medicare.gov/drug-coverage-part-d/costs-for-medicare-drug-coverage/monthly-premium-for-drug-plans](https://www.medicare.gov/drug-coverage-part-d/costs-for-medicare-drug-coverage/monthly-premium-for-drug-plans).

If you have to pay an extra amount, Social Security, not your Medicare plan, will send you a letter telling you what that extra amount will be. The extra amount will be withheld from your Social Security, Railroad Retirement Board, or Office of Personnel Management benefit check, no matter how you usually pay your plan premium, unless your monthly benefit isn't enough to cover the extra amount owed. If your benefit check isn't enough to cover the extra amount, you will get a bill from Medicare. You must pay the extra amount to the government. It cannot be paid with your monthly plan premium. If you do not pay the extra amount you will be disenrolled from the plan and lose prescription drug coverage.

If you disagree about paying an extra amount, you can ask Social Security to review the decision. To find out more about how to do this, contact Social Security at 1-800-772-1213 (TTY 1-800-325-0778).

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## **Section 5      More information about your monthly premium**

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### **Section 5.1      There are several ways you can pay your plan premium**

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There are four ways you can pay your plan premium.

#### **Option 1: Paying by check**

We will send you a monthly bill for your monthly plan premium. Make your payment payable to UnitedHealthcare. Please see your bill for the mailing address and other information. Include your member ID number on your check or money order. If making a payment for more than one member, include a payment slip for each member. Include the member ID number for each member on the check or money order. All payments must be received on or before the due date shown on the monthly bill. If you need your monthly bill replaced, please call Customer Service.

#### **Option 2: Electronic Funds Transfer**



Instead of paying by check, you can have your monthly plan premium automatically deducted from your checking account. Your monthly payment will be deducted around the 5th of each month. If you wish to sign up for Electronic Funds Transfer (EFT), you may follow the instructions on your monthly bill, or you may call Customer Service.

### **Option 3: Paying by credit card**

Instead of paying by check, you can pay your monthly plan premium with your credit card. If you wish to sign up to use your credit card to pay your monthly plan premium please call Customer Service.

### **Option 4: Having your plan premium taken out of your monthly Social Security check**

**Changing the way you pay your premium.** If you decide to change the option by which you pay your premium, it can take up to three months for your new payment method to take effect. While we are processing your request for a new payment method, you are responsible for making sure that your plan premium is paid on time. Please contact Customer Service to notify us of your premium payment option choice or if you'd like to change your existing option. (You can find our phone number on the cover of this booklet.)

### **What to do if you are having trouble paying your plan premium**

Your plan premium is due in our office by the first day of the month. If we have not received your payment by the first day of the month, we will send you a delinquency notice. In addition, we have the right to pursue collection of these premium amounts you owe.

If you are having trouble paying your premium on time, please contact Customer Service to see if we can direct you to programs that will help with your costs.

If we end your membership because you did not pay your plan premium, you will have health coverage under Original Medicare. As long as you are receiving "Extra Help" with your prescription drug costs, you will continue to have Part D drug coverage. Medicare will enroll you into a new prescription drug plan for your Part D coverage.

At the time we end your membership, you may still owe us for premiums you have not paid. We have the right to pursue collection of the premiums you owe. If you request enrollment in one of our plans and have unpaid premiums in a current or prior plan of ours, we have the right to require payment of any premium amounts you owe, before allowing you to enroll.

If you think we have wrongfully ended your membership, you can make a complaint (also called a grievance); see Chapter 9 for how to file a complaint. If you had an emergency circumstance that was out of your control and it caused you to not be able to pay your plan premium within our grace period, you can make a complaint. For complaints, we will review our decision again. Chapter 9, Section 11 of this document tells how to make a complaint or you can call us at 1-844-368-7149 between 8 a.m.-8 p.m.: 7 Days Oct-Mar; M-F Apr-Sept. TTY users should call 711. You must make your request no later than 60 days after the date your membership ends.

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## **Section 5.2 Can we change your monthly plan premium during the year?**

**No.** We are not allowed to change the amount we charge for the plan's monthly plan premium during the year. If the monthly plan premium changes for next year we will tell you in September and the change will take effect on January 1.

However, in some cases the part of the premium that you have to pay can change during the year. This happens if you become eligible for the "Extra Help" program or if you lose your eligibility for the "Extra Help" program during the year. If a member qualifies for "Extra Help" with their prescription drug costs, the "Extra Help" program will pay all or part of the member's monthly plan premium. If Medicare pays only a portion of this premium, we will bill you for the amount Medicare doesn't cover. A member who loses their eligibility during the year will need to start paying their full monthly premium. You can find out more about the "Extra Help" program in Chapter 2, Section 7.

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## Section 6      Keeping your plan membership record up to date

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Your membership record has information from your enrollment form, including your address and telephone number. It shows your specific plan coverage including your Primary Care Provider.

The doctors, hospitals, pharmacists, and other providers in the plan's network need to have correct information about you. **These network providers use your membership record to know what services and drugs are covered and the cost-sharing amounts for you.** Because of this, it is very important that you help us keep your information up to date.

### Let us know about these changes:

- ☐ Changes to your name, your address, or your phone number.
- ☐ Changes in any other medical or drug insurance coverage you have (such as from your employer, your spouse or domestic partner's employer, Workers' Compensation, or Medicaid).
- ☐ If you have any liability claims, such as claims from an automobile accident.
- ☐ If you have been admitted to a nursing home.
- ☐ If you receive care in an out-of-area or out-of-network hospital or emergency room.
- ☐ If your designated responsible party (such as a caregiver) changes.
- ☐ If you are participating in a clinical research study. (Note: You are not required to tell your plan about the clinical research studies you intend to participate in but we encourage you to do so)

If any of this information changes, please let us know by calling Customer Service.

It is also important to contact Social Security if you move or change your mailing address. You can find phone numbers and contact information for Social Security in Chapter 2, Section 5.

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## Section 7      How other insurance works with our plan

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## Other insurance

Medicare requires that we collect information from you about any other medical or drug insurance coverage that you have. That's because we must coordinate any other coverage you have with your benefits under our plan. This is called Coordination of Benefits.

Once each year, we will send you a letter that lists any other medical or drug insurance coverage that we know about. Please read over this information carefully. If it is correct, you don't need to do anything. If the information is incorrect, or if you have other coverage that is not listed, please call Customer Service. You may need to give your plan member ID number to your other insurers (once you have confirmed their identity) so your bills are paid correctly and on time.

When you have other insurance (like employer group health coverage), there are rules set by Medicare that decide whether our plan or your other insurance pays first. The insurance that pays first is called the "primary payer" and pays up to the limits of its coverage. The one that pays second, called the "secondary payer," only pays if there are costs left uncovered by the primary coverage. The secondary payer may not pay all of the uncovered costs. If you have other insurance, tell your doctor, hospital, and pharmacy.

These rules apply for employer or union group health plan coverage:

- ☐ If you have retiree coverage, Medicare pays first.
- ☐ If your group health plan coverage is based on your or a family member's current employment, who pays first depends on your age, the number of people employed by your employer, and whether you have Medicare based on age, disability, or End-Stage Renal Disease (ESRD):
  - ☐ If you're under 65 and disabled and you or your family member is still working, your group health plan pays first if the employer has 100 or more employees or at least one employer in a multiple employer plan that has more than 100 employees.
  - ☐ If you're over 65 and you or your spouse or domestic partner is still working, your group health plan pays first if the employer has 20 or more employees or at least one employer in a multiple employer plan that has more than 20 employees.
- ☐ If you have Medicare because of ESRD, your group health plan will pay first for the first 30 months after you become eligible for Medicare.

These types of coverage usually pay first for services related to each type:

- ☐ No-fault insurance (including automobile insurance)
- ☐ Liability (including automobile insurance)
- ☐ Black lung benefits
- ☐ Workers' Compensation

Nebraska Department of Health and Human Services (Medicaid) and TRICARE never pay first for Medicare-covered services. They only pay after Medicare and/or employer group health plans have paid.

# **Chapter 2**

Important phone numbers and resources

## Section 1 UHC Dual Complete NE-V001 (HMO-POS D-SNP) Contacts (how to contact us, including how to reach Customer Service)

### How to contact our plan's Customer Service

For assistance with claims, billing, or UnitedHealthcare member ID card questions, please call or write to our plan Customer Service. We will be happy to help you.

| Method  | Customer Service - Contact Information                                                                                                                                                                                         |
|---------|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Call    | <b>1-844-368-7149</b><br>Calls to this number are free.<br>Hours of Operation: 8 a.m.-8 p.m.: 7 Days Oct-Mar; M-F Apr-Sept<br>Customer Service also has free language interpreter services available for non-English speakers. |
| TTY     | <b>711</b><br>Calls to this number are free.<br>Hours of Operation: 8 a.m.-8 p.m.: 7 Days Oct-Mar; M-F Apr-Sept                                                                                                                |
| Write   | UnitedHealthcare Customer Service Department<br>P.O. Box 30770, Salt Lake City, UT 84130-0770                                                                                                                                  |
| Website | <b><a href="https://myuhc.com/communityplan">myuhc.com/communityplan</a></b>                                                                                                                                                   |

### How to contact us when you are asking for a coverage decision or appeal about your medical care

A coverage decision is a decision we make about your benefits and coverage or about the amount we will pay for your medical services or Part D prescription drugs. An appeal is a formal way of asking us to review and change a coverage decision we have made. For more information on asking for coverage decisions or appeals about your medical care or Part D prescription drugs, see Chapter 9 (What to do if you have a problem or complaint (coverage decisions, appeals, complaints)).

You may call us if you have questions about our coverage decision process.

| Method | Coverage Decisions for Medical Care – Contact Information                                                                  |
|--------|----------------------------------------------------------------------------------------------------------------------------|
| Call   | <b>1-844-368-7149</b><br>Calls to this number are free.<br>Hours of Operation: 8 a.m.-8 p.m.: 7 Days Oct-Mar; M-F Apr-Sept |
| TTY    | <b>711</b>                                                                                                                 |

| Method         | Coverage Decisions for Medical Care – Contact Information                                                                   |
|----------------|-----------------------------------------------------------------------------------------------------------------------------|
|                | Calls to this number are free.<br>Hours of Operation: 8 a.m.-8 p.m.: 7 Days Oct-Mar; M-F Apr-Sept                           |
| <b>Fax</b>     | <b>1-888-950-1170</b>                                                                                                       |
| <b>Write</b>   | UnitedHealthcare Customer Service Department (Organization Determinations)<br>P.O. Box 30770, Salt Lake City, UT 84130-0770 |
| <b>Website</b> | <b>myuhc.com/communityplan</b>                                                                                              |

| Method         | Appeals for Medical Care – Contact Information                                                                                                                                                                                                                                                           |
|----------------|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <b>Call</b>    | <b>1-844-368-7149</b><br>Calls to this number are free.<br>Hours of Operation: 8 a.m.-8 p.m.: 7 Days Oct-Mar; M-F Apr-Sept<br>For fast/expedited appeals for medical care:<br><b>1-855-409-7041</b><br>Calls to this number are free.<br>Hours of Operation: 8 a.m.-8 p.m.: 7 Days Oct-Mar; M-F Apr-Sept |
| <b>TTY</b>     | <b>711</b><br><br>Calls to this number are free.<br>Hours of Operation: 8 a.m.-8 p.m.: 7 Days Oct-Mar; M-F Apr-Sept                                                                                                                                                                                      |
| <b>Fax</b>     | For fast/expedited appeals only:<br><b>1-866-373-1081</b>                                                                                                                                                                                                                                                |
| <b>Write</b>   | UnitedHealthcare Appeals and Grievances Department<br>P.O. Box 6106, MS CA124-0187, Cypress, CA 90630-0016                                                                                                                                                                                               |
| <b>Website</b> | <b>myuhc.com/communityplan</b>                                                                                                                                                                                                                                                                           |

| Method      | Coverage Decisions for Part D Prescription Drugs – Contact Information                                                     |
|-------------|----------------------------------------------------------------------------------------------------------------------------|
| <b>Call</b> | <b>1-844-368-7149</b><br>Calls to this number are free.<br>Hours of Operation: 8 a.m.-8 p.m.: 7 Days Oct-Mar; M-F Apr-Sept |

| Method         | Coverage Decisions for Part D Prescription Drugs – Contact Information                                              |
|----------------|---------------------------------------------------------------------------------------------------------------------|
| <b>TTY</b>     | <b>711</b><br><br>Calls to this number are free.<br>Hours of Operation: 8 a.m.-8 p.m.: 7 Days Oct-Mar; M-F Apr-Sept |
| <b>Write</b>   | OptumRx Prior Authorization Department<br>P.O. Box 25183, Santa Ana, CA 92799                                       |
| <b>Website</b> | <b>myuhc.com/communityplan</b>                                                                                      |

| Method         | Appeals for Part D Prescription Drugs – Contact Information                                                                                                                                                                                                                                                           |
|----------------|-----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <b>Call</b>    | <b>1-844-368-7149</b><br>Calls to this number are free.<br>Hours of Operation: 8 a.m.-8 p.m.: 7 Days Oct-Mar; M-F Apr-Sept<br>For fast/expedited appeals for Part D prescription drugs:<br><b>1-855-409-7041</b><br>Calls to this number are free.<br>Hours of Operation: 8 a.m.-8 p.m.: 7 Days Oct-Mar; M-F Apr-Sept |
| <b>TTY</b>     | <b>711</b><br><br>Calls to this number are free.<br>Hours of Operation: 8 a.m.-8 p.m.: 7 Days Oct-Mar; M-F Apr-Sept                                                                                                                                                                                                   |
| <b>Fax</b>     | For standard Part D prescription drug appeals:<br><b>1-866-308-6294</b><br>For fast/expedited Part D prescription drug appeals:<br><b>1-866-308-6296</b>                                                                                                                                                              |
| <b>Write</b>   | UnitedHealthcare Part D Appeal and Grievance Department<br>P.O. Box 6106, MS CA124-0197, Cypress, CA 90630-0016                                                                                                                                                                                                       |
| <b>Website</b> | <b>myuhc.com/communityplan</b>                                                                                                                                                                                                                                                                                        |

### How to contact us when you are making a complaint about your medical care

You can make a complaint about us or one of our network providers, or pharmacies, including a complaint about the quality of your care. This type of complaint does not involve coverage or payment disputes. For more information on making a complaint about your medical care, see Chapter 9 (**What to do if you have a problem or complaint (coverage decisions, appeals, complaints)**).



| Method                  | Complaints about Medical Care – Contact Information                                                                                                                                                                                                                                                           |
|-------------------------|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <b>Call</b>             | <b>1-844-368-7149</b><br>Calls to this number are free.<br>Hours of Operation: 8 a.m.-8 p.m.: 7 Days Oct-Mar; M-F Apr-Sept<br>For fast/expedited complaints about medical care:<br><b>1-855-409-7041</b><br>Calls to this number are free.<br>Hours of Operation: 8 a.m.-8 p.m.: 7 Days Oct-Mar; M-F Apr-Sept |
| <b>TTY</b>              | <b>711</b><br><br>Calls to this number are free.<br>Hours of Operation: 8 a.m.-8 p.m.: 7 Days Oct-Mar; M-F Apr-Sept                                                                                                                                                                                           |
| <b>Fax</b>              | For fast/expedited complaints only:<br><b>1-866-373-1081</b>                                                                                                                                                                                                                                                  |
| <b>Write</b>            | UnitedHealthcare Appeals and Grievances Department<br>P.O. Box 6106, MS CA124-0187, Cypress, CA 90630-0016                                                                                                                                                                                                    |
| <b>Medicare Website</b> | You can submit a complaint about UHC Dual Complete NE-V001 (HMO-POS D-SNP) directly to Medicare. To submit an online complaint to Medicare, go to <b><a href="https://www.medicare.gov/MedicareComplaintForm/home.aspx">medicare.gov/MedicareComplaintForm/home.aspx</a></b> .                                |

| Method       | Complaints about Part D Prescription Drugs – Contact Information                                                                                                                                                                                                                                                           |
|--------------|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <b>Call</b>  | <b>1-844-368-7149</b><br>Calls to this number are free.<br>Hours of Operation: 8 a.m.-8 p.m.: 7 Days Oct-Mar; M-F Apr-Sept<br>For fast/expedited complaints about Part D prescription drugs:<br><b>1-855-409-7041</b><br>Calls to this number are free.<br>Hours of Operation: 8 a.m.-8 p.m.: 7 Days Oct-Mar; M-F Apr-Sept |
| <b>TTY</b>   | <b>711</b><br><br>Calls to this number are free.<br>Hours of Operation: 8 a.m.-8 p.m.: 7 Days Oct-Mar; M-F Apr-Sept                                                                                                                                                                                                        |
| <b>Fax</b>   | For standard Part D prescription drug complaints:<br><b>1-866-308-6294</b><br>For fast/expedited Part D prescription drug complaints:<br><b>1-866-308-6296</b>                                                                                                                                                             |
| <b>Write</b> | UnitedHealthcare Part D Appeal and Grievance Department                                                                                                                                                                                                                                                                    |

| Method                  | Complaints about Part D Prescription Drugs – Contact Information                                                                                                                                                                                                               |
|-------------------------|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
|                         | P.O. Box 6106, MS CA124-0197, Cypress, CA 90630-0016                                                                                                                                                                                                                           |
| <b>Medicare Website</b> | You can submit a complaint about UHC Dual Complete NE-V001 (HMO-POS D-SNP) directly to Medicare. To submit an online complaint to Medicare, go to <b><a href="https://www.medicare.gov/MedicareComplaintForm/home.aspx">medicare.gov/MedicareComplaintForm/home.aspx</a></b> . |

**Where to send a request asking us to pay for our share of the cost for medical care or a drug you have received.**

If you have received a bill or paid for services (such as a provider bill) that you think we should pay for, you may need to ask us for reimbursement or to pay the provider bill. See Chapter 7 (**Asking us to pay our share of a bill you have received for covered medical services or drugs**).

Please note: If you send us a payment request and we deny any part of your request, you can appeal our decision. See Chapter 9 (**What to do if you have a problem or complaint (coverage decisions, appeals, complaints)**) for more information.

| Method         | Payment Requests – Contact Information                                                                                                                                                            |
|----------------|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <b>Call</b>    | <b>1-844-368-7149</b><br>Calls to this number are free.<br>Hours of Operation: 8 a.m.-8 p.m.: 7 Days Oct-Mar; M-F Apr-Sept                                                                        |
| <b>TTY</b>     | <b>711</b><br>Calls to this number are free.<br>Hours of Operation: 8 a.m.-8 p.m.: 7 Days Oct-Mar; M-F Apr-Sept                                                                                   |
| <b>Write</b>   | Medical claims payment requests:<br>UnitedHealthcare<br>P.O. Box 5240, Kingston, NY 12402-5240<br>Part D prescription drug payment requests:<br>OptumRx<br>P.O. Box 650287, Dallas, TX 75265-0287 |
| <b>Website</b> | <b><a href="https://myuhc.com/communityplan">myuhc.com/communityplan</a></b>                                                                                                                      |

**Section 2 Medicare (how to get help and information directly from the federal Medicare program)**

Medicare is the Federal health insurance program for people 65 years of age or older, some people under age 65 with disabilities, and people with End-Stage Renal Disease (permanent kidney failure requiring dialysis or a kidney transplant).

The Federal agency in charge of Medicare is the Centers for Medicare & Medicaid Services (sometimes called “CMS”). This agency contracts with Medicare Advantage organizations, including us.

| Method  | Medicare – Contact Information                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                    |
|---------|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Call    | <p><b>1-800-MEDICARE, or 1-800-633-4227</b></p> <p>Calls to this number are free.<br/>24 hours a day, 7 days a week.</p>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                          |
| TTY     | <p><b>1-877-486-2048</b></p> <p>This number requires special telephone equipment and is only for people who have difficulties with hearing or speaking.<br/>Calls to this number are free.</p>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                    |
| Website | <p><b>medicare.gov</b></p> <p>This is the official government website for Medicare. It gives you up-to-date information about Medicare and current Medicare issues. It also has information about hospitals, nursing homes, physicians, home health agencies, and dialysis facilities. It includes documents you can print directly from your computer. You can also find Medicare contacts in your state.</p> <p>The Medicare website also has detailed information about your Medicare eligibility and enrollment options with the following tools:</p> <ul style="list-style-type: none"> <li><input type="checkbox"/> <b>Medicare Eligibility Tool:</b> Provides Medicare eligibility status information.</li> <li><input type="checkbox"/> <b>Medicare Plan Finder:</b> Provides personalized information about available Medicare prescription drug plans, Medicare health plans, and Medigap (Medicare Supplement Insurance) policies in your area. These tools provide an <b>estimate</b> of what your out-of-pocket costs might be in different Medicare plans.</li> </ul> <p>You can also use the website to tell Medicare about any complaints you have about UHC Dual Complete NE-V001 (HMO-POS D-SNP):</p> <ul style="list-style-type: none"> <li><input type="checkbox"/> <b>Tell Medicare about your complaint:</b> You can submit a complaint about UHC Dual Complete NE-V001 (HMO-POS D-SNP) directly to Medicare. To submit a complaint to Medicare, go to <a href="https://www.medicare.gov/MedicareComplaintForm/home.aspx">medicare.gov/MedicareComplaintForm/home.aspx</a>. Medicare takes your complaints seriously and will use this information to help improve the quality of the Medicare program.</li> </ul> <p>If you don’t have a computer, your local library or senior center may be able to help you visit this website using its computer. Or, you can call</p> |

| Method | Medicare – Contact Information                                                                                                                                                                                                                                                   |
|--------|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
|        | Medicare and tell them what information you are looking for. They will find the information on the website and review the information with you. (You can call Medicare at 1-800-MEDICARE (1-800-633-4227), 24 hours a day, 7 days a week. TTY users should call 1-877-486-2048.) |

### Section 3      **State Health Insurance Assistance Program (free help, information, and answers to your questions about Medicare)**

The State Health Insurance Assistance Program (SHIP) is a government program with trained counselors in every state. In your state, the SHIP is called Nebraska Senior Health Insurance Information Program (SHIIP).

Your SHIP is an independent (not connected with any insurance company or health plan) state program that gets money from the Federal government to give free local health insurance counseling to people with Medicare.

SHIP counselors can help you understand your Medicare rights, help you make complaints about your medical care or treatment, and help you straighten out problems with your Medicare bills. SHIP counselors can also help you with Medicare questions or problems and help you understand your Medicare plan choices and answer questions about switching plans.

#### **Method to access SHIP and other resources**

- ☐ Visit <https://www.shiphelp.org> (Click on SHIP LOCATOR in middle of page)
- ☐ Select your STATE from the list. This will take you to a page with phone numbers and resources specific to your state.

| Method  | State Health Insurance Assistance Program (SHIP) – Contact Information<br>Nebraska<br>Nebraska Senior Health Insurance Information Program (SHIIP) |
|---------|----------------------------------------------------------------------------------------------------------------------------------------------------|
| Call    | 1-800-234-7119                                                                                                                                     |
| TTY     | 711                                                                                                                                                |
| Write   | 2717 S. 8th Street, STE 4, Lincoln, NE 68508                                                                                                       |
| Website | <a href="https://doi.nebraska.gov/consumer/senior-health">https://doi.nebraska.gov/consumer/senior-health</a>                                      |

## Section 4 Quality Improvement Organization

There is a designated Quality Improvement Organization serving Medicare beneficiaries in each state. For Nebraska, the Quality Improvement Organization is called Livanta BFCC-QIO Program.

Your state's Quality Improvement Organization has a group of doctors and other health care professionals who are paid by Medicare to check on and help improve the quality of care for people with Medicare. The state's Quality Improvement Organization is an independent organization. It is not connected with our plan.

You should contact your state's Quality Improvement Organization in any of these situations:

- ☐ You have a complaint about the quality of care you have received.
- ☐ You think coverage for your hospital stay is ending too soon.
- ☐ You think coverage for your home health care, skilled nursing facility care, or Comprehensive Outpatient Rehabilitation Facility (CORF) services are ending too soon.

| Method  | Quality Improvement Organization (QIO) – Contact Information<br>Nebraska<br>Livanta BFCC-QIO Program                                             |
|---------|--------------------------------------------------------------------------------------------------------------------------------------------------|
| Call    | <b>1-888-755-5580</b><br><b>9 a.m. - 5 p.m. local time, Monday - Friday; 11 a.m. - 3 p.m. local time, weekends and holidays</b>                  |
| TTY     | <b>1-888-985-9295</b><br>This number requires special telephone equipment and is only for people who have difficulties with hearing or speaking. |
| Write   | 10820 Guilford RD, STE 202, Annapolis Junction, MD 20701                                                                                         |
| Website | <b><a href="http://www.livantaqio.com">www.livantaqio.com</a></b>                                                                                |

## Section 5 Social Security

Social Security is responsible for determining eligibility and handling enrollment for Medicare. U.S. citizens and lawful permanent residents who are 65 or older, or who have a disability or End-Stage Renal Disease and meet certain conditions, are eligible for Medicare. If you are already getting Social Security checks, enrollment into Medicare is automatic. If you are not getting Social Security checks, you have to enroll in Medicare. To apply for Medicare, you can call Social Security or visit your local Social Security office.

Social Security is also responsible for determining who has to pay an extra amount for their Part D drug coverage because they have a higher income. If you got a letter from Social Security telling

you that you have to pay the extra amount and have questions about the amount or if your income went down because of a life-changing event, you can call Social Security to ask for reconsideration. If you move or change your mailing address, it is important that you contact Social Security to let them know.

| Method  | Social Security – Contact Information                                                                                                                                                                                                               |
|---------|-----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Call    | <b>1-800-772-1213</b><br>Calls to this number are free.<br>Available 8:00 am to 7:00 pm, Monday through Friday.<br>You can use Social Security’s automated telephone services to get recorded information and conduct some business 24 hours a day. |
| TTY     | <b>1-800-325-0778</b><br>This number requires special telephone equipment and is only for people who have difficulties with hearing or speaking.<br>Calls to this number are free.<br>Available 8:00 am to 7:00 pm, Monday through Friday.          |
| Website | <b>ssa.gov</b>                                                                                                                                                                                                                                      |

## Section 6 Medicaid

Medicaid is a joint Federal and state government program that helps with medical costs for certain people with limited incomes and resources. As a member of this plan, you qualify for Medicare and Medicaid. Depending on your State and eligibility, Medicaid may pay for homemaker, personal care and other services that are not paid for by Medicare.

The programs offered through Medicaid help people with Medicare pay their Medicare costs, such as their Medicare premiums. These “Medicare Savings Programs” help people with limited income and resources save money each year.

You can enroll in this plan if you are in one of these Medicaid categories:

- **Qualified Medicare Beneficiary Plus (QMB+):** You get Medicaid coverage of Medicare cost-share and are also eligible for full Medicaid benefits. Medicaid pays your Medicare Part A and Part B premiums, deductibles, coinsurance and copayment amounts for Medicare covered services. You pay nothing, except for Part D prescription drug copays (if applicable).
- **Qualified Medicare Beneficiary (QMB):** You get Medicaid coverage of Medicare cost-share but are not eligible for full Medicaid benefits. Medicaid pays your Medicare Part A and Part B premiums, deductibles, coinsurance and copayment amounts only for Medicare covered services. You pay nothing, except for Part D prescription drug copays (if applicable).

- **Qualified Disabled and Working Individual (QDWI):** Medicaid pays your Part A premium only. The State Medicaid Office does not pay your cost-share. You do not have full Medicaid benefits. There may be some services that do not have a member cost share amount
- **Qualifying Individual (QI):** Medicaid pays your part B premium only. The State Medicaid Office does not pay your cost-share. You do not have full Medicaid benefits. You pay the cost share amounts listed in the chart below. There may be some services that do not have a member cost share amount. The State Medicaid Office does not pay your cost-share. You do not have full Medicaid benefits. There may be some services that do not have a member cost share amount.
- **Specified Low-Income Medicare Beneficiary (SLMB+):** Medicaid pays your Part B premium and provides full Medicaid benefits. You are eligible for full Medicaid benefits. At times you may also be eligible for limited assistance from your state Medicaid agency in paying your Medicare cost share amounts. Generally your cost share is 0% when the service is covered by both Medicare and Medicaid. There may be cases where you have to pay cost sharing when a service or benefit is not covered by Medicaid.
- **Specified Low-Income Medicare Beneficiary (SLMB):** Medicaid pays your Part B premium only. The State Medicaid Office does not pay your cost-share. You do not have full Medicaid benefits. There may be some services that do not have a member cost share amount.
- **Full Benefits Dual Eligible (FBDE):** Medicaid may provide limited assistance with Medicare cost-sharing. Medicaid also provides full Medicaid benefits. You are eligible for full Medicaid benefits. At times you may also be eligible for limited assistance from the State Medicaid Office in paying your Medicare cost share amounts. Generally your cost share is 0% when the service is covered by both Medicare and Medicaid. There may be cases where you have to pay cost sharing when a service or benefit is not covered by Medicaid.

If you have questions about the assistance you get from Medicaid, contact Nebraska Department of Health and Human Services (Medicaid).

| Method  | State Medicaid Program – Contact Information<br>Nebraska<br>Nebraska Department of Health and Human Services (Medicaid)                   |
|---------|-------------------------------------------------------------------------------------------------------------------------------------------|
| Call    | 1-402-471-3121<br>8 a.m. - 5 p.m. CT, Monday - Friday                                                                                     |
| TTY     | 1-800-471-7352<br>This number requires special telephone equipment and is only for people who have difficulties with hearing or speaking. |
| Write   | P.O. Box 95026, Lincoln, NE 68509-5026                                                                                                    |
| Website | <a href="https://dhhs.ne.gov/Pages/medicaid-and-long-term-care.aspx">dhhs.ne.gov/Pages/medicaid-and-long-term-care.aspx</a>               |



| <b>Method</b>  | <b>State Medicaid Office (for information about eligibility) – Contact Information</b><br><b>Nebraska</b><br><b>Medicaid Nebraska DHHS Document Imaging Center</b> |
|----------------|--------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <b>Call</b>    | <b>1-855-632-7633</b><br><b>8 a.m. - 5 p.m. CT, Monday - Friday</b>                                                                                                |
| <b>TTY</b>     | <b>1-402-471-7256</b><br>This number requires special telephone equipment and is only for people who have difficulties with hearing or speaking.                   |
| <b>Write</b>   | P.O. Box 2992, Omaha, NE 68103-2992                                                                                                                                |
| <b>Website</b> | <b><a href="https://dhhs.ne.gov/Pages/Medicaid-Eligibility.aspx">dhhs.ne.gov/Pages/Medicaid-Eligibility.aspx</a></b>                                               |

| <b>Method</b>  | <b>State Medicaid Office (for information about coverage and services) – Contact Information</b><br><b>Nebraska</b><br><b>Nebraska Department of Health &amp; Human Services</b> |
|----------------|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <b>Call</b>    | <b>1-402-471-3121</b><br><b>8 a.m. - 5 p.m. CT, Monday - Friday</b>                                                                                                              |
| <b>TTY</b>     | <b>711</b><br>This number requires special telephone equipment and is only for people who have difficulties with hearing or speaking.                                            |
| <b>Write</b>   | P.O. Box 95026, Lincoln, NE 68509-5026                                                                                                                                           |
| <b>Website</b> | <b><a href="https://dhhs.ne.gov/Pages/Medicaid-Services.aspx">dhhs.ne.gov/Pages/Medicaid-Services.aspx</a></b>                                                                   |

The Ombudsman program helps people enrolled in Medicaid with service or billing problems. They can help you file a grievance or appeal with our plan.

| <b>Method</b> | <b>State Ombudsman Program – Contact Information</b><br><b>Nebraska</b><br><b>The Office of the Public Counsel - State Capitol</b> |
|---------------|------------------------------------------------------------------------------------------------------------------------------------|
| <b>Call</b>   | <b>1-800-742-7690</b><br><b>8 a.m. - 5 p.m. local time, Monday - Friday</b>                                                        |

| Method  | State Ombudsman Program – Contact Information<br>Nebraska<br>The Office of the Public Counsel - State Capitol                                    |
|---------|--------------------------------------------------------------------------------------------------------------------------------------------------|
| TTY     | <b>1-800-833-7352</b><br>This number requires special telephone equipment and is only for people who have difficulties with hearing or speaking. |
| Write   | P.O. Box 94604, Lincoln, NE 68509-4604                                                                                                           |
| Website | <b><a href="http://nebraskalegislature.gov/divisions/ombud.php">nebraskalegislature.gov/divisions/ombud.php</a></b>                              |

The long-term care ombudsman program helps people get information about nursing homes and resolve problems between nursing homes and residents or their families.

| Method  | State Long-Term Care Ombudsman Program<br>Nebraska<br>Department of Health and Human Services                                         |
|---------|---------------------------------------------------------------------------------------------------------------------------------------|
| Call    | <b>1-800-942-7830</b><br><b>8 a.m. - 5 p.m. local time, Monday - Friday</b>                                                           |
| TTY     | <b>711</b><br>This number requires special telephone equipment and is only for people who have difficulties with hearing or speaking. |
| Write   | P.O. Box 95026, Lincoln, NE 68509-5026                                                                                                |
| Website | <b><a href="http://dhhs.ne.gov/Pages/Aging-Ombudsman.aspx">http://dhhs.ne.gov/Pages/Aging-Ombudsman.aspx</a></b>                      |

## Section 7 Information about programs to help people pay for their prescription drugs

The Medicare.gov website ([medicare.gov/drug-coverage-part-d/costs-for-medicare-drug-coverage/costs-in-the-coverage-gap/5-ways-to-get-help-with-prescription-costs](http://medicare.gov/drug-coverage-part-d/costs-for-medicare-drug-coverage/costs-in-the-coverage-gap/5-ways-to-get-help-with-prescription-costs)) provides information on how to lower your prescription drug costs. For people with limited incomes, there are also other programs to assist, described below.

### Medicare's "Extra Help" Program

Because you are eligible for Medicaid, you qualify for and are getting "Extra Help" from Medicare to pay for your prescription drug plan costs. You do not need to do anything further to get this "Extra Help."

If you have questions about "Extra Help," call:

- ☐ 1-800-MEDICARE (1-800-633-4227). TTY users should call 1-877-486-2048, 24 hours a day, 7 days a week;
- ☐ The Social Security Office at 1-800-772-1213, between 8 am to 7 pm, Monday through Friday. TTY users should call 1-800-325-0778 (applications); or
- ☐ Your State Medicaid Office (applications). (See Section 6 of this chapter for contact information.)

If you believe that you are paying an incorrect cost-sharing amount when you get your prescription at a pharmacy, our plan has a process for you to either request assistance in obtaining evidence of your proper copayment level, or, if you already have the evidence, to provide this evidence to us.

- ☐ Fax the information to 501-262-7070 or mail it to P.O. Box 29300, Hot Springs, AR 71903-9300.
- ☐ When we receive the evidence showing your copayment level, we will update our system so that you can pay the correct copayment when you get your next prescription at the pharmacy. If you overpay your copayment, we will reimburse you. Either we will forward a check to you in the amount of your overpayment or we will offset future copayments. If the pharmacy hasn't collected a copayment from you and is carrying your copayment as a debt owed by you, we may make the payment directly to the pharmacy. If a state paid on your behalf, we may make payment directly to the state. Please contact Customer Service if you have questions.

### **What if you have coverage from an AIDS Drug Assistance Program (ADAP)?**

#### **What is the AIDS Drug Assistance Program (ADAP)?**

The AIDS Drug Assistance Program (ADAP) helps ADAP-eligible individuals living with HIV/AIDS have access to life-saving HIV medications. Medicare Part D prescription drugs that are also on the ADAP formulary qualify for prescription cost-sharing assistance. **Note:** To be eligible for the ADAP operating in your State, individuals must meet certain criteria, including proof of State residence and HIV status, low income as defined by the State, and uninsured/under-insured status.

If you change plans please notify your local ADAP enrollment worker so you can continue to receive assistance. For information on eligibility criteria, covered drugs, or how to enroll in the program, please call your state ADAP office listed below.

| Method  | AIDS Drug Assistance Program (ADAP) – Contact Information<br>Nebraska Department of Health & Human Services |
|---------|-------------------------------------------------------------------------------------------------------------|
| Call    | 1-402-471-2101<br>8 a.m.-5 p.m. local time, Monday-Friday                                                   |
| Website | <a href="http://dhhs.ne.gov/Pages/Ryan-White.aspx">http://dhhs.ne.gov/Pages/Ryan-White.aspx</a>             |

## **Section 8      How to contact the Railroad Retirement Board**

The Railroad Retirement Board is an independent Federal agency that administers comprehensive benefit programs for the nation's railroad workers and their families. If you receive your Medicare through the Railroad Retirement Board, it is important that you let them know if you move or

change your mailing address. If you have questions regarding your benefits from the Railroad Retirement Board, contact the agency.

| Method  | Railroad Retirement Board – Contact Information                                                                                                                                                                                                                                                                                                                        |
|---------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Call    | <b>1-877-772-5772</b><br>Calls to this number are free.<br>If you press “0,” you may speak with an RRB representative from 9:00 am to 3:30 pm, Monday, Tuesday, Thursday, and Friday, and from 9:00 am to 12:00 pm on Wednesday. If you press “1”, you may access the automated RRB HelpLine and recorded information 24 hours a day, including weekends and holidays. |
| TTY     | <b>1-312-751-4701</b><br>This number requires special telephone equipment and is only for people who have difficulties with hearing or speaking.<br>Calls to this number are <b>not</b> free.                                                                                                                                                                          |
| Website | <b>rrb.gov/</b>                                                                                                                                                                                                                                                                                                                                                        |

## Section 9 Do you have “group insurance” or other health insurance from an employer?

If you (or your spouse or domestic partner) get benefits from your (or your spouse or domestic partner’s) employer or retiree group as part of this plan, you may call the employer/union benefits administrator or Customer Service if you have any questions. You can ask about your (or your spouse or domestic partner’s) employer or retiree health benefits, premiums, or the enrollment period. (Phone numbers for Customer Service are printed on the cover of this document.) You may also call 1-800-MEDICARE (1-800-633-4227; TTY: 1-877-486-2048) with questions related to your Medicare coverage under this plan or enrollment periods to make a change.

If you have other prescription drug coverage through your (or your spouse or domestic partner’s) employer or retiree group, please contact **that group’s benefits administrator**. The benefits administrator can help you determine how your current prescription drug coverage will work with our plan.

# **Chapter 3**

Using the plan for  
your medical services

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## Section 1      Things to know about getting your medical care as a member of our plan

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This chapter explains what you need to know about using the plan to get your medical care covered. It gives definitions of terms and explains the rules you will need to follow to get the medical treatments, services, equipment, prescription drugs, and other medical care that are covered by the plan.

For the details on what medical care is covered by our plan and how much you pay when you get this care, use the benefits chart in the next chapter, Chapter 4 (**Medical Benefits Chart, what is covered and what you pay**).

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### Section 1.1      What are “network providers” and “covered services”?

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- ☐ **“Providers”** are doctors and other health care professionals licensed by the state to provide medical services and care. The term “providers” also includes hospitals and other health care facilities.
- ☐ **“Network providers”** are the doctors and other health care professionals, medical groups, hospitals, and other health care facilities that have an agreement with us to accept our payment and your cost-sharing amount as payment in full. We have arranged for these providers to deliver covered services to members in our plan. The providers in our network bill us directly for care they give you. When you see a network provider, you pay only your share of the cost for covered services.
- ☐ **“Covered services”** include all the medical care, health care services, supplies, equipment, and prescription drugs that are covered by our plan. Your covered services for medical care are listed in the benefits chart in Chapter 4. Your covered services for prescription drugs are discussed in Chapter 5.

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### Section 1.2      Basic rules for getting your medical care covered by the plan

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As a Medicare and Medicaid health plan, UHC Dual Complete NE-V001 (HMO-POS D-SNP) must cover all services covered by Original Medicare and may offer other services in addition to those covered under Original Medicare as noted in Chapter 4.

The plan will generally cover your medical care as long as:

- ☐ **The care you receive is included in the plan’s Medical Benefits Chart** (this chart is in Chapter 4 of this document).
- ☐ **The care you receive is considered medically necessary.** “Medically necessary” means that the services, supplies, equipment, or drugs are needed for the prevention, diagnosis, or treatment of your medical condition and meet accepted standards of medical practice.
- ☐ **You have a network primary care provider (a PCP) who is providing and overseeing your care.** As a member of our plan, you must choose a network PCP (for more information about this, see Section 2.1 in this chapter).

- ❑ **You must receive your care from a network provider** (for more information about this, see Section 2 in this chapter). In most cases, care you receive from an out-of-network provider (a provider who is not part of our plan's network) will not be covered. This means you will have to pay the provider in full for the services furnished. **Here are three exceptions:**
  - ❑ The plan covers emergency care or urgently needed services that you get from an out-of-network provider. For more information about this, and to see what emergency or urgently needed services means, see Section 3 in this chapter.
  - ❑ If you need medical care that Medicare requires our plan to cover but there are no specialists in our network that provide this care, you can get this care from an out-of-network provider at the same cost-sharing you normally pay in-network. In this situation, we will cover these services as if you got the care from a network provider. You must get approval from us before you start receiving care from an out-of-network provider. Please contact Customer Service, or have your PCP or the out-of-network provider call us to get approval (phone numbers are printed on the cover of this booklet).
  - ❑ The plan covers kidney dialysis services that you get at a Medicare-certified dialysis facility when you are temporarily outside the plan's service area or when your provider for this service is temporarily unavailable or inaccessible. The cost-sharing you pay the plan for dialysis can never exceed the cost-sharing in Original Medicare. If you are outside the plan's service area and obtain the dialysis from a provider that is outside the plan's network, your cost-sharing cannot exceed the cost-sharing you pay in-network. However, if your usual in-network provider for dialysis is temporarily unavailable and you choose to obtain services inside the service area from a provider outside the plan's network the cost-sharing for the dialysis may be higher.

While you are a member of our Point of Service (POS) plan you may use either network providers or out-of-network providers for covered routine dental services. Please see Ch. 3, Sec. 2.3.

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## **Section 2                      Using network and out-of-network providers to get your medical care**

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### **Section 2.1                      You must choose a primary care provider (PCP) to provide and oversee your medical care**

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**What is a "PCP" and what does the PCP do for you?**

**What is a PCP?**

A primary care provider (PCP) is a network physician who is selected by you to provide and coordinate your covered services.

**What types of providers may act as a PCP?**

PCPs are generally physicians specializing in Internal Medicine, Family Practice or General Practice.



### **What is the role of my PCP?**

Your relationship with your PCP is an important one because your PCP is responsible for the coordination of your health care and is also responsible for your routine health care needs. You may want to ask your PCP for assistance in selecting a network specialist and follow-up with your PCP after any specialist visits. It is important for you to develop and maintain a relationship with your PCP.

### **How do you choose your PCP?**

You must select a PCP from the **Provider Directory** at the time of your enrollment. You may, however, visit any network provider you choose.

For a copy of the most recent **Provider Directory**, or for help in selecting a PCP, call Customer Service or visit the website listed in Chapter 2 of this booklet for the most up-to-date information about our network providers.

If you do not select a PCP at the time of enrollment, we may pick one for you. You may change your PCP at any time. See “Changing your PCP” below.

### **Changing your PCP**

You may change your PCP for any reason, at any time. Also, it’s possible that your PCP might leave our plan’s network of providers and you would have to find a new PCP.

If you want to change your PCP, call Customer Service or go online. If the PCP is accepting additional plan members, the change will become effective on the first day of the following month. You will receive a new UnitedHealthcare member ID card that shows this change.

---

## **Section 2.2      How to get care from specialists and other network providers**

A specialist is a doctor who provides health care services for a specific disease or part of the body. There are many kinds of specialists. Here are a few examples:

- ☐ Oncologists care for patients with cancer.
- ☐ Cardiologists care for patients with heart conditions.
- ☐ Orthopedists care for patients with certain bone, joint, or muscle conditions.

If you use an out-of-network provider for routine dental services, your share of the costs for your covered services are described in "Covered Routine Dental Benefits" in Chapter 4.

Even though your PCP is trained to handle the majority of common health care needs, there may be a time when you feel that you need to see a network specialist. **You do not need a referral from your PCP to see a network specialist or behavioral/mental health provider.** Although you do not need a referral from your PCP to see a network specialist, your PCP can recommend an appropriate network specialist for your medical condition, answer questions you have regarding a network specialist’s treatment plan and provide follow-up health care as needed. For coordination of care, we recommend you notify your PCP when you see a network specialist.

Please refer to the **Provider Directory** for a listing of plan specialists available through your network, or you may consult the **Provider Directory** online at the website listed in Chapter 2 of this booklet.

### **What if a specialist or another network provider leaves our plan?**

It is important that you know that we may make changes to the hospitals, doctors, and specialists (providers) that are part of your plan during the year. If your doctor or specialist does leave your plan you have certain rights and protections that are summarized below:

- ☐ Even though our network of providers may change during the year, Medicare requires that we furnish you with uninterrupted access to qualified doctors and specialists.
- ☐ We will notify you that your provider is leaving our plan so that you have time to select a new provider.
  - ☐ If your primary care or behavioral health provider leaves our plan, we will notify you if you have seen that provider within the past three years.
  - ☐ If any of your other providers leave our plan, we will notify you if you are assigned to the provider, currently receive care from them, or have seen them within the past three months.
- ☐ We will assist you in selecting a new qualified in-network provider that you may access for continued care.
- ☐ If you are currently undergoing medical treatment or therapies with your current provider, you have the right to request, and we will work with you to ensure, that the medically necessary treatment or therapies you are receiving continues.
- ☐ We will provide you with information about the different enrollment periods available to you and options you may have for changing plans.
- ☐ We will arrange for any medically necessary covered benefit outside of our provider network, but at in-network cost sharing, when an in-network provider or benefit is unavailable or inadequate to meet your medical needs.
- ☐ If you find out your doctor or specialist is leaving your plan, please contact us so we can assist you in finding a new provider to manage your care.
- ☐ If you believe we have not furnished you with a qualified provider to replace your previous provider or that your care is not being appropriately managed, you have the right to file a quality of care complaint to the QIO, a quality of care grievance to the plan, or both. Please see Chapter 9.

You may call Customer Service for assistance at the number listed in Chapter 2 of this booklet.

Some services require prior authorization from the plan in order to be covered. Obtaining prior authorization is the responsibility of the PCP or treating provider. Services and items requiring prior authorization are listed in Medical Benefits Chart in Chapter 4, Section 2.1.

---

### **Section 2.3      How to get care from out-of-network providers**

As a member of our plan, you can choose to receive care from out-of-network providers for routine dental services only. For more information see the "**Covered Routine Dental Benefits**" in **Chapter 4**. Otherwise, care that you receive from out-of-network providers will not be covered unless the care meets one of the three exceptions described in Section 1.2 of this chapter. For information

about getting out-of-network care when you have a medical emergency or urgent need for care, please see Section 3 in this chapter.

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## **Section 3                      How to get services when you have an emergency or urgent need for care or during a disaster**

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### **Section 3.1                      Getting care if you have a medical emergency**

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#### **What is a “medical emergency” and what should you do if you have one?**

A “**medical emergency**” is when you, or any other prudent layperson with an average knowledge of health and medicine, believe that you have medical symptoms that require immediate medical attention to prevent your loss of life (and, if you are a pregnant woman, loss of an unborn child), loss of a limb or function of a limb, or loss of or serious impairment to a bodily function. The medical symptoms may be an illness, injury, severe pain, or a medical condition that is quickly getting worse.

If you have a medical emergency:

- ☐ **Get help as quickly as possible.** Call 911 for help or go to the nearest emergency room or hospital. Call for an ambulance if you need it. You do **not** need to get approval or a referral first from your PCP. You do not need to use a network doctor. You may get covered emergency medical care whenever you need it, anywhere in the world.

#### **What is covered if you have a medical emergency?**

Our plan covers ambulance services in situations where getting to the emergency room in any other way could endanger your health. We also cover medical services during the emergency.

The doctors who are giving you emergency care will decide when your condition is stable and the medical emergency is over.

After the emergency is over you are entitled to follow-up care to be sure your condition continues to be stable. Your doctors will continue to treat you until your doctors contact us and make plans for additional care. Your follow-up care will be covered by our plan. If your emergency care is provided by out-of-network providers, we will try to arrange for network providers to take over your care as soon as your medical condition and the circumstances allow.

#### **What if it wasn’t a medical emergency?**

Sometimes it can be hard to know if you have a medical emergency. For example, you might go in for emergency care – thinking that your health is in serious danger – and the doctor may say that it wasn’t a medical emergency after all. If it turns out that it was not an emergency, as long as you reasonably thought your health was in serious danger, we will cover your care.

However, after the doctor has said that it was **not** an emergency, we will cover additional care **only** if you get the additional care in one of these two ways:

- ☐ You go to a network provider to get the additional care.
- ☐ **–or–** The additional care you get is considered “urgently needed services” and you follow the rules for getting this urgent care (for more information about this, see Section 3.2 below).

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## **Section 3.2      Getting care when you have an urgent need for services**

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### **What are “urgently needed services”?**

An urgently needed service is a non-emergency situation requiring immediate medical care but, given your circumstances, it is not possible or not reasonable to obtain these services from a network provider. The plan must cover urgently needed services provided out-of-network. Some examples of urgently needed services are i) a severe sore throat that occurs over the weekend or ii) an unforeseen flare-up of a known condition when you are temporarily outside the service area.

You should always try to obtain urgently needed services from network providers. However, if providers are temporarily unavailable or inaccessible and it is not reasonable to wait to obtain care from your network provider when the network becomes available, we will cover urgently needed services that you get from an out-of-network provider. Check your **Provider Directory** for a list of network Urgent Care Centers.

Our plan covers worldwide emergency and urgently needed services outside the United States under the following circumstances: emergency services, including emergency or urgently needed care and emergency ambulance transportation from the scene of an emergency to the nearest medical treatment facility. Transportation back to the United States from another country is not covered. Pre-scheduled, pre-planned treatments (including dialysis for an ongoing condition) and/or elective procedures are not covered.

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## **Section 3.3      Getting care during a disaster**

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If the Governor of your state, the U.S. Secretary of Health and Human Services, or the President of the United States declares a state of disaster or emergency in your geographic area, you are still entitled to care from your plan.

Please visit the following website: [uhc.com/disaster-relief-info](https://uhc.com/disaster-relief-info) or contact Customer Service for information on how to obtain needed care during a disaster.

If you cannot use a network provider during a disaster, your plan will allow you to obtain care from out-of-network providers at in-network cost-sharing. If you cannot use a network pharmacy during a disaster, you may be able to fill your prescription drugs at an out-of-network pharmacy. Please see Chapter 5, Section 2.5 for more information.

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## **Section 4      What if you are billed directly for the full cost of your services?**

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### **Section 4.1      You can ask us to pay our share of the cost of covered services**

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If you have paid more than your plan cost-sharing for covered services, or if you have received a bill for the full cost of covered medical services, go to Chapter 7 (**Asking us to pay our share of a bill you have received for covered medical services or drugs**) for information about what to do.

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### **Section 4.2      What should you do if services are not covered by our plan?**

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Our plan covers all medically necessary services as listed in the Medical Benefits Chart in Chapter 4 of this document. If you receive services not covered by our plan or services obtained out-of-network and were not authorized, you are responsible for paying the full cost of services. However, before paying for the cost of the service, contact your state Medicaid office to find out if the service is covered by Medicaid.

For covered services that have a benefit limitation, you also pay the full cost of any services you get after you have used up your benefit for that type of covered service. For example, if your plan covers one routine physical exam per year and you receive that routine physical but choose to have a second routine physical within the same year, you pay the full cost of the second routine physical. Any amounts that you pay after you have reached the benefit limitation do not count toward your annual out-of-pocket maximum. (See Chapter 4 for more information on your plan's out-of-pocket maximum.)

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## **Section 5                      How are your medical services covered when you are in a                                          "clinical research study"?**

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### **Section 5.1                      What is a "clinical research study"?**

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A clinical research study (also called a "clinical trial") is a way that doctors and scientists test new types of medical care, like how well a new cancer drug works. Certain clinical research studies are approved by Medicare. Clinical research studies approved by Medicare typically request volunteers to participate in the study.

Once Medicare approves the study, and you express interest, someone who works on the study will contact you to explain more about the study and see if you meet the requirements set by the scientists who are running the study. You can participate in the study as long as you meet the requirements for the study **and** you have a full understanding and acceptance of what is involved if you participate in the study.

If you participate in a Medicare-approved study, Original Medicare pays most of the costs for the covered services you receive as part of the study. If you tell us that you are in a qualified clinical trial, then you are only responsible for in-network cost-sharing for the services in that trial. If you paid more, for example, if you already paid the Original Medicare cost-sharing amount, we will reimburse the difference between what you paid and the in-network cost-sharing. However, you will need to provide documentation to show us how much you paid. When you are in a clinical research study, you may stay enrolled in our plan and continue to get the rest of your care (the care that is not related to the study) through our plan.

If you want to participate in any Medicare-approved clinical research study, you do **not** need to tell us or get approval from us or your PCP. The providers that deliver your care as part of the clinical research study do **not** need to be part of our plan's network of providers. Please note that this does not include benefits for which our plan is responsible that include, as a component, a clinical trial or registry to assess the benefit. These include certain benefits specified under national coverage determinations (NCDs) and investigational device trials (IDE) and may be subject to prior authorization and other plan rules.

Although you do not need to get our plan's permission to be in a clinical research study, covered for Medicare Advantage enrollees by Original Medicare, we encourage you to notify us in advance when you choose to participate in Medicare-qualified clinical trials.

If you participate in a study that Medicare has not approved you will be responsible for paying all costs for your participation in the study.

## **Section 5.2      When you participate in a clinical research study, who pays for what?**

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Once you join a Medicare-approved clinical research study, Original Medicare covers the routine items and services you receive as part of the study, including:

- ☐ Room and board for a hospital stay that Medicare would pay for even if you weren't in a study.
- ☐ An operation or other medical procedure if it is part of the research study.
- ☐ Treatment of side effects and complications of the new care.

After Medicare has paid its share of the cost for these services, our plan will pay the difference between the cost-sharing in Original Medicare and your in-network cost-sharing as a member of our plan. This means you will pay the same amount for the services you receive as part of the study as you would if you received these services from our plan. However, you are required to submit documentation showing how much cost-sharing you paid. Please see Chapter 7 for more information for submitting requests for payments.

**Here's an example of how the cost-sharing works:** Let's say that you have a lab test that costs \$100 as part of the research study. Let's also say that your share of the costs for this test is \$20 under Original Medicare, but the test would be \$10 under our plan's benefits. In this case, Original Medicare would pay \$80 for the test and you would pay the \$20 copay required under Original Medicare. You would then notify your plan that you received a qualified clinical trial service and submit documentation such as a provider bill to the plan. The plan would then directly pay you \$10. Therefore, your net payment is \$10, the same amount you would pay under our plan's benefits. Please note that in order to receive payment from your plan, you must submit documentation to your plan such as a provider bill.

When you are part of a clinical research study, **neither Medicare nor our plan will pay for any of the following:**

- ☐ Generally, Medicare will **not** pay for the new item or service that the study is testing unless Medicare would cover the item or service even if you were **not** in a study.
- ☐ Items or services provided only to collect data, and not used in your direct health care. For example, Medicare would not pay for monthly CT scans done as part of the study if your medical condition would normally require only one CT scan.

### **Do you want to know more?**

You can get more information about joining a clinical research study by visiting the Medicare website to read or download the publication "Medicare and Clinical Research Studies." (The publication is available at: [medicare.gov/Pubs/pdf/02226-Medicare-and-Clinical-Research-Studies.pdf](https://www.medicare.gov/Pubs/pdf/02226-Medicare-and-Clinical-Research-Studies.pdf).) You can also call 1-800-MEDICARE (1-800-633-4227), 24 hours a day, 7 days a week. TTY users should call 1-877-486-2048.



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## **Section 6                      Rules for getting care in a “religious non-medical health care institution”**

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### **Section 6.1                      What is a religious non-medical health care institution?**

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A religious non-medical health care institution is a facility that provides care for a condition that would ordinarily be treated in a hospital or skilled nursing facility. If getting care in a hospital or a skilled nursing facility is against a member’s religious beliefs, we will instead provide coverage for care in a religious non-medical health care institution. This benefit is provided only for Part A inpatient services (non-medical health care services).

### **Section 6.2                      Receiving care from a religious non-medical health care institution**

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To get care from a religious non-medical health care institution, you must sign a legal document that says you are conscientiously opposed to getting medical treatment that is “non-excepted.”

- ☐ “Non-excepted” medical care or treatment is any medical care or treatment that is **voluntary** and **not required** by any federal, state, or local law.
- ☐ “Excepted” medical treatment is medical care or treatment that you get that is **not** voluntary or **is required** under federal, state, or local law.

To be covered by our plan, the care you get from a religious non-medical health care institution must meet the following conditions:

- ☐ The facility providing the care must be certified by Medicare.
- ☐ Our plan’s coverage of services you receive is limited to **non-religious** aspects of care.
- ☐ If you get services from this institution that are provided to you in a facility, the following conditions apply:
  - ☐ You must have a medical condition that would allow you to receive covered services for inpatient hospital care or skilled nursing facility care.
  - **and** – you must get approval in advance from our plan before you are admitted to the facility or your stay will not be covered.

You are covered for unlimited days in the hospital, as long as your stay meets Medicare coverage guidelines. The coverage limits are described under **inpatient hospital care** in the medical benefits chart in Chapter 4.

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## **Section 7                      Rules for ownership of durable medical equipment**

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### **Section 7.1                      Will you own the durable medical equipment after making a certain number of payments under our plan?**

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Durable medical equipment (DME) includes items such as oxygen equipment and supplies, wheelchairs, walkers, powered mattress systems, crutches, diabetic supplies, speech generating devices, IV infusion pumps, nebulizers, and hospital beds ordered by a provider for use in the home. The member always owns certain items, such as prosthetics. In this section, we discuss other types of DME that you must rent.

In Original Medicare, people who rent certain types of DME own the equipment after paying copayments for the item for 13 months. As a member of our plan, however, you usually will not acquire ownership of rented DME items no matter how many copayments you make for the item while a member of our plan, even if you made up to 12 consecutive payments for the DME item under Original Medicare before you joined our plan. Under certain limited circumstances we will transfer ownership of the durable medical equipment item. Call Customer Service for more information.

### **What happens to payments you made for durable medical equipment if you switch to Original Medicare?**

If you did not acquire ownership of the DME item while in our plan, you will have to make 13 new consecutive payments after you switch to Original Medicare in order to own the item. The payments made while enrolled in your plan do not count.

Example 1: You made 12 or fewer consecutive payments for the item in Original Medicare and then joined our plan. The payments you made in Original Medicare do not count.

Example 2: You made 12 or fewer consecutive payments for the item in Original Medicare and then joined our plan. You were in our plan but did not obtain ownership while in our plan. You then go back to Original Medicare. You will have to make 13 consecutive new payments to own the item once you join Original Medicare again. All previous payments (whether to our plan or to Original Medicare) do not count.

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## **Section 7.2      Rules for oxygen equipment, supplies, and maintenance**

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### **What oxygen benefits are you entitled to?**

If you qualify for Medicare oxygen equipment coverage our plan will cover:

- ☐ Rental of oxygen equipment
- ☐ Delivery of oxygen and oxygen contents
- ☐ Tubing and related oxygen accessories for the delivery of oxygen and oxygen contents
- ☐ Maintenance and repairs of oxygen equipment

If you leave our plan or no longer medically require oxygen equipment, then the oxygen equipment must be returned.

### **What happens if you leave your plan and return to Original Medicare?**

Original Medicare requires an oxygen supplier to provide you services for five years. During the first 36 months you rent the equipment. The remaining 24 months the supplier provides the equipment and maintenance (you are still responsible for the copayment for oxygen). After five years you may choose to stay with the same company or go to another company. At this point, the five-year cycle begins again, even if you remain with the same company, requiring you to pay copayments for the first 36 months. If you join or leave our plan, the five-year cycle starts over.



# **Chapter 4**

Medical Benefits

Chart (what is covered and what  
you pay)

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## **Section 1                      Understanding your out-of-pocket costs for covered services**

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This chapter provides a medical benefits chart that lists your covered services and shows how much you will pay for each covered service as a member of UHC Dual Complete NE-V001 (HMO-POS D-SNP). Later in this chapter, you can find information about medical services that are not covered. It also explains limits on certain services.

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### **Section 1.1                      Types of out-of-pocket costs you may pay for your covered services**

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To understand the payment information we give you in this chapter, you need to know about the types of out-of-pocket costs you may pay for your covered services.

- ☐ A **“copayment”** is the fixed amount you pay each time you receive certain medical services. You pay a copayment at the time you get the medical service. (The medical benefits chart in Section 2 tells you more about your copayments.)
- ☐ **“Coinsurance”** is the percentage you pay of the total cost of certain medical services. You pay a coinsurance at the time you get the medical service. (The medical benefits chart in Section 2 tells you more about your coinsurance.)

**QMB members - you do not have any costs for Medicare-covered services, except your prescription copayments if you are enrolled in Medicare as a Qualified Medicare Beneficiary (QMB) and Nebraska Department of Health and Human Services (Medicaid). Your coinsurance, deductibles and copayments (except for Part D prescription drugs) are paid by Nebraska Department of Health and Human Services (Medicaid).**

Non-QMB members - you may have costs if Nebraska Department of Health and Human Services (Medicaid) does not cover cost-sharing for non-QMB enrollees. Your costs may include premiums, deductibles, copayments and coinsurance.

Show your UnitedHealthcare member ID card and your state Medicaid ID card when getting health care services. These cards will help your health care providers coordinate payment.

Call Customer Service at the telephone number listed in Chapter 2 of this booklet if:

- ☐ you are asked to pay for covered services,
- ☐ your provider will not see you or
- ☐ you have other questions

If you receive notice that your Nebraska Department of Health and Human Services (Medicaid) coverage has expired, please call your Medicaid office right away to reapply for assistance. Your Medicaid Agency phone number is listed in Chapter 2 of this booklet. Please call Customer Service at the number listed in Chapter 2 of this booklet if you have questions.

### **Grace Period**

Members who are (Qualified Medicare Beneficiaries) QMB or have full Medicaid benefits - if you lose your Nebraska Department of Health and Human Services (Medicaid) eligibility, you can remain enrolled in this Medicare plan for up to 6 months. You must re-enroll in Medicaid before the end of the 6 month period to keep your Medicare benefits with this plan. If you go to your provider

during the 6 month period, you will have out-of-pocket costs that your Medicare plan will not cover. You will be responsible for those costs until you regain your Medicaid eligibility. Your out-of-pocket costs may include Medicare plan deductibles, copayments and coinsurance up to the Original Medicare amounts, which can be found at [medicare.gov](https://www.medicare.gov). For in-network Medicare-covered services, you will be responsible for up to \$3,800 of cost sharing calculated at the Original Medicare amounts. Out-of-network services do not count toward this maximum. In addition, if you lose your Part D Extra Help eligibility, you will also need to pay the plan premium. Please call Customer Service (phone numbers are printed on the cover of this booklet) for additional information related to out-of-pocket costs during the grace period.

If you do not re-enroll in Nebraska Department of Health and Human Services (Medicaid) during the 6 month period, you will be disenrolled from our plan. You will be enrolled in Original Medicare.

Non-QMB members - your cost-sharing will not change during the 6 month period. You may have out-of-pocket costs if Nebraska Department of Health and Human Services (Medicaid) does not cover cost-sharing for non-QMB enrollees. Your out-of-pocket costs may include premiums, deductibles, copayments and coinsurance.

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## **Section 1.2      What is the most you will pay for Medicare Part A and Part B covered medical services?**

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**Note:** Because our members also get assistance from Nebraska Department of Health and Human Services (Medicaid), very few members ever reach this out-of-pocket maximum. If you are eligible for Medicare cost-sharing assistance under Medicaid, you are not responsible for paying any out-of-pocket costs toward the maximum out-of-pocket amount for covered Part A and Part B services.

Because you are enrolled in a Medicare Advantage Plan, there is a limit on the amount you have to pay out-of-pocket each year for in-network medical services that are covered under Medicare Part A and Part B. This limit is called the maximum out-of-pocket amount for medical services. For calendar year 2024 this amount is \$3,800.

The amounts you pay for your copayments and coinsurance for in-network covered services count toward this maximum out-of-pocket amount. The amounts you pay for your plan premium and for your Part D prescription drugs do not count toward your maximum out-of-pocket amount. In addition, amounts you pay for some services do not count toward your maximum out-of-pocket amount. These services are marked with an asterisk in the medical benefits chart. If you reach the maximum out-of-pocket amount of \$3,800, you will not have to pay any out-of-pocket costs for the rest of the year for in-network covered Part A and Part B services. However, you must continue to pay your plan premium and the Medicare Part B premium (unless your Part B premium is paid for you by Nebraska Department of Health and Human Services (Medicaid) or another third party).

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## **Section 1.3      Our plan does not allow network providers to “balance bill” you**

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As a member of UHC Dual Complete NE-V001 (HMO-POS D-SNP), an important protection for you is that you only have to pay your cost-sharing amount when you get services covered by our plan. Providers may not add additional separate charges, called “balance billing.” This protection applies even if we pay the provider less than the provider charges for a service and even if there is a dispute and we don’t pay certain provider charges.

Here is how this protection works.

- ☐ If your cost-sharing is a copayment (a set amount of dollars, for example, \$15.00) then you pay only that amount for any covered services from a network provider.
- ☐ If your cost-sharing is a coinsurance (a percentage of the total charges), then you never pay more than that percentage. However, your cost depends on which type of provider you see:
  - ☐ If you receive the covered services from a network provider, you pay the coinsurance percentage multiplied by the plan's reimbursement rate (as determined in the contract between the provider and the plan).
  - ☐ If you receive the covered services from an out-of-network provider who participates with Medicare, you pay the coinsurance percentage multiplied by the Medicare payment rate for participating providers.
  - ☐ If you receive the covered services from an out-of-network provider who does not participate with Medicare, you pay the coinsurance percentage multiplied by the Medicare payment rate for non-participating providers.
- ☐ If you believe a provider has "balance billed" you, call Customer Service.

## Section 2      Use the medical benefits chart to find out what is covered and how much you will pay

### Section 2.1      Your medical benefits and costs as a member of the plan

The medical benefits chart on the following pages lists the services UHC Dual Complete NE-V001 (HMO-POS D-SNP) covers and what you pay out-of-pocket for each service. Part D prescription drug coverage is covered in Chapter 5. The services listed in the medical benefits chart are covered only when the following coverage requirements are met:

- ☐ Your Medicare-covered services must be provided according to the coverage guidelines established by Medicare.
- ☐ Your services (including medical care, services, supplies, equipment, and Part B prescription drugs) **must** be medically necessary. "Medically necessary" means that the services, supplies, or drugs are needed for the prevention, diagnosis, or treatment of your medical condition and meet accepted standards of medical practice.
- ☐ You receive your care from a network provider. In most cases, care you receive from an out-of-network provider will not be covered, unless it is emergent or urgent care or unless your plan or a network provider has given you a referral. This means that you will have to pay the provider in full for the services furnished.
- ☐ You have a Primary care provider (a PCP) who is providing and overseeing your care.
- ☐ Some of the services listed in the medical benefits chart are covered **only** if your doctor or other network provider gets approval in advance (sometimes called "prior authorization") from UHC Dual Complete NE-V001 (HMO-POS D-SNP).
  - ☐ Covered services that may need approval in advance are marked by a double dagger (††) in the medical benefits chart.
  - ☐ Network providers agree by contract to obtain prior authorization from the plan and agree not to balance bill you.

Other important things to know about our coverage:

- ☐ You are covered by both Medicare and Medicaid. Medicare covers health care and prescription drugs. Medicaid covers your cost-sharing for Medicare services. Medicaid also covers services Medicare does not cover, like long-term care and home and community-based services.
- ☐ Like all Medicare health plans, we cover everything that Original Medicare covers. (If you want to know more about the coverage and costs of Original Medicare, look in your **Medicare & You 2024** handbook. View it online at [medicare.gov](https://www.medicare.gov) or ask for a copy by calling 1-800-MEDICARE (1-800-633-4227), 24 hours a day, 7 days a week. TTY users should call 1-877-486-2048.)
- ☐ For all Preventive services that are covered at no cost under Original Medicare, we also cover the service at no cost to you. However, if you also are treated or monitored for an existing medical condition during the visit when you receive the preventive service, a copayment will apply for the care received for the existing medical condition.
- ☐ If Medicare adds coverage for any new services during 2024, either Medicare or our plan will cover those services.
- ☐ If you are within our plan's 6-month period of deemed continued eligibility, we will continue to provide all appropriate Medicare Advantage plan Medicare-covered benefits. However, during this period, we will not continue to cover Medicaid benefits that are included under the Medicaid State Plan, nor will we pay the Medicare premiums or cost sharing for which the state would otherwise be liable.
- ☐ Medicare approved UHC Dual Complete NE-V001 (HMO-POS D-SNP) to provide these benefits and/or lower copayments/coinsurance as part of the Value-Based Insurance Design program. This program lets Medicare try new ways to improve Medicare Advantage plans.
- ☐ Important benefit information for all enrollees participating in wellness and health care planning (WHP) services
- ☐ Because UHC Dual Complete NE-V001 (HMO-POS D-SNP) participates in Food, Over-the-Counter (OTC) and utility bill credit/RX Cost-Share Buy Down, you will be eligible for the following WHP services, including advance care planning (ACP) services:
  - ☐ What are ACP services?

Sometimes people become unable to make health care decisions for themselves due to accidents or serious illness. You have the right to say what you want to happen if you are in this situation. This means that, **if you want to**, you can:

    - ☐ Fill out a written form to give **someone the legal authority to make medical decisions for you** if you ever become unable to make decisions for yourself.
    - ☐ **Give your doctors written instructions** about how you want them to handle your medical care if you become unable to make decisions for yourself.
    - ☐ You may get advance care planning assistance by contacting Customer Service (phone numbers are printed on the cover of this booklet).
- ☐ Important Benefit Information for Enrollees Who Qualify for Extra Help:
- ☐ If you receive Extra Help to pay your Medicare prescription drug program costs, such as premiums, deductibles, and coinsurance, you may be eligible for other targeted supplemental benefits and/or targeted reduced cost sharing.

- ☐ Please go to the Food, Over-the-Counter (OTC) and utility bill credit row in the medical benefits chart in Chapter 4 for further detail.
- ☐ Members qualify for the elimination of their cost sharing for Part D drugs. See Chapter 5 for further detail.

 You will see this apple next to the Preventive services in the benefits chart.

**Medically Necessary** - means health care services, supplies, or drugs needed for the prevention, diagnosis, or treatment of your sickness, injury or illness that are all of the following as determined by us or our designee, within our sole discretion:

- ☐ In accordance with **Generally accepted standards of medical practice**.
- ☐ Most appropriate, in terms of type, frequency, extent, site and duration, and considered effective for your sickness, injury, or illness.
- ☐ Not mainly for your convenience or that of your doctor or other health care provider.
- ☐ Meet, but do not exceed your medical need, are at least as beneficial as an existing and available medically appropriate alternative, and are furnished in the most cost-effective manner that may be provided safely and effectively.

**Generally accepted standards of medical practice** are standards that are based on credible scientific evidence published in peer-reviewed medical literature generally recognized by the relevant medical community, relying primarily on controlled clinical trials, or, if not available, observational studies from more than one institution that suggest a causal relationship between the service or treatment and health outcomes.

If no credible scientific evidence is available, then standards that are based on Physician specialty society recommendations or professional standards of care may be considered. We reserve the right to consult expert opinion in determining whether health care services are Medically Necessary. The decision to apply Physician specialty society recommendations, the choice of expert and the determination of when to use any such expert opinion, shall be within our sole discretion.

## Medical Benefits Chart

| Services that are covered for you                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                           | What you must pay when you get these services                                                                                                                                                                                                                                                                                                                |
|-----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <p><b>Medicare cost sharing</b></p> <p>Original Medicare covered benefits include:</p> <ul style="list-style-type: none"> <li>Abdominal aortic aneurysm screening</li> <li>Acupuncture for chronic low back pain</li> <li>Ambulance services</li> <li>Annual wellness visit</li> <li>Bone mass measurement</li> <li>Breast cancer screening (mammograms)</li> <li>Cardiac rehabilitation services</li> <li>Cardiovascular disease risk reduction visit (therapy for cardiovascular disease)</li> <li>Cardiovascular disease testing</li> <li>Cervical and vaginal cancer screening</li> <li>Chiropractic services</li> <li>Colorectal cancer screening</li> <li>Depression screening</li> <li>Diabetes screening</li> <li>Diabetes self-management training, diabetic services and supplies</li> <li>Durable medical equipment and related supplies</li> <li>Emergency care</li> <li>Hearing services</li> <li>HIV screening</li> <li>Home health agency care</li> <li>Home infusion therapy</li> <li>Hospice care</li> <li>Immunizations</li> <li>Inpatient hospital care</li> <li>Inpatient mental health care</li> </ul> | <p>For Medicare covered services, you pay:</p> <p>\$0 if you are enrolled in Medicaid as a Qualified Medicare Beneficiary (QMB).</p> <p>\$0 if you are enrolled in Medicaid with full benefits as a non-QMB except for services that are not covered by the state Medicaid program.</p> <p>See Chapter 2, Section 6 for more information about Medicaid.</p> |

If you are a Qualified Medicare Beneficiary (QMB) or have full Medicaid benefits then your deductible, coinsurance and/or copayment may be less for services that are covered under Original Medicare.



| Services that are covered for you                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                  | What you must pay when you get these services |
|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-----------------------------------------------|
| <p>Medical nutrition therapy</p> <p>Medicare diabetes prevention program (MDPP)</p> <p>Medicare Part B prescription drugs</p> <p>Obesity screening and therapy to promote sustained weight loss</p> <p>Opioid treatment services</p> <p>Outpatient diagnostic tests and therapeutic services and supplies</p> <p>Outpatient hospital observation</p> <p>Outpatient hospital services</p> <p>Outpatient mental health care</p> <p>Outpatient rehabilitation services</p> <p>Outpatient substance abuse services</p> <p>Outpatient surgery provided at hospital outpatient facilities and ambulatory surgical centers</p> <p>Partial hospitalization services and Intensive outpatient services</p> <p>Physician/practitioner services, including doctor's office visits</p> <p>Podiatry services</p> <p>Prostate cancer screening exams</p> <p>Prosthetic devices and related supplies</p> <p>Pulmonary rehabilitation services</p> <p>Screening and counseling to reduce alcohol misuse</p> <p>Screening for lung cancer with low dose computed tomography (LDCT)</p> <p>Screening for sexually transmitted infections (STIs) and counseling to prevent STIs</p> <p>Services to treat kidney disease and conditions</p> <p>Skilled nursing facility (SNF) care</p> |                                               |

If you are a Qualified Medicare Beneficiary (QMB) or have full Medicaid benefits then your deductible, coinsurance and/or copayment may be less for services that are covered under Original Medicare.

| Services that are covered for you                                                                                                                                                                                                                                                                                                                                                                                    | What you must pay when you get these services                                                                |
|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|--------------------------------------------------------------------------------------------------------------|
| <p>Smoking and tobacco use cessation (counseling to stop smoking or tobacco use)</p> <p>Supervised exercise therapy (SET)</p> <p>Urgently needed services</p> <p>Vision care</p> <p>“Welcome to Medicare” preventive visit</p>                                                                                                                                                                                       |                                                                                                              |
| <p> <b>Abdominal aortic aneurysm screening</b></p> <p>A one-time (once per lifetime) screening ultrasound for people at risk. The plan only covers this screening if you have certain risk factors and if you get a referral for it from your physician, physician assistant, nurse practitioner, or clinical nurse specialist.</p> | <p>There is no coinsurance, copayment, or deductible for members eligible for this preventive screening.</p> |

If you are a Qualified Medicare Beneficiary (QMB) or have full Medicaid benefits then your deductible, coinsurance and/or copayment may be less for services that are covered under Original Medicare.

| Services that are covered for you                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                            | What you must pay when you get these services                                                                                                                                                                                                                                                       |
|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <p><b>Acupuncture for chronic low back pain</b></p> <p>Covered services include:</p> <p>Up to 12 visits in 90 days performed by, or under the supervision of a physician (or other medical provider as described below) are covered for Medicare beneficiaries under the following circumstances:</p> <p>For the purpose of this benefit, chronic low back pain is defined as:</p> <ul style="list-style-type: none"> <li><input type="checkbox"/> Lasting 12 weeks or longer;</li> <li><input type="checkbox"/> nonspecific, in that it has no identifiable systemic cause (i.e., not associated with metastatic, inflammatory, infectious disease, etc.);</li> <li><input type="checkbox"/> not associated with surgery; and</li> <li><input type="checkbox"/> not associated with pregnancy.</li> </ul> <p>An additional eight sessions will be covered for those patients demonstrating an improvement. No more than 20 acupuncture treatments may be administered annually.</p> <p>Treatment must be discontinued if the patient is not improving or is regressing.</p> <p><b>Generally, Medicare-covered acupuncture services are not covered when provided by an acupuncturist or chiropractor.</b></p> <p>Provider Requirements:</p> <p>Physicians (as defined in 1861(r)(1) of the Social Security Act (the Act) may furnish acupuncture in accordance with applicable state requirements.</p> <p>Physician assistants (PAs), nurse practitioners (NPs)/clinical nurse specialists (CNSs) (as identified in 1861(aa)(5) of the Act), and auxiliary personnel may furnish acupuncture if they meet all applicable state requirements and have:</p> <ul style="list-style-type: none"> <li><input type="checkbox"/> a masters or doctoral level degree in acupuncture or Oriental Medicine from a school accredited by the</li> </ul> | <p>You will pay the cost-sharing that applies to primary care services or specialist physician services (as described under “Physician/practitioner services, including doctor’s office visits”) depending on if you receive services from a primary care physician or specialist.<sup>††</sup></p> |

If you are a Qualified Medicare Beneficiary (QMB) or have full Medicaid benefits then your deductible, coinsurance and/or copayment may be less for services that are covered under Original Medicare.

| Services that are covered for you                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                          | What you must pay when you get these services                                                                                                                                                                                                                                                                                                                                                                               |
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| <p>Accreditation Commission on Acupuncture and Oriental Medicine (ACAOM); and,</p> <ul style="list-style-type: none"> <li><input type="checkbox"/> a current, full, active, and unrestricted license to practice acupuncture in a State, Territory, or Commonwealth (i.e. Puerto Rico) of the United States, or District of Columbia.</li> <li><input type="checkbox"/> Benefit is not covered when solely provided by an independent acupuncturist.</li> </ul> <p>Auxiliary personnel furnishing acupuncture must be under the appropriate level of supervision of a physician, PA, or NP/CNS as required by Medicare.</p> <p>Acupuncture services performed by providers that do not meet CMS acupuncture provider requirements are not covered even in locations where there are no providers available that meet CMS requirements.</p> |                                                                                                                                                                                                                                                                                                                                                                                                                             |
| <p><b>Ambulance services</b></p> <p>Covered ambulance services, whether for an emergency or non-emergency situation, include fixed wing, rotary wing, and ground ambulance services, to the nearest appropriate facility that can provide care only if they are furnished to a member whose medical condition is such that other means of transportation could endanger the person's health or if authorized by the plan. If the covered ambulance services are not for an emergency situation, it should be documented that the member's condition is such that other means of transportation could endanger the person's health and that transportation by ambulance is medically required.</p>                                                                                                                                          | <p>\$290 copayment for each one-way Medicare-covered ground trip.</p> <p>\$290 copayment for each one-way Medicare-covered air trip.</p> <p>You pay these amounts until you reach the out-of-pocket maximum. All Medicare-covered trips (in or out-of-network) will apply to the in-network out-of-pocket maximum.</p> <p><i>Your provider may need to obtain prior authorization for non-emergency transportation.</i></p> |

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| Services that are covered for you                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                   | What you must pay when you get these services                                                        |
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| <p><b>Annual routine physical exam</b></p> <p>Includes comprehensive physical examination and evaluation of status of chronic diseases. Doesn't include lab tests, radiological diagnostic tests or non-radiological diagnostic tests. Additional cost share may apply to any lab or diagnostic testing performed during your visit, as described for each separate service in this Medical Benefits Chart. Annual Routine Physical Exam visits do not need to be scheduled 12 months apart but are limited to one visit each calendar year.</p>                                                                                                                                                                                                                                                                                                                                                                    | <p>\$0 copayment for a routine physical exam each year.</p>                                          |
| <p> <b>Annual wellness visit</b></p> <p>If you've had Part B for longer than 12 months, you can get an annual wellness visit to develop or update a personalized prevention plan based on your current health and risk factors. This is covered once every 12 months. Doesn't include lab tests, radiological diagnostic tests or non-radiological diagnostic tests. Additional cost share may apply to any lab or diagnostic testing performed during your visit, as described for each separate service in this Medical Benefits Chart.</p> <p><b>Note:</b> Your first annual wellness visit can't take place within 12 months of your Welcome to Medicare preventive visit. However, you don't need to have had a Welcome to Medicare visit to be covered for annual wellness visits after you've had Part B for 12 months.</p> | <p>There is no coinsurance, copayment, or deductible for the annual wellness visit.</p>              |
| <p> <b>Bone mass measurement</b></p> <p>For qualified individuals (generally, this means people at risk of losing bone mass or at risk of osteoporosis), the following services are covered every 24 months or more frequently if medically necessary: procedures to identify bone mass, detect bone loss, or determine bone quality, including a physician's interpretation of the results.</p>                                                                                                                                                                                                                                                                                                                                                                                                                                 | <p>There is no coinsurance, copayment, or deductible for Medicare-covered bone mass measurement.</p> |

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| Services that are covered for you                                                                                                                                                                                                                                                                                                                                                                                                                                                       | What you must pay when you get these services                                                                                      |
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| <p> <b>Breast cancer screening (mammograms)</b></p> <p>Covered services include:</p> <ul style="list-style-type: none"> <li><input type="checkbox"/> One baseline mammogram between the ages of 35 and 39</li> <li><input type="checkbox"/> One screening mammogram every 12 months for women age 40 and older</li> <li><input type="checkbox"/> Clinical breast exams once every 24 months</li> </ul> | <p>There is no coinsurance, copayment, or deductible for covered screening mammograms.</p>                                         |
| <p><b>Cardiac rehabilitation services</b></p> <p>Comprehensive programs of cardiac rehabilitation services that include exercise, education, and counseling are covered for members who meet certain conditions with a doctor's order. The plan also covers intensive cardiac rehabilitation programs that are typically more rigorous or more intense than cardiac rehabilitation programs.</p>                                                                                        | <p>\$0 copayment for each Medicare-covered cardiac rehabilitative visit.<sup>††</sup></p>                                          |
| <p> <b>Cardiovascular disease risk reduction visit (therapy for cardiovascular disease)</b></p> <p>We cover one visit per year with your primary care doctor to help lower your risk for cardiovascular disease. During this visit, your doctor may discuss aspirin use (if appropriate), check your blood pressure, and give you tips to make sure you're eating healthy.</p>                       | <p>There is no coinsurance, copayment, or deductible for the cardiovascular disease preventive benefit.</p>                        |
| <p> <b>Cardiovascular disease testing</b></p> <p>Blood tests for the detection of cardiovascular disease (or abnormalities associated with an elevated risk of cardiovascular disease) covered once every 5 years (60 months).</p>                                                                                                                                                                   | <p>There is no coinsurance, copayment, or deductible for cardiovascular disease testing that is covered once every five years.</p> |

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| Services that are covered for you                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                 | What you must pay when you get these services                                                                  |
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| <p> <b>Cervical and vaginal cancer screening</b></p> <p>Covered services include:</p> <ul style="list-style-type: none"> <li><input type="checkbox"/> For all women: Pap tests and pelvic exams are covered once every 24 months</li> <li><input type="checkbox"/> If you are at high risk of cervical or vaginal cancer or you are of childbearing age and have had an abnormal Pap test within the past 3 years: one Pap test every 12 months</li> <li><input type="checkbox"/> For asymptomatic women between the ages of 30 and 65: HPV testing once every 5 years, in conjunction with the Pap test</li> </ul>                                                                                                                                                                                                                                                                              | <p>There is no coinsurance, copayment, or deductible for Medicare-covered preventive Pap and pelvic exams.</p> |
| <p><b>Chiropractic services</b></p> <p>Covered services include:</p> <ul style="list-style-type: none"> <li><input type="checkbox"/> Manual manipulation of the spine to correct subluxation (when one or more of the bones of your spine move out of position). Manual manipulation is a treatment that uses hands-on pressure to gently move your joints and tissues.</li> </ul> <p>Excluded from Medicare coverage is any service other than manual manipulation for the treatment of subluxation, including:</p> <ul style="list-style-type: none"> <li><input type="checkbox"/> Maintenance therapy. Chiropractic treatment is considered maintenance therapy when continuous ongoing care is no longer expected to provide clinical improvements and the treatment becomes supportive instead of corrective.</li> <li><input type="checkbox"/> Extra charges when your chiropractor uses a manual, hand-held device to add controlled pressure during treatment.</li> </ul> | <p>\$0 copayment for each Medicare-covered visit.<sup>††</sup></p>                                             |

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| Services that are covered for you                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                        | What you must pay when you get these services                                                                                                                                                                                                                                                                                                                                                  |
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| <p><input type="checkbox"/> X-rays, massage therapy, and acupuncture (unless the acupuncture is for the treatment of chronic low back pain).</p>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                         |                                                                                                                                                                                                                                                                                                                                                                                                |
| <p><b>Routine chiropractic services</b></p> <p>We cover 12 routine chiropractic visits every year. This benefit is in addition to the Medicare-covered chiropractic services benefit listed above.</p> <p>Covered services include routine visits to treat nerve, muscle, and/or bone pain and nausea. No referral required. This benefit does not cover treatment for any other conditions not related to pain relief.</p> <p>You can get more information by viewing the Vendor Information Sheet at <a href="http://myuhc.com/communityplan">myuhc.com/communityplan</a> or by calling Customer Service to have a paper copy sent to you.</p>                                                                                                                                                                                                                                                                                                                                                                                                         | <p>Provided by: OptumHealth Care Solutions, LLC (Optum)</p> <p>\$0 copayment</p>                                                                                                                                                                                                                                                                                                               |
| <p> <b>Colorectal cancer screening</b></p> <p>The following screening tests are covered:</p> <ul style="list-style-type: none"> <li><input type="checkbox"/> Colonoscopy has no minimum or maximum age limitation and is covered once every 120 months (10 years) for patients not at high risk, or 48 months after a previous flexible sigmoidoscopy for patients who are not at high risk for colorectal cancer, and once every 24 months for high risk patients after a previous screening colonoscopy or barium enema.</li> <li><input type="checkbox"/> Flexible sigmoidoscopy for patients 45 years and older. Once every 120 months for patients not at high risk after the patient received a screening colonoscopy. Once every 48 months for high risk patients from the last flexible sigmoidoscopy or barium enema.</li> <li><input type="checkbox"/> Screening fecal-occult blood tests for patients 45 years and older. Once every 12 months.</li> </ul> | <p>There is no coinsurance, copayment, or deductible for a Medicare-covered colorectal cancer screening exam. If your doctor finds and removes a polyp or other tissue during the colonoscopy or flexible sigmoidoscopy, the screening exam becomes an outpatient diagnostic colonoscopy.</p> <p>There is no coinsurance, copayment, or deductible for each Medicare-covered barium enema.</p> |

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| Services that are covered for you                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                         | What you must pay when you get these services                                                                                                      |
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| <ul style="list-style-type: none"> <li>❑ Multitarget stool DNA for patients 45 to 85 years of age and not meeting high risk criteria. Once every 3 years.</li> <li>❑ Blood-based Biomarker Tests for patients 45 to 85 years of age and not meeting high risk criteria. Once every 3 years.</li> <li>❑ Barium Enema as an alternative to colonoscopy for patients at high risk and 24 months since the last screening barium enema or the last screening colonoscopy.</li> <li>❑ Barium Enema as an alternative to flexible sigmoidoscopy for patient not at high risk and 45 years or older. Once at least 48 months following the last screening barium enema or screening flexible sigmoidoscopy.</li> </ul> <p>Colorectal cancer screening tests include a follow-up screening colonoscopy after a Medicare covered non-invasive stool-based colorectal cancer screening test returns a positive result.</p> <p>Outpatient diagnostic colonoscopy</p> | <p>There is no coinsurance, copayment, or deductible for each Medicare-covered diagnostic colonoscopy.<sup>††</sup></p>                            |
| <p><b>Routine dental benefits</b></p> <p>You can get more information about this benefit by viewing the Vendor Information Sheet at <a href="http://myuhc.com/communityplan">myuhc.com/communityplan</a> or by calling Customer Service to have a paper copy sent to you.</p>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                             | <p>You are covered for routine dental benefits. See the routine dental benefit description at the end of this chart for details.<sup>*††</sup></p> |

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| Services that are covered for you                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                           | What you must pay when you get these services                                                                                                                                                                                                                                       |
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| <p> <b>Depression screening</b></p> <p>We cover one screening for depression per year. The screening must be done in a primary care setting that can provide follow-up treatment and/or referrals.</p>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                     | <p>There is no coinsurance, copayment, or deductible for an annual depression screening visit.</p>                                                                                                                                                                                  |
| <p> <b>Diabetes screening</b></p> <p>We cover this screening (includes fasting glucose tests) if you have any of the following risk factors: high blood pressure (hypertension), history of abnormal cholesterol and triglyceride levels (dyslipidemia), obesity, or a history of high blood sugar (glucose). Tests may also be covered if you meet other requirements, like being overweight and having a family history of diabetes.</p> <p>Based on the results of these tests, you may be eligible for up to two diabetes screenings every plan year.</p>                                                                                                                                                                                                                                                                                                                                                                                                              | <p>There is no coinsurance, copayment, or deductible for the Medicare-covered diabetes screening tests.</p>                                                                                                                                                                         |
| <p> <b>Diabetes self-management training, diabetic services and supplies</b></p> <p>For all people who have diabetes (insulin and non-insulin users). Covered services include:</p> <ul style="list-style-type: none"> <li><input type="checkbox"/> Supplies to monitor your blood glucose: blood glucose monitor, blood glucose test strips, lancet devices and lancets, and glucose-control solutions for checking the accuracy of test strips and monitors.</li> </ul> <p>UHC Dual Complete NE-V001 (HMO-POS D-SNP) covers any blood glucose monitors and test strips specified within this list. We will generally not cover alternate brands unless your doctor or other provider tells us that use of an alternate brand is medically necessary in your specific situation. If you are new to UHC Dual Complete NE-V001 (HMO-POS D-SNP) and are using a brand of blood glucose monitors and test strips that is not on our list, you may contact us within the</p> | <p>\$0 copayment for each Medicare-covered diabetes monitoring supply.<sup>††</sup></p> <p>We only cover Accu-Chek® and OneTouch® brands.</p> <p>Covered glucose monitors include: OneTouch Verio Flex®, OneTouch Verio Reflect®, OneTouch® Verio, OneTouch®Ultra 2, Accu-Chek®</p> |

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| Services that are covered for you                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                              | What you must pay when you get these services                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                   |
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| <p>first 90 days of enrollment into the plan to request a temporary supply of the alternate brand while you consult with your doctor or other provider. During this time, you should talk with your doctor to decide whether any of the preferred brands are medically appropriate for you. If you or your doctor believe it is medically necessary for you to maintain use of an alternate brand, you may request a coverage exception to have UHC Dual Complete NE-V001 (HMO-POS D-SNP) maintain coverage of a non-preferred product through the end of the benefit year. Non-preferred products will not be covered following the initial 90 days of the benefit year without an approved coverage exception.</p> <p>If you (or your provider) don't agree with the plan's coverage decision, you or your provider may file an appeal. You can also file an appeal if you don't agree with your provider's decision about what product or brand is appropriate for your medical condition. (For more information about appeals, see Chapter 9, What to do if you have a problem or complaint (coverage decisions, appeals, complaints).)</p> <p><input type="checkbox"/> For people with diabetes who have severe diabetic foot disease: One pair per calendar year of therapeutic custom-molded shoes (including inserts provided with such shoes) and two additional pairs of inserts, or one pair of depth shoes and three pairs of inserts (not including the non-customized removable inserts provided with such shoes). Coverage includes fitting.</p> <p><input type="checkbox"/> Diabetes self-management training is covered under certain conditions. Limited to 20 visits of 30 minutes per year for a maximum of 10 hours the initial year. Follow-up training subsequent years after, limited to 4 visits of 30 minutes for a maximum of 2 hours per year.</p> | <p>Guide Me, and Accu-Chek® Guide.</p> <p>Test strips: OneTouch Verio®, OneTouch Ultra®, Accu-Chek® Guide, Accu-Chek® Aviva Plus, and Accu-Chek® SmartView.</p> <p>Other brands are not covered by your plan.</p> <p>\$0 copayment for each Medicare-covered continuous glucose monitor and supplies in accordance with Medicare guidelines. There are no brand limitations for continuous glucose monitors.<sup>††</sup></p> <p>For cost-sharing applicable to insulin and syringes, see Chapter 6 - What you pay for your Part D prescription drugs.</p> <p>20% coinsurance for each pair of Medicare-covered therapeutic shoes.<sup>††</sup></p> <p>You pay these amounts until you reach the out-of-pocket maximum.</p> <p>\$0 copayment for Medicare-covered benefits.</p> |

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| Services that are covered for you                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                               | What you must pay when you get these services                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                              |
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| <p><b>Durable medical equipment (DME) and related supplies</b><br/> (For a definition of “durable medical equipment,” see Chapter 12 as well as Chapter 3, Section 7 of this document.)</p> <p>Covered items include, but are not limited to: wheelchairs, crutches, powered mattress systems, diabetic supplies, hospital beds ordered by a provider for use in the home, IV infusion pumps, speech generating devices, oxygen equipment, nebulizers, and walkers.</p> <p>We cover all medically necessary DME covered by Original Medicare. If our supplier in your area does not carry a particular brand or manufacturer, you may ask them if they can special order it for you. The most recent list of suppliers is available on our website at <a href="http://myuhc.com/communityplan">myuhc.com/communityplan</a>.</p> | <p>20% coinsurance for Medicare-covered benefits.<sup>††</sup></p> <p>Your cost-sharing for Medicare oxygen equipment coverage is 20% coinsurance, every time you get covered equipment or supplies.<sup>††</sup></p> <p>Your cost-sharing will not change after being enrolled for 36 months.</p> <p>If prior to enrolling in our plan you had made 36 months of rental payment for oxygen equipment coverage, your cost-sharing in our plan is 20% coinsurance.<sup>††</sup></p> <p>You pay these amounts until you reach the out-of-pocket maximum.</p> |
| <p><b>Emergency care</b></p> <p>Emergency care refers to services that are:</p> <ul style="list-style-type: none"> <li><input type="checkbox"/>Furnished by a provider qualified to furnish emergency services, and</li> <li><input type="checkbox"/>Needed to evaluate or stabilize an emergency medical condition.</li> </ul> <p>A medical emergency is when you, or any other prudent layperson with an average knowledge of health and medicine, believe that you have medical symptoms that require immediate medical attention to prevent loss of life (and, if you are a pregnant woman, loss of an unborn child), loss of a limb, or loss of function of a limb. The medical symptoms may be an illness, injury, severe pain, or a medical condition that is quickly getting worse.</p>                                 | <p>\$100 copayment for each emergency room visit.</p> <p>You do not pay this amount if you are admitted to the hospital within 24 hours for the same condition. If you are admitted to a hospital, you will pay cost-sharing as described in the “Inpatient hospital care” section in this benefit chart.</p> <p>You pay these amounts until you reach the out-of-pocket maximum.</p>                                                                                                                                                                      |

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| Services that are covered for you                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                              | What you must pay when you get these services                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                         |
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| <p>Cost-sharing for necessary emergency services furnished out-of-network is the same as for such services furnished in-network.</p> <p>Worldwide coverage for emergency department services outside of the United States.</p> <ul style="list-style-type: none"> <li><input type="checkbox"/> This includes emergency or urgently needed care and emergency ambulance transportation from the scene of an emergency to the nearest medical treatment facility.</li> <li><input type="checkbox"/> Transportation back to the United States from another country is not covered.</li> <li><input type="checkbox"/> Pre-scheduled, pre-planned treatments (including dialysis for an ongoing condition) and/or elective procedures are not covered.</li> <li><input type="checkbox"/> Services provided by a dentist are not covered.</li> </ul> | <p>\$0 copayment for worldwide coverage for emergency services outside of the United States. Please see Chapter 7 Section 1.1 for expense reimbursement for worldwide services.</p> <p>If you receive emergency care at an out-of-network hospital and need inpatient care after your emergency condition is stabilized, you must return to a network hospital in order for your care to continue to be covered or you must have your inpatient care at the out-of-network hospital authorized by the plan and your cost is the cost-sharing you would pay at a network hospital.</p> |
| <p> <b>Fitness program</b></p> <p>Renew Active® by UnitedHealthcare®</p> <p>Renew Active by UnitedHealthcare is the gold standard in Medicare fitness programs for body and mind. It's available to you at no additional cost and includes:</p> <ul style="list-style-type: none"> <li><input type="checkbox"/> A free gym membership, access to our nationwide network of gyms and fitness locations, a personalized fitness plan plus thousands of on-demand workout videos and live streaming fitness classes.</li> </ul>                                                                                                                                                                                                                                | <p>Provided by: Renew Active®</p> <p>\$0 copayment</p> <p>A home-delivered fitness kit is available if you live 15 miles or more from a Renew Active network gym or fitness location.</p>                                                                                                                                                                                                                                                                                                                                                                                             |

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| Services that are covered for you                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                        | What you must pay when you get these services                                        |
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| <ul style="list-style-type: none"> <li><input type="checkbox"/> An online program from AARP® Staying Sharp® offering content about brain health with exclusive content for Renew Active members.</li> <li><input type="checkbox"/> Social activities at local health and wellness classes, clubs and events.</li> <li><input type="checkbox"/> An online Fitbit® Community for Renew Active. No Fitbit device is needed.</li> </ul> <p>You can get more information by viewing the Vendor Information Sheet at <a href="http://myuhc.com/communityplan">myuhc.com/communityplan</a> or by calling Customer Service to have a paper copy sent to you.</p> |                                                                                      |
| <p><b>Hearing services</b></p> <p>Diagnostic hearing and balance evaluations performed by your provider to determine if you need medical treatment are covered as outpatient care when furnished by a physician, audiologist, or other qualified provider.</p>                                                                                                                                                                                                                                                                                                                                                                                           | <p>\$0 copayment for each Medicare-covered exam.<sup>††</sup></p>                    |
| <p><b>Hearing services - routine hearing exam</b></p> <p>We cover 1 hearing exam every year.</p>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                         | <p>Provided by: Plan network providers in your service area</p> <p>\$0 copayment</p> |

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| Services that are covered for you                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                     | What you must pay when you get these services                                                                                                                                                                                                                                                                                                                                                                                   |
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| <p><b>Hearing services - hearing aids:</b></p> <p>Through UnitedHealthcare Hearing, you can choose from a broad selection of over-the-counter (OTC) hearing aids, name-brand prescription hearing aids, or UnitedHealthcare Hearing's brand Relate®. Hearing aids can be fit in-person with a network provider or delivered directly to you with virtual follow-up care (select models).</p> <p>This benefit is limited to 2 hearing aids every year. Hearing aid accessories, additional batteries and optional services are available for purchase, but they are not covered by the plan.</p> <p>To access your hearing aid benefit and get connected with a network provider, you must contact UnitedHealthcare Hearing at 1-877-704-3384, TTY 711 or UHChearing.com/Medicare.</p> | <p>Provided by: UnitedHealthcare Hearing</p> <p>Hearing aid allowance is \$1,100</p> <p>You must obtain prior authorization from UnitedHealthcare Hearing. Additional fees may apply for optional follow-up visits.</p> <p>Home-delivered hearing aids are available nationwide through UnitedHealthcare Hearing (select products only).</p> <p>Hearing aids purchased outside of UnitedHealthcare Hearing are not covered.</p> |
| <p> <b>HIV screening</b></p> <p>For people who ask for an HIV screening test or who are at increased risk for HIV infection, we cover:</p> <ul style="list-style-type: none"> <li><input type="checkbox"/> One screening exam every 12 months</li> </ul> <p>For women who are pregnant, we cover:</p> <ul style="list-style-type: none"> <li><input type="checkbox"/> Up to three screening exams during a pregnancy</li> </ul>                                                                                                                                                                                                                                                                    | <p>There is no coinsurance, copayment, or deductible for members eligible for Medicare-covered preventive HIV screening.</p>                                                                                                                                                                                                                                                                                                    |
| <p><b>Home health agency care</b></p> <p>Prior to receiving home health services, a doctor must certify that you need home health services and will order home health services to be provided by a home health agency. You must be homebound, which means leaving home is a major effort.</p> <p>Covered services include, but are not limited to:</p>                                                                                                                                                                                                                                                                                                                                                                                                                                | <p>\$0 copayment for all home health visits provided by a network home health agency when Medicare criteria are met.<sup>††</sup></p> <p>Other copayments or coinsurance may apply (Please see Durable medical equipment</p>                                                                                                                                                                                                    |

If you are a Qualified Medicare Beneficiary (QMB) or have full Medicaid benefits then your deductible, coinsurance and/or copayment may be less for services that are covered under Original Medicare.

| Services that are covered for you                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                      | What you must pay when you get these services                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                           |
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| <ul style="list-style-type: none"> <li><input type="checkbox"/> Part-time or intermittent skilled nursing and home health aide services (To be covered under the home health care benefit, your skilled nursing and home health aide services combined must total fewer than 8 hours per day and 35 hours per week)</li> <li><input type="checkbox"/> Physical therapy, occupational therapy, and speech therapy</li> <li><input type="checkbox"/> Medical and social services</li> <li><input type="checkbox"/> Medical equipment and supplies</li> </ul>                                                                                                                                                                                                                                                                                                                                                                                                                             | <p>and related supplies for applicable copayments or coinsurance).</p>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                  |
| <p><b>Home infusion therapy</b></p> <p>Home infusion therapy involves the intravenous or subcutaneous administration of drugs or biologicals to an individual at home. The components needed to perform home infusion include the drug (for example, antivirals, immune globulin), equipment (for example, a pump), and supplies (for example, tubing and catheters).</p> <p>Covered services include, but are not limited to:</p> <ul style="list-style-type: none"> <li><input type="checkbox"/> Professional services, including nursing services, furnished in accordance with the plan of care</li> <li><input type="checkbox"/> Patient training and education not otherwise covered under the durable medical equipment benefit</li> <li><input type="checkbox"/> Remote monitoring</li> <li><input type="checkbox"/> Monitoring services for the provision of home infusion therapy and home infusion drugs furnished by a qualified home infusion therapy supplier</li> </ul> | <p>You will pay the cost-sharing that applies to primary care services, specialist physician services, or home health (as described under “Physician/practitioner services, including doctor’s office visits” or “Home health agency care”) depending on where you received administration or monitoring services.<sup>††</sup></p> <p>See "Durable medical equipment" earlier in this chart for any applicable cost-sharing for equipment and supplies related to home infusion therapy.<sup>††</sup></p> <p>See "Medicare Part B prescription drugs" later in this chart for any applicable cost-sharing for drugs related to home infusion therapy.<sup>††</sup></p> |

If you are a Qualified Medicare Beneficiary (QMB) or have full Medicaid benefits then your deductible, coinsurance and/or copayment may be less for services that are covered under Original Medicare.

| Services that are covered for you                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                      | What you must pay when you get these services                                                                                                                                                                                                  |
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| <p><b>Hospice care</b></p> <p>You are eligible for the hospice benefit when your doctor and the hospice medical director have given you a terminal prognosis certifying that you're terminally ill and have 6 months or less to live if your illness runs its normal course. You may receive care from any Medicare-certified hospice program. Your plan is obligated to help you find Medicare-certified hospice programs in the plan's service area, including those the MA organization owns, controls, or has a financial interest in. Your hospice doctor can be a network provider or an out-of-network provider.</p> <p>Covered services include:</p> <ul style="list-style-type: none"> <li><input type="checkbox"/> Drugs for symptom control and pain relief</li> <li><input type="checkbox"/> Short-term respite care</li> <li><input type="checkbox"/> Home care</li> </ul> <p>When you are admitted to a hospice you have the right to remain in your plan; if you chose to remain in your plan you must continue to pay plan premiums.</p> <p><b>For hospice services and for services that are covered by Medicare Part A or B and are related to your terminal prognosis:</b> Original Medicare (rather than our plan) will pay your hospice provider for your hospice services and any Part A and Part B services related to your terminal prognosis. While you are in the hospice program, your hospice provider will bill Original Medicare for the services that Original Medicare pays for. You will be billed Original Medicare cost-sharing.</p> <p><b>For services that are covered by Medicare Part A or B and are not related to your terminal prognosis:</b> If you need non-emergency, non-urgently needed services that are covered under Medicare Part A or B and that are not related to your terminal prognosis, your cost for these services depends on whether you use a provider in our plan's network and follow plan rules (such as if there is a requirement to obtain prior authorization):</p> | <p>When you enroll in a Medicare-certified hospice program, your hospice services and your Part A and Part B services related to your terminal prognosis are paid for by Original Medicare, not UHC Dual Complete NE-V001 (HMO-POS D-SNP).</p> |

If you are a Qualified Medicare Beneficiary (QMB) or have full Medicaid benefits then your deductible, coinsurance and/or copayment may be less for services that are covered under Original Medicare.

| Services that are covered for you                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                            | What you must pay when you get these services                                                                                                                                        |
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| <p><input type="checkbox"/> If you obtain the covered services from a network provider and follow plan rules for obtaining service, you only pay the plan cost-sharing amount for in-network services</p> <p><input type="checkbox"/> If you obtain the covered services from an out-of-network provider, you pay the cost-sharing under Fee-for-Service Medicare (Original Medicare)</p> <p><u>For services that are covered by UHC Dual Complete NE-V001 (HMO-POS D-SNP) but are not covered by Medicare Part A or B: UHC Dual Complete NE-V001 (HMO-POS D-SNP) will continue to cover plan-covered services that are not covered under Part A or B whether or not they are related to your terminal prognosis. You pay your plan cost-sharing amount for these services.</u></p> <p><u>For drugs that may be covered by the plan's Part D benefit:</u></p> <p>If these drugs are unrelated to your terminal hospice condition you pay cost-sharing. If they are related to your terminal hospice condition then you pay Original Medicare cost-sharing. Drugs are never covered by both hospice and our plan at the same time. For more information, please see Chapter 5, Section 9.4 (<b>What if you're in Medicare-certified hospice</b>).</p> <p><b>Note:</b> If you need non-hospice care (care that is not related to your terminal prognosis), you should contact us to arrange the services. Getting your non-hospice care through our network providers will lower your share of the costs for the services.</p> |                                                                                                                                                                                      |
| <p> <b>Immunizations</b></p> <p>Covered Medicare Part B services include:</p> <p><input type="checkbox"/> Pneumonia vaccine</p> <p><input type="checkbox"/> Flu vaccine, one each flu season in the fall and winter, with additional flu vaccine shots if medically necessary</p>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                         | <p>There is no coinsurance, copayment, or deductible for the pneumonia, flu, Hepatitis B, or COVID-19 vaccines.</p> <p>There is no coinsurance, copayment, or deductible for all</p> |

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| Services that are covered for you                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                 | What you must pay when you get these services                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                               |
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| <ul style="list-style-type: none"> <li><input type="checkbox"/> Hepatitis B vaccine if you are at high or intermediate risk of getting Hepatitis B</li> <li><input type="checkbox"/> COVID-19 vaccine</li> <li><input type="checkbox"/> Other vaccines if you are at risk and they meet Medicare Part B coverage rules</li> </ul> <p>We also cover some vaccines under our Part D prescription drug benefit, such as shingles or tetanus booster shots. See Chapter 6 for more information about coverage and applicable cost-sharing.</p>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                        | <p>other Medicare-covered immunizations.</p>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                |
| <p><b>Inpatient hospital care</b></p> <p>Includes inpatient acute, inpatient rehabilitation, long-term care hospitals, and other types of inpatient hospital services. Inpatient hospital care starts the day you are formally admitted to the hospital with a doctor's order. The day before you are discharged is your last inpatient day.</p> <p>Covered services include, but are not limited to:</p> <ul style="list-style-type: none"> <li><input type="checkbox"/> Semi-private room (or a private room if medically necessary)</li> <li><input type="checkbox"/> Meals including special diets</li> <li><input type="checkbox"/> Regular nursing services</li> <li><input type="checkbox"/> Costs of special care units (such as intensive care or coronary care units)</li> <li><input type="checkbox"/> Drugs and medications</li> <li><input type="checkbox"/> Lab tests</li> <li><input type="checkbox"/> X-rays and other radiology services</li> <li><input type="checkbox"/> Necessary surgical and medical supplies</li> <li><input type="checkbox"/> Use of appliances, such as wheelchairs</li> <li><input type="checkbox"/> Operating and recovery room costs</li> <li><input type="checkbox"/> Physical, occupational, and speech language therapy</li> </ul> | <p>\$375 copayment each day for days 1 to 6 for Medicare-covered hospital care each time you are admitted. \$0 copayment for additional Medicare-covered days.<sup>††</sup></p> <p>You pay these amounts until you reach the out-of-pocket maximum.</p> <p>If you get authorized inpatient care at an out-of-network hospital after your emergency condition is stabilized, your cost is the cost-sharing you would pay at a network hospital.</p> <p>Medicare hospital benefit periods do not apply. (See definition of benefit periods in the chapter titled Definitions of important words.) For inpatient hospital care, the cost-sharing described above applies each time you are admitted to the</p> |

If you are a Qualified Medicare Beneficiary (QMB) or have full Medicaid benefits then your deductible, coinsurance and/or copayment may be less for services that are covered under Original Medicare.

| Services that are covered for you                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                             | What you must pay when you get these services                                                                                                                                                                                                                                                                |
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| <p>Under certain conditions, the following types of transplants are covered: corneal, kidney, kidney-pancreatic, heart, liver, lung, heart/lung, bone marrow, stem cell, and intestinal/multivisceral. The plan has a network of facilities that perform organ transplants. The plan's hospital network for organ transplant services is different than the network shown in the 'Hospitals' section of your provider directory. Some hospitals in the plan's network for other medical services are not in the plan's network for transplant services. For information on network facilities for transplant services, please call UHC Dual Complete NE-V001 (HMO-POS D-SNP) Customer Service at 1-844-368-7149 TTY 711. If you need a transplant, we will arrange to have your case reviewed by a Medicare-approved transplant center that will decide whether you are a candidate for a transplant. Transplant providers may be local or outside of the service area. If our in-network transplant services are outside the community pattern of care, you may choose to go locally as long as the local transplant providers are willing to accept the Original Medicare rate. If UHC Dual Complete NE-V001 (HMO-POS D-SNP) provides transplant services at a location outside of the pattern of care for transplants in your community and you chose to obtain transplants at this distant location, we will arrange or pay for appropriate lodging and transportation costs for you and a companion. While you are receiving care at the distant location, we will also reimburse transportation costs to and from the hospital or doctor's office for evaluations, transplant services and follow-up care. (Transportation in the distant location includes, but is not limited to: vehicle mileage, economy/coach airfare, taxi fares, or rideshare services.) Costs for lodging or places to stay such as hotels, motels or short-term housing as a result of travel for a covered organ transplant may also be covered. You can be reimbursed for eligible costs up to \$125 per</p> | <p>hospital. A transfer to a separate facility type (such as an Inpatient Rehabilitation Hospital or Long Term Care Hospital) is considered a new admission. For each inpatient hospital stay, you are covered for unlimited days as long as the hospital stay is covered in accordance with plan rules.</p> |

If you are a Qualified Medicare Beneficiary (QMB) or have full Medicaid benefits then your deductible, coinsurance and/or copayment may be less for services that are covered under Original Medicare.



| Services that are covered for you                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                             | What you must pay when you get these services                                                                                                                                        |
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| <p>day total. Transportation services are not subject to the daily limit amount.</p> <ul style="list-style-type: none"> <li><input type="checkbox"/> Blood - including storage and administration. Coverage begins with the first pint of blood that you need.</li> <li><input type="checkbox"/> Physician services</li> </ul> <p><b>Note:</b> To be an inpatient, your provider must write an order to admit you formally as an inpatient of the hospital. Even if you stay in the hospital overnight, you might still be considered an “outpatient.” This is called an “outpatient observation” stay. If you are not sure if you are an inpatient or an outpatient, you should ask the hospital staff.</p> <p>You can also find more information in a Medicare fact sheet called “Are You a Hospital Inpatient or Outpatient? If You Have Medicare – Ask!” This fact sheet is available on the Web at <a href="https://www.medicare.gov/sites/default/files/2021-10/11435-Inpatient-or-Outpatient.pdf">medicare.gov/sites/default/files/2021-10/11435-Inpatient-or-Outpatient.pdf</a> or by calling 1-800-MEDICARE (1-800-633-4227). TTY users call 1-877-486-2048. You can call these numbers for free, 24 hours a day, 7 days a week.</p> | <p>Outpatient observation cost-sharing is explained in Outpatient surgery and other medical services provided at hospital outpatient facilities and ambulatory surgical centers.</p> |

If you are a Qualified Medicare Beneficiary (QMB) or have full Medicaid benefits then your deductible, coinsurance and/or copayment may be less for services that are covered under Original Medicare.



| Services that are covered for you                                                                                                                                                                                                                                                                                                                                                                                                                                                                          | What you must pay when you get these services                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                            |
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| <p><b>Inpatient services in a psychiatric hospital</b></p> <p>Covered services include:</p> <ul style="list-style-type: none"> <li><input type="checkbox"/> Mental health care services that require a hospital stay. There is a 190-day lifetime limit for inpatient services in a psychiatric hospital. The 190-day limit does not apply to Mental Health services provided in a psychiatric unit of a general hospital.</li> <li><input type="checkbox"/> Inpatient substance abuse services</li> </ul> | <p>\$375 copayment each day for days 1 to 6 for Medicare-covered hospital care each time you are admitted.</p> <p>\$0 copayment for additional Medicare-covered days, up to 90 days per benefit period. Plus an additional 60 lifetime reserve days.<sup>††</sup></p> <p>You pay these amounts until you reach the out-of-pocket maximum.</p> <p>Medicare hospital benefit periods are used to determine the total number of days covered for inpatient mental health care. (See definition of benefit periods in the chapter titled Definitions of important words.) However, the cost-sharing described above applies each time you are admitted to the hospital, even if you are admitted multiple times within a benefit period.</p> |
| <p><b>Inpatient stay: covered services received in a hospital or skilled nursing facility (SNF) during a non-covered inpatient stay</b></p> <p>If you have exhausted your inpatient benefits or if the inpatient stay is not reasonable and necessary, we will not cover your inpatient stay. However, in some cases, we will cover certain services you receive while you are in the hospital or the skilled nursing facility (SNF). Covered services include, but are not limited to:</p>                | <p>When your stay is no longer covered, these services will be covered as described in the following sections:</p>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                       |

If you are a Qualified Medicare Beneficiary (QMB) or have full Medicaid benefits then your deductible, coinsurance and/or copayment may be less for services that are covered under Original Medicare.

| Services that are covered for you                                                                                                                                                                                                                                                                                    | What you must pay when you get these services                                            |
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| <input type="checkbox"/> Physician services                                                                                                                                                                                                                                                                          | Please refer below to Physician/practitioner services, including doctor's office visits. |
| <input type="checkbox"/> Diagnostic tests (like lab tests)                                                                                                                                                                                                                                                           | Please refer below to Outpatient diagnostic tests and therapeutic services and supplies. |
| <input type="checkbox"/> X-ray, radium, and isotope therapy including technician materials and services                                                                                                                                                                                                              | Please refer below to Outpatient diagnostic tests and therapeutic services and supplies. |
| <input type="checkbox"/> Surgical dressings<br><input type="checkbox"/> Splints, casts and other devices used to reduce fractures and dislocations                                                                                                                                                                   | Please refer below to Outpatient diagnostic tests and therapeutic services and supplies. |
| <input type="checkbox"/> Prosthetics and orthotics devices (other than dental) that replace all or part of an internal body organ (including contiguous tissue), or all or part of the function of a permanently inoperative or malfunctioning internal body organ, including replacement or repairs of such devices | Please refer below to Prosthetic devices and related supplies.                           |
| <input type="checkbox"/> Leg, arm, back, and neck braces; trusses, and artificial legs, arms, and eyes including adjustments, repairs, and replacements required because of breakage, wear, loss, or a change in the patient's physical condition                                                                    | Please refer below to Prosthetic devices and related supplies.                           |
| <input type="checkbox"/> Physical therapy, speech language therapy, and occupational therapy                                                                                                                                                                                                                         | Please refer below to Outpatient rehabilitation services.                                |

If you are a Qualified Medicare Beneficiary (QMB) or have full Medicaid benefits then your deductible, coinsurance and/or copayment may be less for services that are covered under Original Medicare.

| Services that are covered for you                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                  | What you must pay when you get these services                                                                                                                           |
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| <p><b>Meal benefit</b></p> <p>This benefit can be used immediately following an inpatient hospital or skilled nursing facility (SNF) stay.</p> <p>Benefit guidelines:</p> <ul style="list-style-type: none"> <li><input type="checkbox"/> Receive up to 28 home-delivered meals for up to 14 days</li> <li><input type="checkbox"/> First meal delivery may take up to 72 hours after ordered</li> </ul>                                                                                                                                                                                                                                                                                                                                                                                                                           | <p>Provided by: Mom's Meals®</p> <p>\$0 copayment</p> <p>Prior authorization is required.</p> <p>Home-delivered meals are available nationwide through Mom's Meals.</p> |
| <p> <b>Medical nutrition therapy</b></p> <p>This benefit is for people with diabetes, renal (kidney) disease (but not on dialysis), or after a kidney transplant when ordered by your doctor.</p> <p>We cover three hours of one-on-one counseling services during your first year that you receive medical nutrition therapy services under Medicare (this includes our plan, any other Medicare Advantage plan, or Original Medicare), and two hours each year after that. If your condition, treatment, or diagnosis changes, you may be able to receive more hours of treatment with a physician's order. A physician must prescribe these services and renew their order yearly if your treatment is needed into the next calendar year.</p> | <p>There is no coinsurance, copayment, or deductible for members eligible for Medicare-covered medical nutrition therapy services.</p>                                  |
| <p> <b>Medicare diabetes prevention program (MDPP)</b></p> <p>MDPP services will be covered for eligible Medicare beneficiaries under all Medicare health plans.</p> <p>MDPP is a structured health behavior change intervention that provides practical training in long-term dietary change, increased physical activity, and problem-solving strategies for overcoming challenges to sustaining weight loss and a healthy lifestyle.</p>                                                                                                                                                                                                                                                                                                     | <p>There is no coinsurance, copayment, or deductible for the MDPP benefit.</p>                                                                                          |

If you are a Qualified Medicare Beneficiary (QMB) or have full Medicaid benefits then your deductible, coinsurance and/or copayment may be less for services that are covered under Original Medicare.

| Services that are covered for you                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                             | What you must pay when you get these services                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                       |
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| <p><b>Medicare Part B Prescription Drugs</b></p> <p>These drugs are covered under Part B of Original Medicare. Members of our plan receive coverage for these drugs through our plan. Covered drugs include:</p> <ul style="list-style-type: none"> <li><input type="checkbox"/> Drugs that usually aren't self-administered by the patient and are injected or infused while you are getting physician, hospital outpatient, or ambulatory surgical center services</li> <li><input type="checkbox"/> Insulin furnished through an item of durable medical equipment (such as a medically necessary insulin pump)</li> <li><input type="checkbox"/> Other drugs you take using durable medical equipment (such as nebulizers) that were authorized by the plan</li> <li><input type="checkbox"/> Clotting factors you give yourself by injection if you have hemophilia</li> <li><input type="checkbox"/> Immunosuppressive drugs, if you were enrolled in Medicare Part A at the time of the organ transplant</li> <li><input type="checkbox"/> Injectable osteoporosis drugs, if you are homebound, have a bone fracture that a doctor certifies was related to post-menopausal osteoporosis, and cannot self-administer the drug</li> <li><input type="checkbox"/> Antigens (for allergy shots)</li> <li><input type="checkbox"/> Certain oral anti-cancer drugs and anti-nausea drugs</li> <li><input type="checkbox"/> Certain drugs for home dialysis, including heparin, the antidote for heparin when medically necessary, topical anesthetics, and erythropoiesis-stimulating agents (such as Epogen®, Procrit®, Epoetin Alfa, Aranesp®, or Darbepoetin Alfa)</li> <li><input type="checkbox"/> Intravenous Immune Globulin for the home treatment of primary immune deficiency diseases</li> </ul> | <p>20% coinsurance for each Medicare-covered chemotherapy drug and the administration of that drug. You may pay less for certain rebatable drugs. This list and the cost of each rebatable drug changes every quarter.<sup>††</sup></p> <p>You pay these amounts until you reach the out-of-pocket maximum.</p> <p>20% coinsurance for each Medicare-covered Part B drug.<sup>††</sup> You may pay less for certain rebatable drugs. This list and the cost of each rebatable drug changes every quarter. For the administration of these drugs, you will pay the cost-sharing that applies to primary care provider services, specialist services, or outpatient hospital services (as described under “Physician/practitioner services, including doctor’s office visits” or “Outpatient hospital services” in this benefit chart) depending on where you received drug administration or infusion services.</p> <p>You will pay a maximum of \$35 for each 1-month supply of Part B covered insulin.</p> <p>You pay these amounts until you reach the out-of-pocket maximum.</p> |

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| Services that are covered for you                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                    | What you must pay when you get these services      |
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| <p><input type="checkbox"/> Chemotherapy Drugs, and the administration of chemotherapy drugs</p> <p>The following link will take you to a list of Part B Drugs that may be subject to Step Therapy: <a href="https://www.medicare.uhc.com/medicare/member/documents/part-b-step-therapy.html">https://www.medicare.uhc.com/medicare/member/documents/part-b-step-therapy.html</a></p> <p>You or your doctor may need to provide more information about how a Medicare Part B prescription drug is used in order to determine coverage. There may be effective, lower-cost drugs that treat the same medical condition. If you are prescribed a new Part B medication or have not recently filled the medication under Part B, you may be required to try one or more of these other drugs before the plan will cover your drug. If you have already tried other drugs or your doctor thinks they are not right for you, you or your doctor can ask the plan to cover the Part B drug. (For more information, see Chapter 9, What to do if you have a problem or complaint (coverage decisions, appeals, complaints).) Please contact Customer Service for more information.</p> <p>We also cover some vaccines under our Part B and Part D prescription drug benefit.</p> <p>Chapter 5 explains the Part D prescription drug benefit, including rules you must follow to have prescriptions covered. What you pay for your Part D prescription drugs through our plan is explained in Chapter 6.</p> |                                                    |
| <p><b>Nurse Hotline</b></p> <p>Nurse Hotline services available, 24 hours a day, 7 days a week. Speak to a registered nurse (RN) about your medical concerns and questions.</p> <p>You can view the Vendor Information Sheet at <a href="https://myuhc.com/communityplan">myuhc.com/communityplan</a>, or call Customer Service to have a paper copy sent to you.</p>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                | <p>Provided by: NurseLine</p> <p>\$0 copayment</p> |

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| Services that are covered for you                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                | What you must pay when you get these services                                                          |
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| <p> <b>Obesity screening and therapy to promote sustained weight loss</b></p> <p>If you have a body mass index of 30 or more, we cover intensive counseling to help you lose weight. This counseling is covered if you get it in a primary care setting, where it can be coordinated with your comprehensive prevention plan. Talk to your primary care doctor or practitioner to find out more.</p>                                                                                                                                                                                                                                                                                                                                                            | <p>There is no coinsurance, copayment, or deductible for preventive obesity screening and therapy.</p> |
| <p><b>Opioid treatment program services</b></p> <p>Members of our plan with opioid use disorder (OUD) can receive coverage of services to treat OUD through an Opioid Treatment Program (OTP) which includes the following services:</p> <ul style="list-style-type: none"> <li><input type="checkbox"/> U.S. Food and Drug Administration (FDA)-approved opioid agonist and antagonist medication-assisted treatment (MAT) medications.</li> <li><input type="checkbox"/> Dispensing and administration of MAT medications (if applicable)</li> <li><input type="checkbox"/> Substance use counseling</li> <li><input type="checkbox"/> Individual and group therapy</li> <li><input type="checkbox"/> Toxicology testing</li> <li><input type="checkbox"/> Intake activities</li> <li><input type="checkbox"/> Periodic assessments</li> </ul> | <p>\$0 copayment for Medicare-covered opioid treatment program services.<sup>††</sup></p>              |
| <p><b>Outpatient diagnostic tests and therapeutic services and supplies</b></p> <p>Covered services include, but are not limited to:</p>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                         |                                                                                                        |

If you are a Qualified Medicare Beneficiary (QMB) or have full Medicaid benefits then your deductible, coinsurance and/or copayment may be less for services that are covered under Original Medicare.

| Services that are covered for you                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                    | What you must pay when you get these services                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                           |
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| <p><input type="checkbox"/> X-rays</p> <p><input type="checkbox"/> Radiation (radium and isotope) therapy including technician materials and supplies</p> <p><input type="checkbox"/> Surgical supplies, such as dressings</p> <p><input type="checkbox"/> Splints, casts, and other devices used to reduce fractures and dislocations</p> <p>Note: There is no separate charge for medical supplies routinely used in the course of an office visit and included in the provider's charges for that visit (such as bandages, cotton swabs, and other routine supplies.) However, supplies for which an appropriate separate charge is made by providers (such as, chemical agents used in certain diagnostic procedures) are subject to cost-sharing as shown.</p> <p><input type="checkbox"/> Laboratory tests</p> | <p>\$25 copayment for each Medicare-covered standard X-ray service.<sup>††</sup></p> <p>You pay these amounts until you reach the out-of-pocket maximum.</p> <p>\$60 copayment for each Medicare-covered radiation therapy service.<sup>††</sup></p> <p>You pay these amounts until you reach the out-of-pocket maximum.</p> <p>20% coinsurance for each Medicare-covered medical supply.<sup>††</sup></p> <p>You pay these amounts until you reach the out-of-pocket maximum.</p> <p>\$0 copayment for Medicare-covered lab services.<sup>††</sup></p> |

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| Services that are covered for you                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                     | What you must pay when you get these services                                                                                                                                                                                                                                                                                                                                              |
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| <ul style="list-style-type: none"> <li><input type="checkbox"/> Blood - including storage and administration (this means processing and handling of blood). Coverage begins with the first pint of blood that you need.</li> <li><input type="checkbox"/> In addition, for the administration of blood infusion, you will pay the cost-sharing as described under the following sections of this chart, depending on where you received infusion services:             <ul style="list-style-type: none"> <li><input type="checkbox"/> Physician/practitioner services, including doctor's office visits</li> <li><input type="checkbox"/> Outpatient surgery and other medical services provided at hospital outpatient facilities and ambulatory surgical centers</li> </ul> </li> <li><input type="checkbox"/> Other outpatient diagnostic tests - non-radiological diagnostic services</li> </ul> | <p>\$0 copayment for Medicare-covered blood services.<sup>††</sup></p> <p>\$20 copayment for Medicare-covered non-radiological diagnostic services.<sup>††</sup></p> <p>Examples include, but are not limited to EKG's, pulmonary function tests, home or lab-based sleep studies, and treadmill stress tests.</p> <p>You pay these amounts until you reach the out-of-pocket maximum.</p> |

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| Services that are covered for you                                                                    | What you must pay when you get these services                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                         |
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| <p>☐ Other outpatient diagnostic tests - radiological diagnostic services, not including x-rays.</p> | <p>\$0 copayment for each diagnostic mammogram.<sup>††</sup></p> <p>\$0 copayment for each vascular screening by a doctor in your home or a nursing home in which you reside.<sup>††</sup></p> <p>\$250 copayment for other Medicare-covered radiological diagnostic services, not including X-rays, performed in a physician's office or at a free-standing facility (such as a radiology center or medical clinic).<sup>††</sup></p> <p>You pay these amounts until you reach the out-of-pocket maximum.</p> <p>The diagnostic radiology services require specialized equipment beyond standard X-ray equipment and must be performed by specially trained or certified personnel. Examples include, but are not limited to, specialized scans, CT, SPECT, PET, MRI, MRA, nuclear studies, ultrasounds, diagnostic mammograms and interventional radiological procedures (myelogram, cystogram, angiogram, and barium studies).</p> |

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| Services that are covered for you                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                      | What you must pay when you get these services                                                                                                                                        |
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| <p><b>Outpatient hospital observation</b><br/>                     Observation services are hospital outpatient services given to determine if you need to be admitted as an inpatient or can be discharged. For outpatient hospital observation services to be covered, they must meet the Medicare criteria and be considered reasonable and necessary. Observation services are covered only when provided by the order of a physician or another individual authorized by state licensure law and hospital staff bylaws to admit patients to the hospital or order outpatient tests.</p> <p><b>Note:</b> Unless the provider has written an order to admit you as an inpatient to the hospital, you are an outpatient and pay the cost-sharing amounts for outpatient hospital services. Even if you stay in the hospital overnight, you might still be considered an “outpatient.” If you are not sure if you are an outpatient, you should ask the hospital staff. You can also find more information in a Medicare fact sheet called “Are You a Hospital Inpatient or Outpatient? If You Have Medicare – Ask!” This fact sheet is available on the Web at <a href="https://www.medicare.gov/sites/default/files/2021-10/11435-Inpatient-or-Outpatient.pdf">medicare.gov/sites/default/files/2021-10/11435-Inpatient-or-Outpatient.pdf</a> or by calling 1-800-MEDICARE (1-800-633-4227). TTY users call 1-877-486-2048. You can call these numbers for free, 24 hours a day, 7 days a week.</p> | <p>Outpatient observation cost-sharing is explained in Outpatient surgery and other medical services provided at hospital outpatient facilities and ambulatory surgical centers.</p> |
| <p><b>Outpatient hospital services</b><br/>                     We cover medically-necessary services you get in the outpatient department of a hospital for diagnosis or treatment of an illness or injury.<br/>                     Covered services include, but are not limited to:</p> <p><input type="checkbox"/> Services in an emergency department</p>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                        | <p>Please refer to Emergency Care.</p>                                                                                                                                               |

If you are a Qualified Medicare Beneficiary (QMB) or have full Medicaid benefits then your deductible, coinsurance and/or copayment may be less for services that are covered under Original Medicare.

| Services that are covered for you                                                                                                                                                                                                                                                                                                                                                          | What you must pay when you get these services                                      |
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| <input type="checkbox"/> Laboratory and diagnostic tests billed by the hospital                                                                                                                                                                                                                                                                                                            | Please refer to Outpatient diagnostic tests and therapeutic services and supplies. |
| <input type="checkbox"/> Mental health care, including care in a partial-hospitalization program, if a doctor certifies that inpatient treatment would be required without it                                                                                                                                                                                                              | Please refer to Outpatient mental health care.                                     |
| <input type="checkbox"/> X-rays and other radiology services billed by the hospital                                                                                                                                                                                                                                                                                                        | Please refer to Outpatient diagnostic tests and therapeutic services and supplies. |
| <input type="checkbox"/> Medical supplies such as splints and casts                                                                                                                                                                                                                                                                                                                        | Please refer to Outpatient diagnostic tests and therapeutic services and supplies. |
| <input type="checkbox"/> Certain screenings and preventive services                                                                                                                                                                                                                                                                                                                        | Please refer to the benefits preceded by the “Apple” icon.                         |
| <input type="checkbox"/> Certain drugs and biologicals that you can’t give yourself (Note: Self-administered drugs in an outpatient hospital are not usually covered under your Part B prescription drug benefit. Under certain circumstances, they may be covered under your Part D prescription drug benefit. For more information on Part D payment requests, see Chapter 7 Section 2.) | Please refer to Medicare Part B prescription drugs.                                |
| <input type="checkbox"/> Services performed at an outpatient clinic                                                                                                                                                                                                                                                                                                                        | Please refer to Physician/practitioner services, including doctor's office visits. |
| <input type="checkbox"/> Outpatient surgery or observation                                                                                                                                                                                                                                                                                                                                 | Please refer to Outpatient surgery and other medical                               |

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| Services that are covered for you                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                     | What you must pay when you get these services                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                 |
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| <p><input type="checkbox"/> Outpatient infusion therapy</p> <p>For the drug that is infused, you will pay the cost-sharing as described in "Medicare Part B prescription drugs" in this benefit chart. In addition, for the administration of infusion therapy drugs, you will pay the cost-sharing that applies to primary care provider services, specialist services, or outpatient hospital services (as described under "Physician/practitioner services, including doctor's office visits" or "Outpatient surgery and other medical services provided at hospital outpatient facilities and ambulatory surgical centers" in this benefit chart) depending on where you received drug administration or infusion services.</p> <p><b>Note:</b> Unless the provider has written an order to admit you as an inpatient to the hospital, you are an outpatient and pay the cost-sharing amounts for outpatient hospital services. Even if you stay in the hospital overnight, you might still be considered an "outpatient." This is called an "outpatient observation" stay. If you are not sure if you are an outpatient, you should ask the hospital staff.</p> <p>You can also find more information in a Medicare fact sheet called "Are You a Hospital Inpatient or Outpatient? If You Have Medicare – Ask!" This fact sheet is available on the Web at <a href="https://www.medicare.gov/sites/default/files/2021-10/11435-Inpatient-or-Outpatient.pdf">medicare.gov/sites/default/files/2021-10/11435-Inpatient-or-Outpatient.pdf</a> or by calling 1-800-MEDICARE (1-800-633-4227). TTY users call 1-877-486-2048. You can call these numbers for free, 24 hours a day, 7 days a week.</p> | <p>services provided at hospital outpatient facilities and ambulatory surgical centers.</p> <p>Please refer to Medicare Part B prescription drugs and Physician/practitioner services, including doctor's office visits or Outpatient surgery and other medical services provided at hospital outpatient facilities and ambulatory surgical centers.</p> <p>Outpatient observation cost-sharing is explained in Outpatient surgery and other medical services provided at hospital outpatient facilities and ambulatory surgical centers.</p> |

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| Services that are covered for you                                                                                                                                                                                                                                                                                                                                                                                                                                                      | What you must pay when you get these services                                                                                                                                                                                                                                                                                                             |
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| <p><b>Outpatient mental health care</b></p> <p>Covered services include:</p> <p>Mental health services provided by a state-licensed psychiatrist or doctor, clinical psychologist, clinical social worker, clinical nurse specialist, licensed professional counselor (LPC), licensed marriage and family therapist (LMFT), nurse practitioner (NP), physician assistant (PA), or other Medicare-qualified mental health care professional as allowed under applicable state laws.</p> | <p>\$25 copayment for each Medicare-covered <b>individual</b> therapy session.<sup>††</sup></p> <p>\$15 copayment for each Medicare-covered <b>group</b> therapy session.<sup>††</sup></p> <p>You pay these amounts until you reach the out-of-pocket maximum.</p>                                                                                        |
| <p><b>Outpatient rehabilitation services</b></p> <p>Covered services include: physical therapy, occupational therapy, and speech language therapy.</p> <p>Outpatient rehabilitation services are provided in various outpatient settings, such as hospital outpatient departments, independent therapist offices, physician offices, and Comprehensive Outpatient Rehabilitation Facilities (CORFs).</p>                                                                               | <p>\$35 copayment for each Medicare-covered physical therapy and speech-language therapy visit.<sup>††</sup></p> <p>You pay these amounts until you reach the out-of-pocket maximum.</p> <p>\$35 copayment for each Medicare-covered occupational therapy visit.<sup>††</sup></p> <p>You pay these amounts until you reach the out-of-pocket maximum.</p> |
| <p><b>Outpatient substance abuse services</b></p> <p>Outpatient treatment and counseling for substance abuse.</p>                                                                                                                                                                                                                                                                                                                                                                      | <p>\$25 copayment for each Medicare-covered <b>individual</b> therapy session.<sup>††</sup></p> <p>\$15 copayment for each Medicare-covered <b>group</b> therapy session.<sup>††</sup></p> <p>You pay these amounts until you reach the out-of-pocket maximum.</p>                                                                                        |

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| Services that are covered for you                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                       | What you must pay when you get these services                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                         |
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| <p><b>Outpatient surgery and other medical services provided at hospital outpatient facilities and ambulatory surgical centers</b></p> <p><b>Note:</b> If you are having surgery in a hospital facility, you should check with your provider about whether you will be an inpatient or outpatient. Unless the provider writes an order to admit you as an inpatient to the hospital, you are an outpatient and pay the cost-sharing amounts for outpatient surgery. Even if you stay in the hospital overnight, you might still be considered an “outpatient.” This is called an “Outpatient Observation” stay. If you are not sure if you are an outpatient, you should ask your doctor or the hospital staff.</p> <p>If you receive any services or items other than surgery, including but not limited to diagnostic tests, therapeutic services, prosthetics, orthotics, supplies or Part B drugs, there may be additional cost-sharing for those services or items. Please refer to the appropriate section in this chart for the additional service or item you received for the specific cost-sharing required.</p> <p>See “Colorectal cancer screening” earlier in this chart for screening and diagnostic colonoscopy benefit information.</p> | <p>\$0 copayment for a colonoscopy at an ambulatory surgical center.<sup>††</sup></p> <p>\$325 copayment for Medicare-covered surgery or other services provided to you at an ambulatory surgical center, including but not limited to hospital or other facility charges and physician or surgical charges.<sup>††</sup></p> <p>You pay these amounts until you reach the out-of-pocket maximum.</p> <p>\$0 copayment for a colonoscopy at an outpatient hospital.<sup>††</sup></p> <p>\$375 copayment for Medicare-covered surgery or other services provided to you at an outpatient hospital, including but not limited to hospital or other facility charges and physician or surgical charges.<sup>††</sup></p> <p>You pay these amounts until you reach the out-of-pocket maximum.</p> <p>Outpatient surgical services that can be delivered in an available ambulatory surgery center must be delivered in an ambulatory surgery center unless a hospital</p> |

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| Services that are covered for you                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                               | What you must pay when you get these services                                                                                                                                                                                                                                                                                                                        |
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|                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                 | <p>outpatient department is medically necessary.</p> <p>\$375 copayment for each day of Medicare-covered observation services provided to you at an outpatient hospital, including but not limited to hospital or other facility charges and physician or surgical charges.<sup>††</sup></p> <p>You pay these amounts until you reach the out-of-pocket maximum.</p> |
| <p><b>Food, over-the-counter (OTC) and utility bill credit</b></p> <p>With this benefit, you'll get a credit loaded to your UnitedHealthcare UCard® each month to pay for covered healthy food, OTC items and utility bills. Unused credits expire at the end of each month.</p> <p>Covered items include:</p> <ul style="list-style-type: none"><li>☐ Healthy foods like fruits, vegetables, meat, seafood, dairy products, water and more.</li><li>☐ Brand name and generic OTC products, like vitamins, pain relievers, toothpaste, first aid products and more.</li><li>☐ Eligible utility bills like electricity, home heat like natural gas, water and home internet. The service address must match an address we have on file for you.</li></ul> <p>The credit cannot be used to buy tobacco or alcohol.</p> <p><b>Home and bath safety devices</b></p> | <p>Provided by: Solutran</p> <p>Monthly credit is \$62</p> <p>Combined with food, OTC and utility bill credit amount</p>                                                                                                                                                                                                                                             |

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| Services that are covered for you                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                      | What you must pay when you get these services                                                                                                      |
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| <p>You can also use your OTC credit on covered home and bath safety devices like bathmats, grab bars and shower chairs.</p> <p>You can use your credit at thousands of participating stores or place an order online. Home shipping is free and there is a \$35 minimum to place an order. You can also use your credit to pay eligible utility bills from network companies online or at your local Walmart MoneyCenter or Customer Service Desk.</p> <p>Visit the UCard Hub at <a href="https://myuhc.com/communityplan">myuhc.com/communityplan</a> to find participating stores, check your balance, place an order online or pay utility bills.</p>                                                                                                                                               | <p>Home shipped food, OTC products and home and bath safety devices are available nationwide.</p>                                                  |
| <p><b>Partial hospitalization services and Intensive outpatient services</b></p> <p>“Partial hospitalization” is a structured program of active psychiatric treatment provided as a hospital outpatient service, or by a community mental health center, that is more intense than the care received in your doctor’s or therapist’s office and is an alternative to inpatient hospitalization.</p> <p>Intensive outpatient service is a structured program of active behavioral (mental) health therapy treatment provided in a hospital outpatient department, a community mental health center, a Federally qualified health center, or a rural health clinic that is more intense than the care received in your doctor’s or therapist’s office but less intense than partial hospitalization.</p> | <p>\$55 copayment each day for Medicare-covered benefits.<sup>††</sup></p> <p>You pay these amounts until you reach the out-of-pocket maximum.</p> |
| <p><b>Personal emergency response system</b></p> <p>With a personal emergency response system (PERS), help is only a button press away. A PERS device can quickly connect you to the help you need, 24 hours a day in any</p>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                          | <p>Provided by: Lifeline</p> <p>\$0 copayment</p>                                                                                                  |

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| Services that are covered for you                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                         | What you must pay when you get these services                                                                                                                                                                                                                                                                                                                                                                                                                 |
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| <p>situation. It's a lightweight, discreet button that can be worn on your wrist or as a pendant. It's also safe to wear in the shower or bath. Depending on the model you choose, it may even automatically detect falls.</p> <p>You must have a working landline or live in an area that has appropriate wireless coverage to get a PERS device. The cellular device works nationwide with the wireless network but does not require you to have a contract with the network.</p> <p>You can get more information by viewing the Vendor Information Sheet at <a href="http://myuhc.com/communityplan">myuhc.com/communityplan</a>, or by calling Customer Service to have a paper copy sent to you.</p> | <p>A home-delivered device is available nationwide through Lifeline.</p>                                                                                                                                                                                                                                                                                                                                                                                      |
| <p><b>Physician/practitioner services, including doctor's office visits</b></p> <p>Covered services include:</p> <ul style="list-style-type: none"> <li><input type="checkbox"/> Medically-necessary medical or surgical services furnished in a physician's office.</li> <li><input type="checkbox"/> Medically-necessary medical or surgical services furnished in a certified ambulatory surgical center or hospital outpatient department.</li> </ul>                                                                                                                                                                                                                                                 | <p>\$0 copayment for services from a primary care physician or under certain circumstances, treatment by a nurse practitioner, physician's assistant or other non-physician health care professional in a primary care physician's office (as allowed by Medicare).</p> <p>See "Outpatient surgery" earlier in this chart for any applicable copayments or coinsurance amounts for ambulatory surgical center visits or in a hospital outpatient setting.</p> |

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| Services that are covered for you                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                              | What you must pay when you get these services                                                                                                                                                                                                                                                                                         |
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| <input type="checkbox"/> Consultation, diagnosis, and treatment by a specialist.                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                               | <p>\$35 copayment for services from a specialist or under certain circumstances, treatment by a nurse practitioner, physician's assistant or other non-physician health care professional in a specialist's office (as allowed by Medicare).<sup>††</sup></p> <p>You pay these amounts until you reach the out-of-pocket maximum.</p> |
| <input type="checkbox"/> Basic hearing and balance exams performed by your specialist, if your doctor orders it to see if you need medical treatment.                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                          | <p>\$0 copayment for each Medicare-covered exam.<sup>††</sup></p>                                                                                                                                                                                                                                                                     |
| <input type="checkbox"/> Medicare-covered telehealth services including: <ul style="list-style-type: none"> <li><input type="checkbox"/> Medical and mental health visits delivered to you outside of medical facilities by providers that have appropriate online technology and live audio/video capabilities to conduct the visit.</li> <li><input type="checkbox"/> Not all medical conditions can be treated through virtual visits. The virtual visit doctor will identify if you need to see an in-person doctor for treatment.</li> <li><input type="checkbox"/> Telehealth services for monthly end-stage renal disease-related visits for home dialysis members in a hospital-based or critical access hospital-based renal dialysis center, renal dialysis facility, or the member's home.</li> <li><input type="checkbox"/> Telehealth services provided by rural health clinics and federally qualified health centers.</li> <li><input type="checkbox"/> Medicare-covered remote monitoring services.</li> </ul> | <p>\$0 copayment for each Medicare-covered visit.<sup>††</sup></p>                                                                                                                                                                                                                                                                    |

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| Services that are covered for you                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                            | What you must pay when you get these services                                                                                                                                                                                                                                                                                         |
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| <ul style="list-style-type: none"> <li><input type="checkbox"/> Virtual check-ins (for example, by phone or video chat) with your doctor for 5-10 minutes if: <ul style="list-style-type: none"> <li><input type="checkbox"/> You're not a new patient and</li> <li><input type="checkbox"/> The check-in isn't related to an office visit in the past 7 days and</li> <li><input type="checkbox"/> The check-in doesn't lead to an office visit within 24 hours or the soonest available appointment.</li> </ul> </li> <li><input type="checkbox"/> Evaluation of video and/or images you send to your doctor, and interpretation and follow-up by your doctor within 24 hours if: <ul style="list-style-type: none"> <li><input type="checkbox"/> You're not a new patient and</li> <li><input type="checkbox"/> The evaluation isn't related to an office visit in the past 7 days and</li> <li><input type="checkbox"/> The evaluation doesn't lead to an office visit within 24 hours or the soonest available appointment.</li> </ul> </li> <li><input type="checkbox"/> Consultation your doctor has with other doctors by phone, internet, or electronic health record.</li> <br/> <li><input type="checkbox"/> Second opinion by another network provider prior to surgery.</li> <br/> <li><input type="checkbox"/> Non-routine dental care (covered services are limited to surgery of the jaw or related structures, setting fractures of the jaw or facial bones, extraction of teeth to prepare the jaw for radiation treatments of neoplastic cancer disease, oral exams before a kidney transplant or services that would be covered when provided by a physician). Dental services provided by a dentist in connection with care, treatment, filling, removal, or</li> </ul> | <p>You will pay the cost-sharing that applies to specialist services (as described under “Physician/practitioner services, including doctor’s office visits” above).<sup>††</sup></p><br><p>20% coinsurance for each Medicare-covered visit.<sup>††</sup></p> <p>You pay these amounts until you reach the out-of-pocket maximum.</p> |

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| Services that are covered for you                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                       | What you must pay when you get these services                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                         |
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| <p>replacement of teeth or structures directly supporting the teeth are not Medicare-covered benefits and not covered under this benefit.</p> <p><input type="checkbox"/> Monitoring services in a physician’s office or outpatient hospital setting if you are taking anticoagulation medications, such as Coumadin, Heparin or Warfarin (these services may also be referred to as ‘Coumadin Clinic’ services).</p> <p><input type="checkbox"/> Medically-necessary medical or surgical services that are covered benefits and are furnished by a physician/ non-physician health care professional in your home.</p> | <p>You will pay the cost-sharing that applies to primary care provider services, specialist services, or outpatient hospital services (as described under “Physician/practitioner services, including doctor’s office visits” or “Outpatient hospital services” in this benefit chart) depending on where you receive services.<sup>††</sup></p> <p>\$0 copayment for primary care provider services or, in certain circumstances, nurse practitioner, physician’s assistant or other non-physician health care professional services.<sup>††</sup></p> <p>\$35 copayment for specialist physician services.<sup>††</sup></p> <p>You pay these amounts until you reach the out-of-pocket maximum.</p> |
| <p><b>Podiatry services</b></p> <p>Covered services include:</p> <p><input type="checkbox"/> Diagnosis and the medical or surgical treatment of injuries and diseases of the feet (such as hammer toe or heel spurs).</p>                                                                                                                                                                                                                                                                                                                                                                                               | <p>\$35 copayment for each Medicare-covered visit in an office or home setting.<sup>††</sup></p> <p>For services rendered in an outpatient hospital setting, such as surgery, please refer to</p>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                     |

If you are a Qualified Medicare Beneficiary (QMB) or have full Medicaid benefits then your deductible, coinsurance and/or copayment may be less for services that are covered under Original Medicare.

| Services that are covered for you                                                                                                                                                                                                                                                                                                                          | What you must pay when you get these services                                                                                                                                                                                                                                                                                                        |
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| <p><input type="checkbox"/> Routine foot care for members with certain medical conditions affecting the lower limbs.</p>                                                                                                                                                                                                                                   | <p>Outpatient surgery and other medical services provided at hospital outpatient facilities and ambulatory surgical centers.</p> <p>You pay these amounts until you reach the out-of-pocket maximum.</p>                                                                                                                                             |
| <p><b>Additional routine foot care</b></p> <p>We cover 6 routine foot care visits every year. This benefit is in addition to the Medicare-covered podiatry services benefit listed above.</p> <p>Covered services include treatment of the foot which is generally considered preventive, i.e., cutting or removal of corns, warts, calluses or nails.</p> | <p>\$35 copayment for each routine visit.*</p>                                                                                                                                                                                                                                                                                                       |
| <p> <b>Prostate cancer screening exams</b></p> <p>For men age 50 and older, covered services include the following - once every 12 months:</p> <p><input type="checkbox"/> Digital rectal exam</p> <p><input type="checkbox"/> Prostate Specific Antigen (PSA) test</p> | <p>There is no coinsurance, copayment, or deductible for each Medicare-covered digital rectal exam.</p> <p>There is no coinsurance, copayment, or deductible for an annual PSA test.</p> <p>Diagnostic PSA exams are subject to cost-sharing as described under Outpatient diagnostic tests and therapeutic services and supplies in this chart.</p> |
| <p><b>Prosthetic devices and related supplies</b></p> <p>Devices (other than dental) that replace all or part of a body part or function. These include, but are not limited to: colostomy bags and supplies directly related to colostomy</p>                                                                                                             | <p>20% coinsurance for each Medicare-covered prosthetic or orthotic device, including</p>                                                                                                                                                                                                                                                            |

If you are a Qualified Medicare Beneficiary (QMB) or have full Medicaid benefits then your deductible, coinsurance and/or copayment may be less for services that are covered under Original Medicare.



| Services that are covered for you                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                 | What you must pay when you get these services                                                                                                                        |
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| <p>care, pacemakers, braces, prosthetic shoes, artificial limbs, and breast prostheses (including a surgical brassiere after a mastectomy). Includes certain supplies related to prosthetic devices, and repair and/or replacement of prosthetic devices. Also includes some coverage following cataract removal or cataract surgery – see “Vision services” later in this section for more detail.</p>                                                                                                                                                                           | <p>replacement or repairs of such devices, and related supplies.<sup>††</sup></p> <p>You pay these amounts until you reach the out-of-pocket maximum.</p>            |
| <p><b>Pulmonary rehabilitation services</b></p> <p>Comprehensive programs of pulmonary rehabilitation are covered for members who have moderate to very severe chronic obstructive pulmonary disease (COPD) and an order for pulmonary rehabilitation from the doctor treating the chronic respiratory disease. Medicare covers up to two (2) one-hour sessions per day, for up to 36 lifetime sessions (in some cases, up to 72 lifetime sessions) of pulmonary rehabilitation services.</p>                                                                                     | <p>\$15 copayment for each Medicare-covered pulmonary rehabilitative visit.<sup>††</sup></p> <p>You pay these amounts until you reach the out-of-pocket maximum.</p> |
| <p> <b>Screening and counseling to reduce alcohol misuse</b></p> <p>We cover one alcohol misuse screening per year for adults with Medicare (including pregnant women) who misuse alcohol, but aren’t alcohol dependent.</p> <p>If you screen positive for alcohol misuse, you can get up to four brief face-to-face counseling sessions per year (if you’re competent and alert during counseling) provided by a qualified primary care doctor or practitioner in a primary care setting.</p> | <p>There is no coinsurance, copayment, or deductible for the Medicare-covered screening and counseling to reduce alcohol misuse preventive benefit.</p>              |
| <p> <b>Screening for lung cancer with low dose computed tomography (LDCT)</b></p> <p>For qualified individuals, a LDCT is covered every 12 months.</p>                                                                                                                                                                                                                                                                                                                                         | <p>There is no coinsurance, copayment, or deductible for the Medicare-covered counseling and shared decision making visit or for the LDCT.</p>                       |

If you are a Qualified Medicare Beneficiary (QMB) or have full Medicaid benefits then your deductible, coinsurance and/or copayment may be less for services that are covered under Original Medicare.

| Services that are covered for you                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                               | What you must pay when you get these services                                                                                                    |
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| <p><b>Eligible members are:</b> people aged 50 – 77 years who have no signs or symptoms of lung cancer, but who have a history of tobacco smoking of at least 20 pack-years and who currently smoke or have quit smoking within the last 15 years, who receive a written order for LDCT during a lung cancer screening counseling and shared decision making visit that meets the Medicare criteria for such visits and be furnished by a physician or qualified non-physician practitioner.</p> <p><b>For LDCT lung cancer screenings after the initial LDCT screening:</b> the member must receive a written order for LDCT lung cancer screening, which may be furnished during any appropriate visit with a physician or qualified non-physician practitioner. If a physician or qualified non-physician practitioner elects to provide a lung cancer screening counseling and shared decision-making visit for subsequent lung cancer screenings with LDCT, the visit must meet the Medicare criteria for such visits.</p> |                                                                                                                                                  |
| <p> <b>Screening for sexually transmitted infections (STIs) and counseling to prevent STIs</b></p> <p>We cover sexually transmitted infection (STI) screenings for chlamydia, gonorrhea, syphilis, and Hepatitis B. These screenings are covered for pregnant women and for certain people who are at increased risk for an STI when the tests are ordered by a primary care provider. We cover these tests once every 12 months or at certain times during pregnancy. We also cover up to two individual 20 to 30 minute, face-to-face high-intensity behavioral counseling sessions each year for sexually active adults at increased risk for STIs. We will only cover these counseling sessions as a preventive service if they are provided by a primary care provider and take place in a primary care setting, such as a doctor's office.</p>                                                                                         | <p>There is no coinsurance, copayment, or deductible for the Medicare-covered screening for STIs and counseling for STIs preventive benefit.</p> |
| <p><b>Services to treat kidney disease</b></p>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                  |                                                                                                                                                  |

If you are a Qualified Medicare Beneficiary (QMB) or have full Medicaid benefits then your deductible, coinsurance and/or copayment may be less for services that are covered under Original Medicare.

| Services that are covered for you                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                        | What you must pay when you get these services                                                                                                                                                                                                                                                                                                                                                                                                                                                           |
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| <p>Covered services include:</p> <ul style="list-style-type: none"> <li><input type="checkbox"/> Kidney disease education services to teach kidney care and help members make informed decisions about their care. For members with stage IV chronic kidney disease when referred by their doctor, we cover up to six sessions of kidney disease education services per lifetime.</li> <li><input type="checkbox"/> Outpatient dialysis treatments (including dialysis treatments when temporarily out of the service area, as explained in Chapter 3, or when your provider for this service is temporarily unavailable or inaccessible)</li> <li><input type="checkbox"/> Self-dialysis training (includes training for you and anyone helping you with your home dialysis treatments)</li> <li><input type="checkbox"/> Inpatient dialysis treatments (if you are admitted as an inpatient to a hospital for special care)</li> <li><input type="checkbox"/> Home dialysis equipment and supplies</li> <li><input type="checkbox"/> Certain home support services (such as, when necessary, visits by trained dialysis workers to check on your home dialysis, to help in emergencies, and check your dialysis equipment and water supply)</li> </ul> | <p>\$0 copayment for Medicare-covered benefits.</p> <p>20% coinsurance for Medicare-covered benefits.<sup>††</sup></p> <p>You pay these amounts until you reach the out-of-pocket maximum.</p> <p>\$0 copayment for Medicare-covered benefits.</p> <p>These services will be covered as described in the following sections:</p> <p>Please refer to Inpatient hospital care.</p> <p>Please refer to Durable medical equipment and related supplies.</p> <p>Please refer to Home health agency care.</p> |

If you are a Qualified Medicare Beneficiary (QMB) or have full Medicaid benefits then your deductible, coinsurance and/or copayment may be less for services that are covered under Original Medicare.

| Services that are covered for you                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                 | What you must pay when you get these services                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                            |
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| <p>Certain drugs for dialysis are covered under your Medicare Part B drug benefit. For information about coverage for Part B Drugs, please go to the section, “Medicare Part B prescription drugs.”</p>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                           |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                          |
| <p><b>Skilled nursing facility (SNF) care</b><br/>(For a definition of “skilled nursing facility care,” see Chapter 12 of this document. Skilled nursing facilities are sometimes called “SNFs.”)<br/>Covered services include, but are not limited to:</p> <ul style="list-style-type: none"> <li><input type="checkbox"/> Semiprivate room (or a private room if medically necessary)</li> <li><input type="checkbox"/> Meals, including special diets</li> <li><input type="checkbox"/> Skilled nursing services</li> <li><input type="checkbox"/> Physical therapy, occupational therapy, and speech language therapy</li> <li><input type="checkbox"/> Drugs administered to you as part of your plan of care (This includes substances that are naturally present in the body, such as blood clotting factors.)</li> <li><input type="checkbox"/> Blood - including storage and administration. Coverage begins with the first pint of blood that you need.</li> <li><input type="checkbox"/> Medical and surgical supplies ordinarily provided by SNFs</li> <li><input type="checkbox"/> Laboratory tests ordinarily provided by SNFs</li> <li><input type="checkbox"/> X-rays and other radiology services ordinarily provided by SNFs</li> <li><input type="checkbox"/> Use of appliances such as wheelchairs ordinarily provided by SNFs</li> <li><input type="checkbox"/> Physician/practitioner services</li> </ul> <p>A 3-day prior hospital stay is not required.<br/>Generally, you will get your SNF care from network facilities. However, under certain conditions listed below, you may be</p> | <p>\$0 copayment each day for Medicare-covered days 1 - 20.<br/>\$203 copayment for additional Medicare-covered days, up to 100 days.<sup>††</sup><br/>You pay these amounts until you reach the out-of-pocket maximum.<br/>You are covered for up to 100 days each benefit period for inpatient services in a SNF, in accordance with Medicare guidelines.<br/>A benefit period begins on the first day you go to a Medicare-covered inpatient hospital or a skilled nursing facility. The benefit period ends when you haven’t been an inpatient at any hospital or SNF for 60 days in a row. If you go to the hospital (or SNF) after one benefit period has ended, a new benefit period begins. There is no limit to the number of benefit periods you can have.</p> |

If you are a Qualified Medicare Beneficiary (QMB) or have full Medicaid benefits then your deductible, coinsurance and/or copayment may be less for services that are covered under Original Medicare.

| Services that are covered for you                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                              | What you must pay when you get these services                                                                                                                                 |
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| <p>able to get your care from a facility that isn't a network provider, if the facility accepts our plan's amounts for payment.</p> <ul style="list-style-type: none"> <li><input type="checkbox"/> A nursing home or continuing care retirement community where you were living right before you went to the hospital (as long as it provides skilled nursing facility care).</li> <li><input type="checkbox"/> A SNF where your spouse or domestic partner is living at the time you leave the hospital.</li> </ul>                                                                                                                                                                                                                                                                                                                                                                                                                                                                          |                                                                                                                                                                               |
| <p> <b>Smoking and tobacco use cessation (counseling to stop smoking or tobacco use)</b></p> <p>If you use tobacco, we cover two counseling quit attempts within a 12-month period as a preventive service. Each counseling attempt includes up to four face-to-face visits.</p>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                              | <p>There is no coinsurance, copayment, or deductible for the Medicare-covered smoking and tobacco use cessation preventive benefits.</p>                                      |
| <p><b>Supervised exercise therapy (SET)</b></p> <p>SET is covered for members who have symptomatic peripheral artery disease (PAD) and have a referral from the physician responsible for PAD treatment.</p> <p>Up to 36 sessions over a 12-week period are covered if the SET program requirements are met.</p> <p>The SET program must:</p> <ul style="list-style-type: none"> <li><input type="checkbox"/> Consist of sessions lasting 30-60 minutes, comprising of a therapeutic exercise-training program for PAD in patients with claudication</li> <li><input type="checkbox"/> Be conducted in a hospital outpatient setting or a physician's office</li> <li><input type="checkbox"/> Be delivered by qualified auxiliary personnel necessary to ensure benefits exceed harms, and who are trained in exercise therapy for PAD</li> <li><input type="checkbox"/> Be under the direct supervision of a physician, physician assistant, or nurse practitioner/clinical nurse</li> </ul> | <p>\$15 copayment for each Medicare-covered supervised exercise therapy (SET) visit.<sup>††</sup></p> <p>You pay these amounts until you reach the out-of-pocket maximum.</p> |

If you are a Qualified Medicare Beneficiary (QMB) or have full Medicaid benefits then your deductible, coinsurance and/or copayment may be less for services that are covered under Original Medicare.

| Services that are covered for you                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                     | What you must pay when you get these services                                                                                                                                                                                                                                                                   |
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| <p>specialist who must be trained in both basic and advanced life support techniques</p> <p>SET may be covered beyond 36 sessions over 12 weeks for an additional 36 sessions over an extended period of time if deemed medically necessary by a health care provider.</p>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                            |                                                                                                                                                                                                                                                                                                                 |
| <p><b>Urgently needed services</b></p> <p>Urgently needed services are provided to treat a non-emergency, unforeseen medical illness, injury, or condition that requires immediate medical care but given your circumstances, it is not possible, or it is unreasonable, to obtain services from network providers. If it is unreasonable given your circumstances to immediately obtain the medical care from a network provider, then your plan will cover the urgently needed services from a provider out-of-network. Services must be immediately needed and medically necessary. Examples of urgently needed services that the plan must cover out of network occur if: You are temporarily outside the service area of the plan and require medically needed immediate services for an unforeseen condition but it is not a medical emergency; or it is unreasonable given your circumstances to immediately obtain the medical care from a network provider.</p> <p>Cost sharing for necessary urgently needed services furnished out-of-network in the United States is the same as for such services furnished in-network.</p> <p>Worldwide coverage for ‘urgently needed services’ when medical services are needed right away because of an illness, injury, or condition that you did not expect or anticipate, and you can’t wait until you are back in our plan’s service area to obtain services. Services provided by a dentist are not covered.</p> | <p>\$40 copayment for each visit.</p> <p>\$0 copayment for worldwide coverage of urgently needed services received outside of the United States. Please see Chapter 7 Section 1.1 for expense reimbursement for worldwide services.</p> <p>You pay these amounts until you reach the out-of-pocket maximum.</p> |

If you are a Qualified Medicare Beneficiary (QMB) or have full Medicaid benefits then your deductible, coinsurance and/or copayment may be less for services that are covered under Original Medicare.

| Services that are covered for you                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                              | What you must pay when you get these services                                                                                                                                                                                                                                                                                                  |
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| <p> <b>Vision services</b></p> <p>Covered services include:</p> <ul style="list-style-type: none"> <li><input type="checkbox"/> Outpatient physician services provided by an ophthalmologist or optometrist for the diagnosis and treatment of diseases and injuries of the eye, including diagnosis or treatment for age-related macular degeneration or cataracts. Original Medicare doesn't cover routine eye exams (eye refractions) for eyeglasses/contacts.</li> <li><input type="checkbox"/> For people who are at high risk of glaucoma, we will cover one glaucoma screening each year. People at high risk of glaucoma include: people with a family history of glaucoma, people with diabetes, African Americans who are age 50 and older, and Hispanic Americans who are 65 or older.</li> <li><input type="checkbox"/> For people with diabetes or signs and symptoms of eye disease, eye exams to evaluate for eye disease are covered per Medicare guidelines. Annual examinations by an ophthalmologist or optometrist are recommended for asymptomatic diabetics.</li> <li><input type="checkbox"/> For people with diabetes, screening for diabetic retinopathy is covered once per year.</li> <li><input type="checkbox"/> One pair of eyeglasses or contact lenses after each cataract surgery that includes insertion of an intraocular lens (additional pairs of eyeglasses or contacts are not covered by Medicare). If you have two separate cataract operations, you cannot reserve the benefit after the first surgery and purchase two eyeglasses after the second surgery. Covered eyeglasses after cataract surgery includes standard frames and lenses as defined by</li> </ul> | <p>\$0 copayment for each Medicare-covered exam.<sup>††</sup></p> <p>\$0 copayment for Medicare-covered glaucoma screening.</p> <p>\$0 copayment for Medicare-covered eye exams to evaluate for eye disease.<sup>††</sup></p> <p>\$0 copayment for one pair of Medicare-covered standard glasses or contact lenses after cataract surgery.</p> |

If you are a Qualified Medicare Beneficiary (QMB) or have full Medicaid benefits then your deductible, coinsurance and/or copayment may be less for services that are covered under Original Medicare.



| Services that are covered for you                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                 | What you must pay when you get these services                                                                                                                                                                                                                                                                                                      |
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| <p>Medicare; any upgrades are not covered (including, but not limited to, deluxe frames, tinting, progressive lenses or anti-reflective coating).</p>                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                             |                                                                                                                                                                                                                                                                                                                                                    |
| <p><b>Vision services - routine eye exam</b><br/>                     We cover 1 routine eye exam (eye refraction) every year</p> <p>You can get more information by viewing the Vendor Information Sheet at <a href="http://myuhc.com/communityplan">myuhc.com/communityplan</a> or by calling Customer Service to have a paper copy sent to you.</p>                                                                                                                                                                                                                                                                                                                                            | <p>Provided by:<br/>                     MARCH® Vision Care<br/>                     \$0 copayment</p>                                                                                                                                                                                                                                             |
| <p><b>Vision services - routine eyewear</b></p> <p>☐ 1 Pair of lenses/frames and contact lenses every year</p> <p>You are responsible for any amount over the plan credit for eyewear.</p> <p>You can get more information by viewing the Vendor Information Sheet at <a href="http://myuhc.com/communityplan">myuhc.com/communityplan</a> or by calling Customer service to have a paper copy sent to you.</p>                                                                                                                                                                                                                                                                                   | <p>Provided by:<br/>                     MARCH® Vision Care<br/>                     \$0 copayment<br/>                     Plan pays up to \$350 toward your purchase of lenses/frames and contact lenses.</p>                                                                                                                                    |
| <p> <b>“Welcome to Medicare” Preventive Visit</b></p> <p>The plan covers the one-time “Welcome to Medicare” preventive visit. The visit includes a review of your health, as well as education and counseling about the preventive services you need (including certain screenings and shots), and referrals for other care if needed. Doesn’t include lab tests, radiological diagnostic tests or non-radiological diagnostic tests. Additional cost share may apply to any lab or diagnostic testing performed during your visit, as described for each separate service in this medical benefits chart.</p> | <p>There is no coinsurance, copayment, or deductible for the “Welcome to Medicare” preventive visit.</p> <p>There is no coinsurance, copayment, or deductible for a one-time Medicare-covered EKG screening if ordered as a result of your “Welcome to Medicare” preventive visit. Please refer to outpatient diagnostic tests and therapeutic</p> |

If you are a Qualified Medicare Beneficiary (QMB) or have full Medicaid benefits then your deductible, coinsurance and/or copayment may be less for services that are covered under Original Medicare.

| Services that are covered for you                                                                                                                                                                                                                                     | What you must pay when you get these services |
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| <b>Important:</b> We cover the “Welcome to Medicare” preventive visit only within the first 12 months you have Medicare Part B. When you make your appointment, let your doctor’s office know you would like to schedule your “Welcome to Medicare” preventive visit. | services and supplies for other EKG’s.        |

\* Covered services that do not count toward your maximum out-of-pocket amount.

†† Covered services where your provider may need to request prior authorization.

If you are a Qualified Medicare Beneficiary (QMB) or have full Medicaid benefits then your deductible, coinsurance and/or copayment may be less for services that are covered under Original Medicare.

**Covered Routine Dental Benefits Included with Your Plan:**

**Annual Maximum: \$1,750**

- ☐ As a part of your UnitedHealthcare Medicare Advantage plan you get a Routine Dental Benefit that provides coverage for non-Medicare covered preventive and other necessary dental services such as:
  - Exams
  - Cleanings (Prophylaxis, Periodontal Maintenance, & Deep Cleanings)
  - Fillings
  - X-rays
  - Crowns
  - Bridges
  - Root Canals
  - Extractions
  - Partial Dentures
  - Complete Dentures
- ☐ \$0 copay for preventive and diagnostic dental care such as exams, routine cleanings, x-rays, and fluoride.
- ☐ A 50% coinsurance applies to dentures and bridges (coinsurance applies to the actual dentures and bridges, not to modifications and adjustments for existing dentures or bridges).
- ☐ All other covered comprehensive services are offered at a \$0 copay (including modification and adjustments to dentures and bridges).
- ☐ Some covered services may consider prior tooth history and procedures in conjunction with frequency limitations. If you wish to discuss detailed information about your plan with your dentist or see the full list of covered dental services with associated frequency limitations, you can find it in the UHC Dental Medicare quick reference guide at [uhcmedicare dentalproviderqrg.com](https://uhcmedicare dentalproviderqrg.com).
- ☐ Procedures used for cosmetic-only reasons (tooth bleaching/whitening, veneers, gingival recontouring, enamel microabrasion), orthodontics, space maintenance, implants and implant-related services, sales tax, charges for failure to keep appointments, dental case management, dental charges related to COVID screening, testing and vaccination, and unspecified procedures by report are not covered by the plan.
- ☐ After the annual maximum is exhausted, any remaining charges are your responsibility. Other limitations and exclusions are listed below.
- ☐ This dental plan offers access to the robust UHC Dental National Medicare Advantage Network. Network dentists have agreed to provide services at a negotiated rate. If you see

a network dentist, you cannot be billed more than that rate for covered services within the limitations of the plan. Any fees associated with non-covered services are your responsibility.

- ☐ For assistance finding a provider, please use the dental provider search tool at [myuhc.com/communityplan](https://myuhc.com/communityplan). You may also call 1-844-368-7149 for help with finding a provider or scheduling a dental appointment
- ☐ This dental plan offers both in-network and out-of-network dental coverage. Out-of-network dentists are not contracted to accept plan payment as payment in full, so they might charge you for more than what the plan pays, even for services listed as \$0 copayment. Seeing a provider from the robust dental network can therefore result in substantial savings. Benefits received out-of-network are subject to any in-network benefit maximums, limitations and/or exclusions.
- ☐ When you have covered dental services performed at a network dentist, the dentist will submit the claim on your behalf. When you see an out-of-network dentist, often the dentist will submit a claim on your behalf. If they do not, then you can submit it directly using the following instructions:
  - The claim submission must contain the following information:
    - Full member name and member ID number
    - Full provider name and address
    - List of dental services rendered with the corresponding ADA code(s)
    - Proof of payment in the form of a receipt, check copy, EOB, or a ledger statement from the provider showing a positive payment against the services rendered
  - Mail all required claim information within 365 days from the date of service to: **P.O. Box 644, Milwaukee, WI 53201**
  - Payment will be sent to the address listed on your account. To update your address or for assistance with submitting claims, contact Customer Service at 1-844-368-7149 TTY 711.
  - Claims are paid within 30 days and an Explanation of Payment (EOP) will accompany check payment
- ☐ Dentists may ask you to sign an informed consent document detailing the risks, benefits, costs, and alternatives to all recommended treatments. If you would like to learn more how your dental plan coverage relates to your proposed dental treatment and costs, you may ask your dentist to obtain a pre-treatment cost calculation from UHC Dental. If the provider has questions about how to obtain this information, they can contact UHC Dental using the number or website on the back of your Member ID card.

- ☐ For all other questions or more information, please call 1-844-368-7149 TTY 711 or visit [myuhc.com/communityplan](https://myuhc.com/communityplan)

**Exclusions:**

1. Services performed by an out-of-network dentist if your plan does not have out-of-network coverage.
2. Dental services that are not necessary.
3. Hospitalization or other facility charges.
4. Any dental procedure performed solely for cosmetic and/or aesthetic reasons.
5. Any dental procedure not directly associated with a dental disease.
6. Any procedure not performed in a dental setting.
7. Reconstructive surgery of any type, including reconstructive surgery related to a dental disease, injury, or congenital anomaly.
8. Procedures that are considered experimental, investigational or unproven. This includes pharmacological regimens not accepted by the American Dental Association Council on dental therapeutics. The fact that an experimental, investigational or unproven service, treatment, device or pharmacological regimen is the only available treatment for a particular condition will not result in coverage if the procedure is considered to be experimental, investigational or unproven in the treatment of that particular condition.
9. Service for injuries or conditions covered by workmen's compensation or employer liability laws, and services that are provided without cost to the covered persons by any municipality, county, or other political subdivision. This exclusion does NOT apply to any services covered by Medicaid or Medicare.
10. Expenses for dental procedures begun prior to the covered person's eligibility with the plan.
11. Dental services rendered (including otherwise covered dental services) after the date on which individual coverage under the policy terminates, including dental services for dental conditions arising prior to the date on which individual coverage under the policy terminates.
12. Services rendered by a provider with the same legal residence as a covered person or who is a member of a covered person's family, including a spouse, brother, sister, parent or child.
13. Charges for failure to keep a scheduled appointment without giving the dental office 24 hours notice, sales tax, or duplicating/copying patient records.
14. Implants and implant-related services.

15. Tooth bleaching and/or enamel microabrasion
16. Veneers
17. Orthodontics
18. Sustained release of therapeutic drug (D9613)
19. COVID screening, testing, and vaccination
20. Charges aligned to dental case management, case presentation, consultation with other medical professionals or translation/sign language services.
21. Space Maintenance
22. Any unspecified procedure by report (Dental codes: D##99)

**Disclaimer:** Treatment plans and recommended dental procedures may vary. Talk to your dentist about treatment options, risks, benefits, and fees. CDT code changes are issued annually by the American Dental Association. Procedure codes may be altered during the plan year in accordance with discontinuation of certain dental codes.

### Section 3 What services are covered outside of UHC Dual Complete NE-V001 (HMO-POS D-SNP)?

#### Section 3.1 Services not covered by UHC Dual Complete NE-V001 (HMO-POS D-SNP)

##### Benefits covered outside of UHC Dual Complete NE-V001 (HMO-POS D-SNP)

For services that are not covered by UHC Dual Complete NE-V001 (HMO-POS D-SNP) but are available through Medicaid please see your Medicaid Member Handbook.

### Section 4 What services are not covered by the plan?

#### Section 4.1 Services we do not cover (exclusions)

This section tells you what services are “excluded” from Medicare coverage and therefore, are not covered by this plan.

The chart below describes some services and items that aren’t covered by Medicare under any conditions or are covered by Medicare only under specific conditions.

If you get services that are excluded (not covered), you must pay for them yourself, except under the specific conditions listed below. Even if you receive the excluded services at an emergency facility, the excluded services are still not covered and our plan will not pay for them.

The only exception is if the service is appealed and decided upon appeal to be a medical service that we should have paid for or covered because of your specific situation. (For information about appealing a decision we have made to not cover a medical service, go to Chapter 9, Section 6.3 in this document.)

| Services not covered by Medicare                                                                                                                                                           | Not covered under any condition | Covered only under specific conditions                                                                                                                                              |
|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|---------------------------------|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Services considered not reasonable and necessary, according to Original Medicare standards.                                                                                                | Not covered under any condition |                                                                                                                                                                                     |
| Experimental medical and surgical procedures, equipment and medications.<br><br>Experimental procedures and items are those items and procedures determined by Original Medicare to not be |                                 | May be covered by Original Medicare under a Medicare-approved clinical research study or by our plan. (See Chapter 3, Section 5 for more information on clinical research studies.) |



| Services not covered by Medicare                                                                                                                                                                                                             | Not covered under any condition | Covered only under specific conditions                                                                                                                                                                         |
|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|---------------------------------|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| generally accepted by the medical community.                                                                                                                                                                                                 |                                 |                                                                                                                                                                                                                |
| Private room in a hospital.                                                                                                                                                                                                                  |                                 | Covered only when medically necessary.                                                                                                                                                                         |
| Personal items in your room at a hospital or a skilled nursing facility, such as a telephone or a television.                                                                                                                                | Not covered under any condition |                                                                                                                                                                                                                |
| Full-time nursing care in your home.                                                                                                                                                                                                         | Not covered under any condition |                                                                                                                                                                                                                |
| Custodial care.<br><br>Custodial care is personal care that does not require the continuing attention of trained medical or paramedical personnel, such as care that helps you with activities of daily living, such as bathing or dressing. | Not covered under any condition |                                                                                                                                                                                                                |
| Homemaker services including basic household assistance, such as light housekeeping or light meal preparation.                                                                                                                               | Not covered under any condition |                                                                                                                                                                                                                |
| Fees charged for care by your immediate relatives or members of your household.                                                                                                                                                              | Not covered under any condition |                                                                                                                                                                                                                |
| Cosmetic surgery or procedures.                                                                                                                                                                                                              |                                 | Covered in cases of an accidental injury or for improvement of the functioning of a malformed body member.<br><br>Covered for all stages of reconstruction for a breast after a mastectomy, as well as for the |

| Services not covered by Medicare                                                                                                                                                                                                        | Not covered under any condition | Covered only under specific conditions                                                                                                                                                                                          |
|-----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|---------------------------------|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
|                                                                                                                                                                                                                                         |                                 | unaffected breast to produce a symmetrical appearance.                                                                                                                                                                          |
| Chiropractic services (Medicare-covered)                                                                                                                                                                                                |                                 | Manual manipulation of the spine to correct a subluxation is covered. Excluded from Medicare coverage is any service other than manual manipulation of the spine for the treatment of subluxation.                              |
| Non-routine dental care.                                                                                                                                                                                                                |                                 | Dental care required to treat illness or injury may be covered as inpatient or outpatient care.                                                                                                                                 |
| Orthopedic shoes or supportive devices for the feet.                                                                                                                                                                                    |                                 | Shoes that are part of a leg brace and are included in the cost of the brace. Orthopedic or therapeutic shoes for people with diabetic foot disease. (As specifically described in the medical benefits chart in this chapter.) |
| Outpatient prescription drugs.                                                                                                                                                                                                          |                                 | Some coverage provided according to Medicare guidelines. (As specifically described in the medical benefits chart in this chapter or as outlined in Chapter 6.)                                                                 |
| Elective hysterectomy, tubal ligation, or vasectomy, if the primary indication for these procedures is sterilization. Reversal of sterilization procedures, penile vacuum erection devices, or non-prescription contraceptive supplies. | Not covered under any condition |                                                                                                                                                                                                                                 |

| Services not covered by Medicare                                                                                                                                                                                                                                                                                                         | Not covered under any condition | Covered only under specific conditions                                                                                                                                                                                               |
|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|---------------------------------|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Acupuncture (Medicare-covered).                                                                                                                                                                                                                                                                                                          |                                 | Available for people with chronic low back pain under certain circumstances.<br>(As specifically described in the medical benefits chart in this chapter.)                                                                           |
| Naturopath services (uses natural or alternative treatments).                                                                                                                                                                                                                                                                            | Not covered under any condition |                                                                                                                                                                                                                                      |
| Paramedic intercept service (advanced life support provided by an emergency service entity, such as a paramedic services unit, which do not provide ambulance transport)                                                                                                                                                                 |                                 | Services are only covered when the ambulance pick-up address is located in rural New York and applicable conditions are met. Members are responsible for all paramedic intercept service costs that occur outside of rural New York. |
| Optional, additional, or deluxe features or accessories to durable medical equipment, corrective appliances or prosthetics which are primarily for the comfort or convenience of the member, or for ambulation primarily in the community, including but not limited to home and car remodeling or modification, and exercise equipment. | Not covered under any condition |                                                                                                                                                                                                                                      |
| Immunizations for foreign travel purposes.                                                                                                                                                                                                                                                                                               | Not covered under any condition |                                                                                                                                                                                                                                      |
| Requests for payment (asking the plan to pay its share of the costs) for covered drugs sent after 36 months of getting your prescription filled.                                                                                                                                                                                         | Not covered under any condition |                                                                                                                                                                                                                                      |

| Services not covered by Medicare                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                        | Not covered under any condition             | Covered only under specific conditions                                                                                                 |
|-----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|---------------------------------------------|----------------------------------------------------------------------------------------------------------------------------------------|
| Equipment or supplies that condition the air, heating pads, hot water bottles, wigs and their care, support stockings and other primarily non-medical equipment.                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                        | Not covered by Medicare under any condition | Disposable or nonreusable items such as incontinence supplies are not covered under Medicare but may be covered under the OTC benefit. |
| Any non-emergency care received outside of the United States and the U.S. Territories.                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                  | Not covered under any condition             |                                                                                                                                        |
| <p>For transplants: items not covered include, but are not limited to the below.</p> <p>For transportation:</p> <ul style="list-style-type: none"> <li><input type="checkbox"/> Vehicle rental, purchase, or maintenance/repairs</li> <li><input type="checkbox"/> Auto clubs (roadside assistance)</li> <li><input type="checkbox"/> Gas</li> <li><input type="checkbox"/> Travel by air or ground ambulance (may be covered under your medical benefit).</li> <li><input type="checkbox"/> Air or ground travel not related to medical appointments</li> <li><input type="checkbox"/> Parking fees incurred other than at lodging or hospital</li> </ul> <p>For lodging:</p> <ul style="list-style-type: none"> <li><input type="checkbox"/> Deposits</li> <li><input type="checkbox"/> Utilities (if billed separate from the rent payment)</li> <li><input type="checkbox"/> Phone calls, newspapers, movie rentals and gift cards</li> <li><input type="checkbox"/> Expenses for lodging when staying with a relative or friend</li> <li><input type="checkbox"/> Meals</li> </ul> | Not covered under any condition             |                                                                                                                                        |

| Services not covered by Medicare                  | Not covered under any condition | Covered only under specific conditions  |
|---------------------------------------------------|---------------------------------|-----------------------------------------|
| Self-administered drugs in an outpatient hospital |                                 | Covered only under specific conditions. |

We regularly review new procedures, devices and drugs to determine whether or not they are safe and effective for members. New procedures and technology that are safe and effective are eligible to become covered services. If the technology becomes a covered service, it will be subject to all other terms and conditions of the plan, including medical necessity and any applicable member copayments, coinsurance, deductibles or other payment contributions.

In determining whether to cover a service, we use proprietary technology guidelines to review new devices, procedures and drugs, including those related to behavioral/mental health. When clinical necessity requires a rapid determination of the safe and effective use of a new technology or new application of an existing technology for an individual member, one of our medical directors makes a medical necessity determination based on individual member medical documentation, review of published scientific evidence, and, when appropriate, relevant specialty or professional opinion from an individual who has expertise in the technology.

# **Chapter 5**

Using the plan's coverage for Part D  
prescription drugs



### How can you get information about your drug costs?

Because you are eligible for Nebraska Department of Health and Human Services (Medicaid), you qualify for and are getting “Extra Help” from Medicare to pay for your prescription drug plan costs. Because you are in the “Extra Help” program, **some information in this Evidence of Coverage about the costs for Part D prescription drugs does not apply to you.**

## Section 1 Introduction

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This chapter **explains rules for using your coverage for Part D drugs**. Please see Chapter 4 for Medicare Part B drug benefits and hospice drug benefits.

In addition to the drugs covered by Medicare, some prescription drugs are covered for you under your Medicaid benefits. You can learn more about prescription drug coverage under your Medicaid benefits by contacting your Medicaid health plan or Nebraska Department of Health and Human Services (Medicaid) listed in Chapter 2 of this booklet. Your Medicaid health plan or Nebraska Department of Health and Human Services (Medicaid) may also be able to provide a Medicaid Drug List that tells you how to find out which drugs are covered under Medicaid.

### Section 1.1 Basic rules for the plan's Part D drug coverage

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The plan will generally cover your drugs as long as you follow these basic rules:

- ☐ You must have a provider (a doctor, dentist, or other prescriber) write you a prescription which must be valid under applicable state law.
- ☐ Your prescriber must not be on Medicare's Exclusion or Preclusion Lists.
- ☐ You generally must use a network pharmacy to fill your prescription. (See Section 2, **Fill your prescriptions at a network pharmacy or through the plan's mail-order service.**)
- ☐ Your drug must be on the plan's **List of Covered Drugs (Formulary)** (we call it the Drug List for short). (See Section 3, **Your drugs need to be on the plan's Drug List.**)
- ☐ Your drug must be used for a medically accepted indication. A “medically accepted indication” is a use of the drug that is either approved by the Food and Drug Administration or supported by certain references. (See Section 3 for more information about a medically accepted indication.)

## Section 2 Fill your prescription at a network pharmacy or through the plan's mail-order service

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### Section 2.1 Use a network pharmacy

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In most cases, your prescriptions are covered **only** if they are filled at the plan's network pharmacies. (See Section 2.5 for information about when we would cover prescriptions filled at out-of-network pharmacies.)



A network pharmacy is a pharmacy that has a contract with the plan to provide your covered prescription drugs. The term “covered drugs” means all of the Part D prescription drugs that are on the plan's Drug List.

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## **Section 2.2      Network pharmacies**

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### **How do you find a network pharmacy in your area?**

To find a network pharmacy, you can look in your **Pharmacy Directory**, visit our website ([myuhc.com/communityplan](http://myuhc.com/communityplan)), and/or call Customer Service.

You may go to any of our network pharmacies.

### **What if the pharmacy you have been using leaves the network?**

If the pharmacy you have been using leaves the plan's network, you will have to find a new pharmacy that is in the network. To find another pharmacy in your area, you can get help from Customer Service or use the **Pharmacy Directory**. You can also find information on our website at [myuhc.com/communityplan](http://myuhc.com/communityplan).

### **What if you need a specialized pharmacy?**

Some prescriptions must be filled at a specialized pharmacy. Specialized pharmacies include:

- ☐ Pharmacies that supply drugs for home infusion therapy.
- ☐ Pharmacies that supply drugs for residents of a long-term care (LTC) facility. Usually, a LTC facility (such as a nursing home) has its own pharmacy. If you have any difficulty accessing your Part D benefits in an LTC facility, please contact Customer Service.
- ☐ Pharmacies that serve the Indian Health Service / Tribal / Urban Indian Health Program (not available in Puerto Rico). Except in emergencies, only Native Americans or Alaska Natives have access to these pharmacies in our network.
- ☐ Pharmacies that dispense drugs that are restricted by the FDA to certain locations or that require special handling, provider coordination, or education on their use. (Note: This scenario should happen rarely.)

To locate a specialized pharmacy, look in your **Pharmacy Directory** or call Customer Service.

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## **Section 2.3      Using the plan's mail-order service**

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Our plan's mail-order service allows you to order **up to a 100-day supply**.

To get order forms and information about filling your prescriptions by mail, please reference your **Pharmacy Directory** to find the mail service pharmacies in our network. If you use a mail-order pharmacy not in the plan's network, your prescription will not be covered.

Usually a mail-order pharmacy order will be delivered to you in no more than 10 business days. However, sometimes your mail-order may be delayed. If your mail-order is delayed, please follow these steps:

If your prescription is on file at your local pharmacy, go to your pharmacy to fill the prescription. If your delayed prescription is not on file at your local pharmacy, then please ask your doctor to call in a new prescription to your pharmacist. Or, your pharmacist can call the doctor's office for you to

request the prescription. Your pharmacist can call the Pharmacy help desk at 1-877-889-6510, (TTY) 711, 24 hours a day, 7 days a week if he/she has any problems, questions, concerns, or needs a claim override for a delayed prescription.

**New prescriptions the pharmacy receives directly from your doctor's office.**

The pharmacy will automatically fill and deliver new prescriptions it receives from health care providers, without checking with you first, if either:

- ☐ You used mail-order services with this plan in the past, or
- ☐ You sign up for automatic delivery of all new prescriptions received directly from health care providers. You may request automatic delivery of all new prescriptions at any time by phone or mail.

If you receive a prescription automatically by mail that you do not want, and you were not contacted to see if you wanted it before it shipped, you may be eligible for a refund.

If you used mail order in the past and do not want the pharmacy to automatically fill and ship each new prescription, please contact us by phone or mail.

If you have never used our mail-order delivery and/or decide to stop automatic fills of new prescriptions, the pharmacy will contact you each time it gets a new prescription from a health care provider to see if you want the medication filled and shipped immediately. It is important that you respond each time you are contacted by the pharmacy, to let them know whether to ship, delay, or cancel the new prescription.

Refills on mail-order prescriptions. For refills of your drugs, you have the option to sign up for an automatic refill program. Under this program we will start to process your next refill automatically when our records show you should be close to running out of your drug. The pharmacy will contact you prior to shipping each refill to make sure you are in need of more medication, and you can cancel scheduled refills if you have enough of your medication or if your medication has changed. If you choose not to use our auto-refill program but still want the mail-order pharmacy to send you your prescription, please contact your pharmacy 10 days before your current prescription will run out. This will ensure your order is shipped to you in time.

To opt out of our program that automatically prepares mail-order refills, please contact us by calling Optum Rx at 1-877-889-5802.

If you receive a refill automatically by mail that you do not want, you may be eligible for a refund.

Please keep your mail order pharmacy informed about the best way(s) to contact you, so the pharmacy can reach you to confirm your order before shipping. You can do this by contacting the mail order pharmacy when you set up your auto refill program and also when you receive notifications about upcoming refill shipments.

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**Section 2.4      How can you get a long-term supply of drugs?**

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When you get a long-term supply of drugs, your cost-sharing may be lower. The plan offers two ways to get a long-term supply (also called an "extended supply") of "maintenance" drugs on our plan's Drug List. (Maintenance drugs are drugs that you take on a regular basis, for a chronic or long-term medical condition.)

1. **Some retail pharmacies** in our network allow you to get a long-term supply of maintenance drugs. Your **Pharmacy Directory** tells you which pharmacies in our network can give you a long-term supply of maintenance drugs. You can also call Customer Service for more information.
2. You may also receive maintenance drugs through our mail-order program. Please see Section 2.3 for more information.

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## Section 2.5 When can you use a pharmacy that is not in the plan's network?

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### Your prescription may be covered in certain situations

Generally, we cover drugs filled at an out-of-network pharmacy **only** when you are not able to use a network pharmacy. To help you, we have network pharmacies outside of our service area where you can get your prescriptions filled as a member of our plan. **Please check first with Customer Service** to see if there is a network pharmacy nearby. You will most likely be required to pay the difference between what you pay for the drug at the out-of-network pharmacy and the cost that we would cover at an in-network pharmacy.

Here are the circumstances when we would cover prescriptions filled at an out-of-network pharmacy:

- ☐ **Prescriptions for a medical emergency**

We will cover prescriptions that are filled at an out-of-network pharmacy if the prescriptions are related to care for a medical emergency or urgently needed care, are included in our Drug List without restrictions, and are not excluded from Medicare Part D coverage.

- ☐ **Coverage when traveling or out of the service area**

When traveling within the U.S. you have access to network pharmacies nationwide. Bring your prescriptions and medication with you and be sure to check the pharmacy directory for your travel plans to locate a network pharmacy while traveling. If you are leaving the country, you may be able to obtain a greater day supply to take with you before leaving for the country where there are no network pharmacies available.

- ☐ If you are unable to obtain a covered drug in a timely manner within the service area because a network pharmacy that provides 24-hour service is not within reasonable driving distance.
- ☐ If you are trying to fill a prescription drug not regularly stocked at an accessible network retail or mail-order pharmacy (including high cost and unique drugs).
- ☐ If you need a prescription while a patient in an emergency department, provider based clinic, outpatient surgery, or other outpatient setting.

### How do you ask for reimbursement from the plan?

If you must use an out-of-network pharmacy, you will generally have to pay the full cost (rather than your normal cost share) at the time you fill your prescription. You can ask us to reimburse you for our share of the cost. (Chapter 7, Section 2 explains how to ask the plan to pay you back.)

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## Section 3 Your drugs need to be on the plan's Drug List

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### Section 3.1 The Drug List tells which Part D drugs are covered

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The plan has a “**List of Covered Drugs (Formulary).**” In this **Evidence of Coverage, we call it the Drug List for short.**

The drugs on this list are selected by the plan with the help of a team of doctors and pharmacists. The list meets Medicare's requirements and has been approved by Medicare.

The Drug List includes the drugs covered under Medicare Part D. In addition to the drugs covered by Medicare, some prescription drugs are covered for you under your Medicaid benefits. You can learn more about prescription drug coverage under your Medicaid benefits by contacting your Medicaid health plan or Nebraska Department of Health and Human Services (Medicaid) listed in Chapter 2 of this booklet. Your Medicaid health plan or Nebraska Department of Health and Human Services (Medicaid) may also be able to provide a Medicaid Drug List that tells you how to find out which drugs are covered under Medicaid.

We will generally cover a drug on the plan's Drug List as long as you follow the other coverage rules explained in this chapter and the use of the drug is a medically accepted indication. A “medically accepted indication” is a use of the drug that is **either**:

- ☐ Approved by the Food and Drug Administration for the diagnosis or condition for which it is being prescribed.
- ☐ **or** – Supported by certain references, such as the American Hospital Formulary Service Drug Information and the DRUGDEX Information System.

### **The Drug List includes brand name drugs and generic drugs.**

A brand name drug is a prescription drug that is sold under a trademarked name owned by the drug manufacturer. Brand name drugs that are more complex than typical drugs (for example, drugs that are based on a protein) are called biological products. On the drug list, when we refer to “drugs,” this could mean a drug or a biological product.

A generic drug is a prescription drug that has the same active ingredients as the brand name drug. Generally, generics work just as well as the brand name drug and usually cost less. There are generic drug substitutes available for many brand name drugs.

### **What is not on the Drug List?**

The plan does not cover all prescription drugs.

- ☐ In some cases, the law does not allow any Medicare plan to cover certain types of drugs (for more information about this, see Section 7.1 in this chapter).
- ☐ In other cases, we have decided not to include a particular drug on the Drug List. In some cases, you may be able to obtain a drug that is not on the drug list. For more information, please see Chapter 9.
- ☐ Nebraska Department of Health and Human Services (Medicaid)-covered drugs may not be included on this plan's Drug list. If this plan does not cover a specific drug, please check your Nebraska Department of Health and Human Services (Medicaid) Drug list to see if the drug is covered.

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## **Section 3.2      How can you find out if a specific drug is on the Drug List?**

You have four ways to find out:

1. Check the most recent Drug List we provided electronically.
2. Visit the plan's website ([myuhc.com/communityplan](http://myuhc.com/communityplan)). The Drug List on the website is always the most current.
3. Call Customer Service to find out if a particular drug is on the plan's Drug List or to ask for a copy of the list.
4. Use the plan's "Real-Time Benefit Tool" ([myuhc.com/communityplan](http://myuhc.com/communityplan) or by calling Customer Service). With this tool you can search for drugs on the "Drug List" to see an estimate of what you will pay and if there are alternative drugs on the "Drug List" that could treat the same condition.

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## **Section 4            There are restrictions on coverage for some drugs**

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### **Section 4.1            Why do some drugs have restrictions?**

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For certain prescription drugs, special rules restrict how and when the plan covers them. A team of doctors and pharmacists developed these rules to encourage you and your provider to use drugs in the most effective ways. To find out if any of these restrictions apply to a drug you take or want to take, check the Drug List. If a safe, lower-cost drug will work just as well medically as a higher-cost drug, the plan's rules are designed to encourage you and your provider to use that lower-cost option.

Please note that sometimes a drug may appear more than once in our Drug List. This is because the same drugs can differ based on the strength, amount, or form of the drug prescribed by your health care provider, and different restrictions or cost-sharing may apply to the different versions of the drug (for instance, 10 mg versus 100 mg; one per day versus two per day; tablet versus liquid).

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### **Section 4.2            What kinds of restrictions?**

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The sections below tell you more about the types of restrictions we use for certain drugs.

**If there is a restriction for your drug, it usually means that you or your provider will have to take extra steps in order for us to cover the drug.** Contact Customer Service to learn what you or your provider would need to do to get coverage for the drug. If you want us to waive the restriction for you, you will need to use the coverage decision process and ask us to make an exception. We may or may not agree to waive the restriction for you. (See Chapter 9)

#### **Restricting brand name drugs when a generic version is available**

Generally, a generic drug works the same as a brand name drug and usually costs less. **In most cases, when a generic version of a brand name drug is available, our network pharmacies will provide you the generic version instead of the brand name drug.** However, if your provider has told us the medical reason that the generic drug will not work for you OR has written "No substitutions" on your prescription for a brand name drug OR has told us the medical reason that neither the generic drug nor other covered drugs that treat the same condition will work for you, then we will cover the brand name drug. (Your share of the cost may be greater for the brand name drug than for the generic drug.)

#### **What is a compounded drug?**

A compounded drug is created by a pharmacist by combining or mixing ingredients to create a prescription medication customized to the needs of an individual patient.

### Does my Part D plan cover compounded drugs?

Generally compounded drugs are non-formulary drugs (not covered) by your plan. You may need to ask for and receive an approved coverage determination from us to have your compounded drug covered. Compounded drugs may be Part D eligible if they meet all of the following requirements:

1. Contains at least one FDA, or Compendia, approved drug ingredient, and all ingredients in the compound (including their intended route of administration) are supported in the Compendia.
2. Does not contain a non-FDA approved or Part D excluded drug ingredient
3. Does not contain an ingredient covered under Part B. (If it does, the compound may be covered under Part B rather than Part D)
4. Prescribed for a medically accepted condition

The chart below explains the basic requirements for how a compound with 2 or more ingredients may or may not be covered under Part D rules, as well as potential costs to you.

| Compound Type                                                                                                                                                                              | Medicare Coverage                                                                                                                                                                                                                              |
|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Compound containing a Part B eligible ingredient                                                                                                                                           | Compound is covered only by Part B                                                                                                                                                                                                             |
| Compound containing all ingredients eligible for Part D coverage and all ingredients are approved for use in a compound                                                                    | Compound may be covered by Part D upon approved coverage determination                                                                                                                                                                         |
| Compound containing ingredients eligible for Part D coverage and approved for use in a compound, and ingredients excluded from Part D coverage (for example, over the counter drugs, etc.) | Compound may be covered by Part D upon approved coverage determination. However, the ingredients excluded from Part D coverage will not be covered and you are not responsible for the cost of those ingredients excluded from Part D coverage |
| Compound containing an ingredient not approved or supported for use in a compound                                                                                                          | Compound is not covered by Part D. You are responsible for the entire cost                                                                                                                                                                     |

### What do I have to pay for a covered compounded drug?

A compounded drug that is Part D eligible may require an approved coverage determination to be covered by your plan.

### Getting plan approval in advance

For certain drugs, you or your provider need to get approval from the plan before we will agree to cover the drug for you. This is called “**prior authorization**.” This is put in place to ensure



medication safety and help guide appropriate use of certain drugs. If you do not get this approval, your drug might not be covered by the plan.

### **Trying a different drug first**

This requirement encourages you to try less costly but usually just as effective drugs before the plan covers another drug. For example, if Drug A and Drug B treat the same medical condition and Drug A is less costly, the plan may require you to try Drug A first. If Drug A does not work for you, the plan will then cover Drug B. This requirement to try a different drug first is called “**step therapy.**”

### **Quantity limits**

For certain drugs, we limit how much of a drug you can get each time you fill your prescription. For example, if it is normally considered safe to take only one pill per day for a certain drug, we may limit coverage for your prescription to no more than one pill per day.

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## **Section 5                      What if one of your drugs is not covered in the way you'd like it to be covered?**

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### **Section 5.1                      There are things you can do if your drug is not covered in the way you'd like it to be covered**

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There are situations where there is a prescription drug you are taking, or one that you and your provider think you should be taking, that is not on our drug list (formulary) or is on our formulary with restrictions. For example:

- ☐ The drug might not be covered at all. Or maybe a generic version of the drug is covered but the brand name version you want to take is not covered.
- ☐ The drug is covered, but there are extra rules or restrictions on coverage for that drug, as explained in Section 4.
- ☐ There are things you can do if your drug is not covered in the way that you'd like it to be covered. If your drug is not on the Drug List or if your drug is restricted, go to Section 5.2 to learn what you can do.

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### **Section 5.2                      What can you do if your drug is not on the Drug List or if the drug is restricted in some way?**

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If your drug is not on the Drug List or is restricted, here are options:

- ☐ You may be able to get a temporary supply of the drug.
- ☐ You can change to another drug.
- ☐ You can request an exception and ask the plan to cover the drug or remove restrictions from the drug.

#### **You may be able to get a temporary supply**

Under certain circumstances, the plan must provide a temporary supply of a drug that you are already taking. This temporary supply gives you time to talk with your provider about the change in coverage and decide what to do.



To be eligible for a temporary supply, the drug you have been taking must no longer be on the plan's Drug List OR is now restricted in some way.

- ☐ If you are a new member, we will cover a temporary supply of your drug during the first 90 days of your membership in the plan.
- ☐ If you were in the plan last year, we will cover a temporary supply of your drug during the first 90 days of the calendar year.
- ☐ This temporary supply will be for at least a 30-day supply. If your prescription is written for fewer days, we will allow multiple fills to provide up to at least a 30-day supply of medication. The prescription must be filled at a network pharmacy. (Please note that the long-term care pharmacy may provide the drug in smaller amounts at a time to prevent waste.)
- ☐ **For those members who have been in the plan for more than 90 days and reside in a long-term care facility and need a supply right away:**

We will cover at least a 31-day emergency supply of a particular drug, or less if your prescription is written for fewer days. This is in addition to the above temporary supply.

- ☐ **For those current members with level of care changes:**

There may be unplanned transitions such as hospital discharges (including psychiatric hospitals) or level of care changes (i.e., changing long-term care facilities, exiting and entering a long-term care facility, ending Part A coverage within a skilled nursing facility, or ending hospice coverage and reverting to Medicare coverage) that can occur anytime. If you are prescribed a drug that is not on our Drug List or your ability to get your drugs is restricted in some way, you are required to use the plan's exception process. For most drugs, you may request a one-time temporary supply of at least 30 days to allow you time to discuss alternative treatment with your doctor or to request a Drug List (formulary) exception. If your doctor writes your prescription for fewer days, you may refill the drug until you've received at least a 30 day supply.

For questions about a temporary supply, call Customer Service.

During the time when you are using a temporary supply of a drug, you should talk with your provider to decide what to do when your temporary supply runs out. You have two options:

**1) You can change to another drug**

Talk with your provider about whether there is a different drug covered by the plan that may work just as well for you. You can call Customer Service to ask for a list of covered drugs that treat the same medical condition. This list can help your provider find a covered drug that might work for you.

**2) You can ask for an exception**

You and your provider can ask the plan to make an exception and cover the drug in the way you would like it covered. If your provider says that you have medical reasons that justify asking us for an exception, your provider can help you request an exception. For example, you can ask the plan to cover a drug even though it is not on the plan's Drug List. Or you can ask the plan to make an exception and cover the drug without restrictions.

If you are a current member and a drug you are taking will be removed from the formulary or restricted in some way for next year, we will tell you about any change prior to the new year. You

can ask for an exception before next year and we will give you an answer within 72 hours after we receive your request (or your prescriber's supporting statement). If we approve your request, we will authorize the coverage before the change takes effect.

If you and your provider want to ask for an exception, Chapter 9, Section 7.4 tells you what to do. It explains the procedures and deadlines that have been set by Medicare to make sure your request is handled promptly and fairly.

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## **Section 6            What if your coverage changes for one of your drugs?**

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### **Section 6.1            The Drug List can change during the year**

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Most of the changes in drug coverage happen at the beginning of each year (January 1). However, during the year, the plan can make some changes to the Drug List. For example, the plan might:

- ☐ **Add or remove drugs from the Drug List.**
- ☐ **Add or remove a restriction on coverage for a drug.**
- ☐ **Replace a brand name drug with a generic version of the drug.**

We must follow Medicare requirements before we change the plan's Drug List.

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### **Section 6.2            What happens if coverage changes for a drug you are taking?**

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#### **Information on changes to drug coverage**

When changes to the Drug List occur, we post information on our website about those changes. We also update our online Drug List on a regularly scheduled basis. Below we point out the times that you would get direct notice if changes are made to a drug that you are taking.

#### **Changes to your drug coverage that affect you during the current plan year**

- ☐ **A new generic drug replaces a brand name drug on the Drug List (or we change the cost-sharing tier or add new restrictions to the brand name drug or both)**
  - ☐ We may immediately remove a brand name drug on our Drug List if we are replacing it with a newly approved generic version of the same drug. The generic drug will appear on the same or lower cost-sharing tier and with the same or fewer restrictions. We may decide to keep the brand name drug on our Drug List, but immediately move it to a higher cost-sharing tier or add new restrictions or both when the new generic is added.
  - ☐ We may not tell you in advance before we make that change—even if you are currently taking the brand name drug. If you are taking the brand name drug at the time we make the change, we will provide you with information about the specific change(s). This will also include information on the steps you may take to request an exception to cover the brand name drug. You may not get this notice before we make the change.
  - ☐ You or your prescriber can ask us to make an exception and continue to cover the brand name drug for you. For information on how to ask for an exception, see Chapter 9.
- ☐ **Unsafe drugs and other drugs on the Drug List that are withdrawn from the market**

- ☐ Sometimes a drug may be deemed unsafe or taken off the market for another reason. If this happens, we may immediately remove the drug from the Drug List. If you are taking that drug, we will tell you right away.
- ☐ Your prescriber will also know about this change, and can work with you to find another drug for your condition.
- ☐ **Other changes to drugs on the Drug List**
  - ☐ We may make other changes once the year has started that affect drugs you are taking. For example, we might add a generic drug that is not new to the market to replace a brand name drug on the Drug List or change the cost-sharing tier or add new restrictions to the brand name drug or both. We also might make changes based on FDA boxed warnings or new clinical guidelines recognized by Medicare.
  - ☐ For these changes, we must give you at least 30-days' advance notice of the change or give you notice of the change and a 30-day refill of the drug you are taking at a network pharmacy.
  - ☐ After you receive notice of the change, you should work with your provider to switch to a different drug that we cover or to satisfy any new restrictions on the drug you are taking.
  - ☐ You or your prescriber can ask us to make an exception and continue to cover the drug for you. For information on how to ask for an exception, see Chapter 9.

### **Changes to the Drug List that do not affect you during this plan year**

We may make certain changes to the Drug List that are not described above. In these cases, the change will not apply to you if you are taking the drug when the change is made; however, these changes will likely affect you starting January 1 of the next plan year if you stay in the same plan.

In general, changes that will not affect you during the current plan year are:

- ☐ We put a new restriction on the use of your drug.
- ☐ We remove your drug from the Drug List.

If any of these changes happen for a drug you are taking (except for market withdrawal, a generic drug replacing a brand name drug, or other change noted in the sections above), then the change won't affect your use or what you pay as your share of the cost until January 1 of the next year. Until that date, you probably won't see any increase in your payments or any added restrictions to your use of the drug.

We will not tell you about these types of changes directly during the current plan year. You will need to check the Drug List for the next plan year (when the list is available during the open enrollment period) to see if there are any changes to the drugs you are taking that will impact you during the next plan year.

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## **Section 7                      What types of drugs are not covered by the plan?**

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### **Section 7.1                  Types of drugs we do not cover**

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This section tells you what kinds of prescription drugs are “excluded.” This means Medicare does not pay for these drugs.

If you appeal and the requested drug is found not to be excluded under Part D, we will pay for or cover it. (For information about appealing a decision, go to Chapter 9.) If the drug excluded by our plan is also excluded by Medicaid, you must pay for it yourself.

Here are four general rules about drugs that Medicare drug plans will not cover under Part D:

- ☐ Our plan's Part D drug coverage cannot cover a drug that would be covered under Medicare Part A or Part B.
- ☐ Our plan cannot cover a drug purchased outside the United States or its territories.
- ☐ Our plan usually cannot cover off-label use. “Off-label use” is any use of the drug other than those indicated on a drug's label as approved by the Food and Drug Administration.
- ☐ Coverage for “off-label use” is allowed only when the use is supported by certain references, such as the American Hospital Formulary Service Drug Information and the DRUGDEX Information System.

In addition, by law, the following categories of drugs listed below are not covered by Medicare. However, some of these drugs may be covered for you under your Nebraska Department of Health and Human Services (Medicaid) drug coverage. Please check your Nebraska Department of Health and Human Services (Medicaid) Drug list to see if any of the drugs listed below are covered:

- ☐ Non-prescription drugs (also called over-the-counter drugs).
- ☐ Drugs used to promote fertility.
- ☐ Drugs used for the relief of cough or cold symptoms.
- ☐ Drugs used for cosmetic purposes or to promote hair growth.
- ☐ Prescription vitamins and mineral products, except prenatal vitamins and fluoride preparations.
- ☐ Drugs used for the treatment of sexual or erectile dysfunction.
- ☐ Drugs used for treatment of anorexia, weight loss, or weight gain.
- ☐ Outpatient drugs for which the manufacturer seeks to require that associated tests or monitoring services be purchased exclusively from the manufacturer as a condition of sale.

In addition, if you are **receiving Extra Help** from Medicare to pay for your prescriptions, the Extra Help program will not pay for the drugs not normally covered. (Please refer to the plan's Drug List or call Customer Service for more information.) However, if you have drug coverage through Medicaid, your state Medicaid program may cover some prescription drugs not normally covered in a Medicare drug plan. Please contact your state Medicaid program to determine what drug coverage may be available to you. (You can find phone numbers and contact information for Medicaid in Chapter 2, Section 6.)

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## Section 8      Filling a prescription

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### Section 8.1      Provide your UnitedHealthcare member ID information

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To fill your prescription, provide your UnitedHealthcare member ID information, which can be found on your member ID card, at the network pharmacy you choose. The network pharmacy will automatically bill the plan for your drug.

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## **Section 8.2      What if you don't have your UnitedHealthcare member ID information with you?**

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If you don't have your plan membership information with you when you fill your prescription, you or the pharmacy can call the plan to get the necessary information, or you can ask the pharmacy to look up your plan enrollment information.

If the pharmacy is not able to get the necessary information, **you may have to pay the full cost of the prescription when you pick it up.** (You can then **ask us to reimburse you** for our share. See Chapter 7, Section 2 for information about how to ask the plan for reimbursement.)

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## **Section 9      Part D drug coverage in special situations**

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### **Section 9.1      What if you're in a hospital or a skilled nursing facility for a stay that is covered by the plan?**

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If you are admitted to a hospital or to a skilled nursing facility for a stay covered by the plan, we will generally cover the cost of your prescription drugs during your stay. Once you leave the hospital or skilled nursing facility, the plan will cover your prescription drugs as long as the drugs meet all of our rules for coverage described in this Chapter.

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### **Section 9.2      What if you're a resident in a long-term care (LTC) facility?**

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Usually, a long-term care (LTC) facility (such as a nursing home) has its own pharmacy, or uses a pharmacy that supplies drugs for all of its residents. If you are a resident of an LTC facility, you may get your prescription drugs through the facility's pharmacy or the one it uses, as long as it is part of our network.

Check your **Pharmacy Directory** to find out if your LTC facility's pharmacy or the one that it uses is part of our network. If it isn't, or if you need more information or assistance, please contact Customer Service. If you are in an LTC facility, we must ensure that you are able to routinely receive your Part D benefits through our network of LTC pharmacies.

**What if you're a resident in a long-term care (LTC) facility and need a drug that is not on our Drug List or is restricted in some way?**

Please refer to Section 5.2 about a temporary or emergency supply.

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### **Section 9.3      What if you're also getting drug coverage from an employer or retiree group plan?**

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If you currently have other prescription drug coverage through your (or your spouse or domestic partner's) employer or retiree group, please contact **that group's benefits administrator**. He or she can help you determine how your current prescription drug coverage will work with our plan.

In general, if you have employee or retiree group coverage, the drug coverage you get from us will be **secondary** to your group coverage. That means your group coverage would pay first.

**Special note about 'creditable coverage':**

Each year your employer or retiree group should send you a notice that tells if your prescription drug coverage for the next calendar year is "creditable."

If the coverage from the group plan is "**creditable**," it means that the plan has drug coverage that is expected to pay, on average, at least as much as Medicare's standard prescription drug coverage.

**Keep this notice about creditable coverage**, because you may need it later. If you enroll in a Medicare plan that includes Part D drug coverage, you may need this notice to show that you have maintained creditable coverage. If you didn't get the creditable coverage notice, request a copy from the employer or retiree group's benefits administrator or the employer or union.

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## **Section 9.4      What if you're in Medicare-certified hospice?**

Hospice and our plan do not cover the same drug at the same time. If you are enrolled in Medicare hospice and require certain drugs (e.g., anti-nausea drugs, laxatives, pain medication or anti-anxiety drugs) that are not covered by your hospice because they are unrelated to your terminal illness and related conditions, our plan must receive notification from either the prescriber or your hospice provider that the drugs are unrelated before our plan can cover the drugs. To prevent delays in receiving these drugs that should be covered by our plan, ask your hospice provider or prescriber to provide notification before your prescription is filled.

In the event you either revoke your hospice election or are discharged from hospice, our plan should cover your drugs as explained in this document. To prevent any delays at a pharmacy when your Medicare hospice benefit ends, bring documentation to the pharmacy to verify your revocation or discharge.

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## **Section 10      Programs on drug safety and managing medications**

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### **Section 10.1      Programs to help members use drugs safely**

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We conduct drug use reviews for our members to help make sure that they are getting safe and appropriate care.

We do a review each time you fill a prescription. We also review our records on a regular basis. During these reviews, we look for potential problems such as:

- ☐ Possible medication errors
- ☐ Drugs that may not be necessary because you are taking another drug to treat the same condition
- ☐ Drugs that may not be safe or appropriate because of your age or gender
- ☐ Certain combinations of drugs that could harm you if taken at the same time
- ☐ Prescriptions for drugs that have ingredients you are allergic to
- ☐ Possible errors in the amount (dosage) of a drug you are taking
- ☐ Unsafe amounts of opioid pain medications



If we see a possible problem in your use of medications, we will work with your provider to correct the problem.

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## **Section 10.2      Drug Management Program (DMP) to help members safely use their opioid medications**

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We have a program that helps make sure members safely use prescription opioids and other frequently abused medications. This program is called a Drug Management Program (DMP). If you use opioid medications that you get from several doctors or pharmacies, or if you had a recent opioid overdose, we may talk to your doctors to make sure your use of opioid medications is appropriate and medically necessary. Working with your doctors, if we decide your use of prescription opioid or benzodiazepine medications may not be safe, we may limit how you can get those medications. If we place you in our DMP, the limitations may be:

- ☐ Requiring you to get all your prescriptions for opioid or benzodiazepine medications from a certain pharmacy(ies)
- ☐ Requiring you to get all your prescriptions for opioid or benzodiazepine medications from a certain doctor(s)
- ☐ Limiting the amount of opioid or benzodiazepine medications we will cover for you

If we plan on limiting how you may get these medications or how much you can get, we will send you a letter in advance. The letter will tell you if we will limit coverage of these drugs for you, or if you'll be required to get the prescriptions for these drugs only from a specific doctor or pharmacy. You will have an opportunity to tell us which doctors or pharmacies you prefer to use, and about any other information you think is important for us to know. After you've had the opportunity to respond, if we decide to limit your coverage for these medications, we will send you another letter confirming the limitation. If you think we made a mistake or you disagree with our decision or with the limitation, you and your prescriber have the right to appeal. If you appeal, we will review your case and give you a decision. If we continue to deny any part of your request related to the limitations that apply to your access to medications, we will automatically send your case to an independent reviewer outside of our plan. See Chapter 9 for information about how to ask for an appeal.

You will not be placed in our DMP if you have certain medical conditions, such as active cancer-related pain or sickle cell disease, you are receiving hospice, palliative, or end-of-life care, or live in a long-term care facility.

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## **Section 10.3      Medication Therapy Management (MTM) programs to help members manage their medications**

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We have programs that can help our members with complex health needs. One program is called a Medication Therapy Management (MTM) program. These programs are voluntary and free. A team of pharmacists and doctors developed the programs for us to help make sure that our members get the most benefit from the drugs they take.

Some members who take medications for different medical conditions and have high drug costs, or are in a DMP to help members use their opioids safely, may be able to get services through an



MTM program. If you qualify for the program, a pharmacist or other health professional will give you a comprehensive review of all your medications. During the review, you can talk about your medications, your costs, and any problems or questions you have about your prescription and over-the-counter medications. You'll get a written summary which has a recommended to-do list that includes steps you should take to get the best results from your medications. You'll also get a medication list that will include all the medications you're taking, how much you take, and when and why you take them. In addition, members in the MTM program will receive information on the safe disposal of prescription medications that are controlled substances.

It's a good idea to talk to your doctor about your recommended to-do list and medication list. Bring the summary with you to your visit or anytime you talk with your doctors, pharmacists, and other health care providers. Also, keep your medication list up to date and keep it with you (for example, with your ID) in case you go to the hospital or emergency room.

If we have a program that fits your needs, we will automatically enroll you in the program and send you information. If you decide not to participate, please notify us and we will withdraw you. If you have any questions about these programs, please contact Customer Service.

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## **Section 11      We send you reports that explain payments for your drugs and which payment stage you are in**

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### **Section 11.1      We send you a monthly report called the "Part D Explanation of Benefits" (the "Part D EOB")**

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Our plan keeps track of the costs of your prescription drugs and the payments you have made when you get your prescriptions filled or refilled at the pharmacy. This way, we can tell you when you have moved from one drug payment stage to the next. In particular, there are two types of costs we keep track of:

- ☐ We keep track of how much you have paid. This is called your "out-of-pocket" cost.
- ☐ We keep track of your "total drug costs." This is the amount you pay out-of-pocket or others pay on your behalf plus the amount paid by the plan.

Our plan will prepare a written report called the Part D Explanation of Benefits (it is sometimes called the "Part D EOB") when you have had one or more prescriptions filled through the plan during the previous month. The Part D EOB provides more information about the drugs you take, such as increases in price and other drugs with lower cost-sharing that may be available. You should consult with your prescriber about these lower cost options. It includes:

- ☐ Information for that month. This report gives the payment details about the prescriptions you have filled during the previous month. It shows the total drug costs, what the plan paid, and what you and others on your behalf paid.
- ☐ Totals for the year since January 1. This is called "year-to-date" information. It shows you the total drug costs and total payments for your drugs since the year began.
- ☐ Drug price information. This information will display cumulative percentage increases for each prescription claim.

- ☐ Available lower cost alternative prescriptions. This will include information about other drugs with lower cost-sharing for each prescription claim that may be available.

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### **Section 11.2      Help us keep our information about your drug payments up to date**

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To keep track of your drug costs and the payments you make for drugs, we use records we get from pharmacies. Here is how you can help us keep your information correct and up to date:

- ☐ Show your UnitedHealthcare member ID card when you get a prescription filled. To make sure we know about the prescriptions you are filling and what you are paying, show your UnitedHealthcare member ID card every time you get a prescription filled.
- ☐ Make sure we have the information we need. There are times you may pay for prescription drugs when we will not automatically get the information we need to keep track of your out-of-pocket costs. To help us keep track of your out-of-pocket costs, you may give us copies of receipts for drugs that you have purchased. (If you are billed for a covered drug, you can ask our plan to pay our share of the cost for the drug. For instructions on how to do this, go to Chapter 7, Section 2 of this booklet.) Here are some types of situations when you may want to give us copies of your drug receipts to be sure we have a complete record of what you have spent for your drugs:
  - ☐ When you purchase a covered drug at a network pharmacy at a special price or using a discount card that is not part of our plan's benefit.
  - ☐ When you made a copayment for drugs that are provided under a drug manufacturer patient assistance program.
  - ☐ Any time you have purchased covered drugs at out-of-network pharmacies or other times you have paid the full price for a covered drug under special circumstances.
- ☐ Send us information about the payments others have made for you. Payments made by certain other individuals and organizations also count toward your out-of-pocket costs and help qualify you for catastrophic coverage. For example, payments made by an AIDS drug assistance program (ADAP), the Indian Health Service, and most charities count toward your out-of-pocket costs. You should keep a record of these payments and send them to us so we can track your costs.
- ☐ Check the written report we send you. When you receive a Part D Explanation of Benefits (a Part D EOB) in the mail, please look it over to be sure the information is complete and correct.

If you think something is missing from the report, or you have any questions, please call us at Customer Service (phone numbers are printed on the cover of this booklet). Be sure to keep these reports. They are an important record of your drug expenses.

# **Chapter 6**

What you pay for  
your Part D prescription drugs



### **What you pay for your Part D prescription drugs**

Because you are eligible for Nebraska Department of Health and Human Services (Medicaid), you qualify for and are getting “Extra Help” from Medicare to pay for your prescription drug plan costs. As a member of UHC Dual Complete NE-V001 (HMO-POS D-SNP) getting “Extra Help” from Medicare, you don’t pay anything for covered Part D drugs. Your yearly deductible is \$0 and you don’t pay any copays or coinsurance.

**Important message about what you pay for vaccines** – Some vaccines are considered medical benefits. Other vaccines are considered Part D drugs. You can find these vaccines listed in the plan’s Drug List. Our plan covers most adult Part D vaccines at no cost to you. Refer to your plan’s Drug List or contact Customer Service for coverage and cost sharing details about specific vaccines.

Your membership in our plan will not be affected by your Extra Help. You’ll get the same coverage as someone not getting Extra Help.

### **What is Extra Help?**

The Extra Help program is offered by the Social Security Administration. It helps you save on prescription drug costs. This means you get help paying your prescription drug plan’s monthly premium, yearly deductible, and copays or coinsurance.

### **Can the amount I pay for covered Part D drugs change throughout the year?**

The amount you pay will stay the same throughout the year.

See Chapter 2 section 7 for more information about Extra Help and other programs that help people pay for their prescription drugs.

## **Chapter 7**

Asking us to pay our share of a bill  
you have received for covered medical  
services or drugs

## **Section 1                      Situations in which you should ask us to pay for your covered services or drugs**

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Our network providers bill the plan directly for your covered services and drugs. If you get a bill for the full cost of medical care or drugs you have received, you should send this bill to us so that we can pay it. When you send us the bill, we will look at the bill and decide whether the services should be covered. If we decide they should be covered, we will pay the provider directly.

**If you have already paid for a Medicare service or item covered by the plan**, you can ask our plan to pay you back (paying you back is often called “reimbursing” you). It is your right to be paid back by our plan whenever you’ve paid more than your share of the cost for medical services or drugs that are covered by our plan. There may be deadlines that you must meet to get paid back. Please see Section 2 of this chapter. When you send us a bill you have already paid, we will look at the bill and decide whether the services or drugs should be covered. If we decide they should be covered, we will pay you back for the services or drugs.

There may also be times when you get a bill from a provider for the full cost of medical care you have received or possibly for more than your share of cost-sharing as discussed in the document. First try to resolve the bill with the provider. If that does not work, send the bill to us instead of paying it. We will look at the bill and decide whether the services should be covered. If we decide they should be covered, we will pay the provider directly. If we decide not to pay it, we will notify the provider. You should never pay more than plan-allowed cost-sharing. If this provider is contracted you still have the right to treatment.

Here are examples of situations in which you may need to ask our plan to pay you back or to pay a bill you have received:

### **1. When you’ve received emergency or urgently needed medical care from a provider who is not in our plan’s network**

Outside the service area, you can receive emergency or urgently needed services from any provider in the United States, whether or not the provider is a part of our network. In these cases, ask the provider to bill the plan.

- ☐ If you pay the entire amount yourself at the time you receive the care, ask us to pay you back for our share of the cost. Send us the bill, along with documentation of any payments you have made.
- ☐ You may get a bill from the provider asking for payment that you think you do not owe. Send us this bill, along with documentation of any payments you have already made.
  - ☐ If the provider is owed anything, we will pay the provider directly.
  - ☐ If you have already paid more than your share of the cost for the service, we will determine how much you owed and pay you back for our share of the cost.

### **2. When a network provider sends you a bill you think you should not pay**

Network providers should always bill the plan directly. But sometimes they make mistakes and ask you to pay more than your share of the cost.

- ☐ If you are eligible for Medicare cost-sharing assistance under Medicaid, we do not allow providers to bill you for covered services. We pay our providers directly, and we protect you from any charges. This is true even if we pay the provider less than the provider charges for a service.
- ☐ Whenever you get a bill from a network provider that you think is more than you should pay, send us the bill. We will contact the provider directly and resolve the billing problem.
- ☐ If you have already paid a bill to a network provider, but you feel that you paid too much, send us the bill along with documentation of any payment you have made. You should ask us to pay you back for the difference between the amount you paid and the amount you owed under the plan.

### **3.If you are retroactively enrolled in our plan**

Sometimes a person's enrollment in the plan is retroactive. (This means that the first day of their enrollment has already passed. The enrollment date may even have occurred last year.)

If you were retroactively enrolled in our plan and you paid out-of-pocket for any of your covered services or drugs after your enrollment date, you can ask us to pay you back for our share of the costs. You will need to submit paperwork such as receipts and bills for us to handle the reimbursement.

### **4.When you use an out-of-network pharmacy to get a prescription filled**

If you go to an out-of-network pharmacy, the pharmacy may not be able to submit the claim directly to us. When that happens, you will have to pay the full cost of your prescription.

Save your receipt and send a copy to us when you ask us to pay you back for our share of the cost. Remember that we only cover out-of-network pharmacies in limited circumstances. See Chapter 5, Section 2.5 for a discussion of these circumstances.

### **5.When you pay the full cost for a prescription because you don't have your UnitedHealthcare member ID card with you**

If you do not have your UnitedHealthcare member ID card with you, you can ask the pharmacy to call the plan or to look up your plan enrollment information. However, if the pharmacy cannot get the enrollment information they need right away, you may need to pay the full cost of the prescription yourself.

Save your receipt and send a copy to us when you ask us to pay you back for our share of the cost.

### **6.When you pay the full cost for a prescription in other situations**

You may pay the full cost of the prescription because you find that the drug is not covered for some reason.

- ☐ For example, the drug may not be on the plan's **Drug List** or it could have a requirement or restriction that you didn't know about or don't think should apply to you. If you decide to get the drug immediately, you may need to pay the full cost for it.
- ☐ Save your receipt and send a copy to us when you ask us to pay you back. In some situations, we may need to get more information from your doctor in order to pay you back for our share of the cost of the drug.



### **7. When you utilize your worldwide emergency coverage, worldwide urgently needed services, or worldwide emergency transportation benefits**

You will pay the full cost of emergency services received outside of the United States at the time you receive services. To receive reimbursement from us, you must do the following:

- ☐ Pay your bill at the time it is received. We will reimburse you for the difference between the amount of your bill and your cost share for the services as outlined in Chapter 4 of this document.
- ☐ Save all of your receipts and send us copies when you ask us to pay you back. In some situations, we may need to get more information from you or the provider who rendered services to you in order to pay you back for our share of the cost. Please see Chapter 7 Section 2.1 for expense reimbursement for worldwide services.
- ☐ If you are being asked to pay your bill for worldwide emergency services and are unable to make the payment, please call Customer Service for additional assistance and we may be able to help coordinate payment for covered services on your behalf.

When you send us a request for payment, we will review your request and decide whether the service or drug should be covered. This is called making a “coverage decision.” If we decide it should be covered, we will pay for our share of the cost for the service or drug. If we deny your request for payment, you can appeal our decision. Chapter 9 of this document has information about how to make an appeal.

## **Section 2                      How to ask us to pay you back or to pay a bill you have received**

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You may request us to pay you back by sending us a request in writing. If you send a request in writing, send your bill and documentation of any payment you have made. It's a good idea to make a copy of your bill and receipt(s) for your records.

To make sure you are giving us all the information we need to make a decision, you can fill out our claim form to make your request for payment.

- ☐ You don't have to use the form, but it will help us process the information faster.
- ☐ Either download a copy of the form from our website ([myuhc.com/communityplan](https://myuhc.com/communityplan)) or call Customer Service and ask for the form.

Mail your request for payment together with any bills or paid receipts to us at this address:

Part D Prescription drug payment requests:

OptumRx

P.O. Box 650287

Dallas, TX 75265-0287

Medical claims payment requests:

UnitedHealthcare

P.O. Box 5240

Kingston, NY 12402-5240

**You must submit your Part C (medical) claim to us within 12 months** of the date you received the service, item, or Part B drug.

**You must submit your Part D (prescription drug) claim to us within 36 months** of the date you received the service, item, or drug.

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### **Section 3                      We will consider your request for payment and say yes or no**

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#### **Section 3.1                      We check to see whether we should cover the service or drug and how much we owe**

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When we receive your request for payment, we will let you know if we need any additional information from you. Otherwise, we will consider your request and make a coverage decision.

- ☐ If we decide that the medical care or drug is covered and you followed all the rules, we will pay for our share of the cost for the service. If you have already paid for the service or drug, we will mail your reimbursement of our share of the cost to you. If you have not paid for the service or drug yet, we will mail the payment directly to the provider.
- ☐ If we decide that the medical care or drug is **not** covered, or you did **not** follow all the rules, we will not pay for our share of the cost of the care or drug. We will send you a letter explaining the reasons why we are not sending the payment and your rights to appeal that decision.

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#### **Section 3.2                      If we tell you that we will not pay for all or part of the medical care or drug, you can make an appeal**

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If you think we have made a mistake in turning down your request for payment or the amount we are paying, you can make an appeal. If you make an appeal, it means you are asking us to change the decision we made when we turned down your request for payment. The appeals process is a formal process with detailed procedures and important deadlines. For details on how to make this appeal, go to Chapter 9 of this document.

# **Chapter 8**

## Your rights and responsibilities

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## **Section 1            Our plan must honor your rights and cultural sensitivities as a member of the plan**

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### **Section 1.1            You have a right to receive information about the organization, its services, its practitioners and providers and member rights and responsibilities. We must provide information in a way that works for you and consistent with your cultural sensitivities (in languages other than English, in braille, in large print, or other alternate formats, etc.)**

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Your plan is required to ensure that all services, both clinical and non-clinical, are provided in a culturally competent manner and are accessible to all enrollees, including those with limited English proficiency, limited reading skills, hearing incapacity, or those with diverse cultural and ethnic backgrounds. Examples of how a plan may meet these accessibility requirements include, but are not limited to provision of translator services, interpreter services, teletypewriters, or TTY (text telephone or teletypewriter phone) connection.

Our plan has free interpreter services available to answer questions from non-English speaking members. We can also give you information in braille, in large print, or other alternate formats at no cost if you need it. We are required to give you information about the plan's benefits in a format that is accessible and appropriate for you. To get information from us in a way that works for you, please call Customer Service.

Our plan is required to give female enrollees the option of direct access to a women's health specialist within the network for women's routine and preventive health care services.

If providers in the plan's network for a specialty are not available, it is the plan's responsibility to locate specialty providers outside the network who will provide you with the necessary care. In this case, you will only pay in-network cost-sharing. If you find yourself in a situation where there are no specialists in the plan's network that cover a service you need, call the plan for information on where to go to obtain this service at in-network cost-sharing.

If you have any trouble getting information from our plan in a format that is accessible and appropriate for you, seeing a women's health specialists or finding a network specialist, please call to file a grievance with Customer Service (phone numbers are printed on the cover of this booklet). You may also file a complaint with Medicare by calling 1-800-MEDICARE (1-800-633-4227) or directly with the Office for Civil Rights 1-800-368-1019 or TTY 1-800-537-7697.

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### **Sección 1.1            Usted tiene derecho a recibir información sobre la organización, sus servicios, sus profesionales del cuidado de la salud y proveedores, además de los derechos y las responsabilidades de los miembros. Debemos brindarle información útil y en otros idiomas aparte del inglés, en braille, en letras grandes o en otros formatos alternativos**

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Para recibir información nuestra de una forma que le resulte conveniente, llame a Servicio al Cliente (los números de teléfono aparecen en la portada de esta guía).

Nuestro plan cuenta con personas y servicios gratuitos de intérpretes para responder las preguntas de los miembros discapacitados y los que no hablan inglés. Esta información está disponible sin costo en otros idiomas. También podemos proporcionarle información en braille, en letras grandes o en otros formatos alternativos sin costo, si es necesario. Se nos exige que le proporcionemos la información sobre los beneficios del plan en un formato que sea accesible y apropiado para usted. Para recibir información nuestra de una forma que le resulte conveniente, llame a Servicio al Cliente (los números de teléfono aparecen en la portada de esta guía) o comuníquese con nuestro Coordinador de Derechos Civiles.

Si tiene alguna dificultad para obtener información de nuestro plan en un formato que sea accesible y apropiado para usted, llame a Servicio al Cliente para presentar una queja formal (los números de teléfono aparecen en la portada de esta guía). También puede presentar una queja ante Medicare si llama al 1-800-MEDICARE (1-800-633-4227) o directamente ante la Oficina de Derechos Civiles. La información de contacto se incluye en esta **Evidencia de Cobertura** o con esta correspondencia o, para obtener información adicional, puede comunicarse con Servicio al Cliente.

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## **Section 1.2            We must ensure that you get timely access to your covered services and drugs**

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You have the right to choose a primary care provider (PCP) in the plan's network to provide and arrange for your covered services (Chapter 3 explains more about this). You also have the right to go to a women's health specialist (such as a gynecologist) without a referral.

You have the right to get appointments and covered services from the plan's network of providers, **within a reasonable amount of time**. This includes the right to get timely services from specialists when you need that care. You also have the right to get your prescriptions filled or refilled at any of our network pharmacies without long delays.

### **How to Receive Care After Hours**

If you need to talk to or see your Primary Care Provider after the office has closed for the day, call your Primary Care Provider's office. When the on-call physician returns your call he or she will advise you on how to proceed.

If you think that you are not getting your medical care or Part D drugs within a reasonable amount of time, Chapter 9 tells what you can do.

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## **Section 1.3            We must protect the privacy of your personal health information**

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Federal and state laws protect the privacy of your medical records and personal health information. We protect your personal health information as required by these laws.

- ☐ Your "personal health information" includes the personal information you gave us when you enrolled in this plan as well as your medical records and other medical and health information.

- ☐ You have rights related to your information and controlling how your health information is used. We give you a written notice, called a “Notice of Privacy Practice,” that tells about these rights and explains how we protect the privacy of your health information.

**How do we protect the privacy of your health information?**

- ☐ We make sure that unauthorized people don’t see or change your records.
- ☐ Except for the circumstances noted below, if we intend to give your health information to anyone who isn’t providing your care or paying for your care, we are required to get written permission from you or someone you have given legal power to make decisions for you first.
- ☐ There are certain exceptions that do not require us to get your written permission first. These exceptions are allowed or required by law.
  - ☐ We are required to release health information to government agencies that are checking on quality of care.
  - ☐ Because you are a member of our plan through Medicare, we are required to give Medicare your health information including information about your Part D prescription drugs. If Medicare releases your information for research or other uses, this will be done according to Federal statutes and regulations; typically, this requires that information that uniquely identifies you not be shared

**You can see the information in your records and know how it has been shared with others**

You have the right to look at your medical records held at the plan, and to get a copy of your records. We are allowed to charge you a fee for making copies. You also have the right to ask us to make additions or corrections to your medical records. If you ask us to do this, we will work with your health care provider to decide whether the changes should be made.

You have the right to know how your health information has been shared with others for any purposes that are not routine.

If you have questions or concerns about the privacy of your personal health information, please call Customer Service.

**HEALTH PLAN NOTICES OF PRIVACY PRACTICES**

**THIS NOTICE DESCRIBES HOW MEDICAL INFORMATION ABOUT YOU MAY BE USED AND DISCLOSED AND HOW YOU CAN GET ACCESS TO THIS INFORMATION. PLEASE REVIEW IT CAREFULLY.**

Effective January 1, 2024

By law, we<sup>1</sup> must protect the privacy of your health information (“HI”). We must send you this notice. It tells you:

- ☐ How we may use your HI.
- ☐ When we can share your HI with others.
- ☐ What rights you have for your HI.

By law, we must follow the terms of our current notice.

HI is information about your health or medical services. We have the right to make changes to this notice of privacy practices. If we make important changes, we will notify you by mail or e-mail. We

will also post the new notice on our website. Any changes to the notice will apply to all HI we have. We will notify you of a breach of your HI.

### **How We Collect, Use, and Share Your Information**

#### **We collect, use and share your HI with:**

- ☐ You or your legal or personal representative.
- ☐ Certain Government agencies. To check to make sure we are following privacy laws.

**We have the right to collect, use and share your HI for certain purposes.** This may be for your treatment, to pay for your care, or to run our business. We may use and share your HI as follows.

- ☐ **For Payment.** To process payments and pay claims. For example, we may tell a doctor whether we will pay for certain medical procedures and what percentage of the bill may be covered.
- ☐ **For Treatment or Managing Care.** To help with your care. For example, we may share your HI with a hospital you are in, to help them provide medical care to you.
- ☐ **For Health Care Operations.** To run our business. For example, we may talk to your doctor to tell him or her about a special disease management or wellness program available to you. We may study data to improve our services.
- ☐ **To Tell You about Health Programs or Products.** We may tell you about other treatments, products, and services. These activities may be limited by law.
- ☐ **For Plan Sponsors.** If you receive health insurance through your employer, we may give enrollment, disenrollment, and summary HI to your employer. We may give them other HI if they properly limit its use.
- ☐ **For Underwriting Purposes.** To make health insurance underwriting decisions. We will not use your genetic information for underwriting purposes.
- ☐ **For Reminders on Benefits or Care.** We may send reminders about appointments you have and information about your health benefits.
- ☐ **For Communications to You.** We may contact you about your health insurance benefits, healthcare or payments.

#### **We may collect, use, and share your HI as follows.**

- ☐ **As Required by Law.** To follow the laws that apply to us.
- ☐ **To Persons Involved with Your Care.** A family member or other person that helps with your medical care or pays for your care. This also may be to a family member in an emergency. This may happen if you are unable to tell us if we can share your HI or not. If you are unable to tell us what you want, we will use our best judgment. If allowed, after you pass away, we may share HI with family members or friends who helped with your care or paid for your care.
- ☐ **For Public Health Activities.** For example, to prevent diseases from spreading or to report problems with products or medicines.



- ☐ **For Reporting Abuse, Neglect or Domestic Violence.** We may only share with certain entities allowed by law to get this HI. This may be a social or protective service agency.
- ☐ **For Health Oversight Activities** to an agency allowed by the law to get the HI. This may be for licensure, audits and fraud and abuse investigations.
- ☐ **For Judicial or Administrative Proceedings,** for example, to answer a court order or subpoena.
- ☐ **For Law Enforcement.** To find a missing person or report a crime.
- ☐ **For Threats to Health or Safety.** To public health agencies or law enforcement, for example, in an emergency or disaster.
- ☐ **For Government Functions.** For military and veteran use, national security, or certain protection services.
- ☐ **For Workers' Compensation.** If you were hurt at work or to comply with employment laws.
- ☐ **For Research.** For example, to study a disease or medical condition. We also may use HI to help prepare a research study.
- ☐ **To Give Information on Decedents.** For example, to a coroner or medical examiner who may help identify the person who died, why they died, or to meet certain laws. We also may give HI to funeral directors.
- ☐ **For Organ Transplant.** For example, to help get, store or transplant organs, eyes or tissues.
- ☐ **To Correctional Institutions or Law Enforcement.** For persons in custody, for example: (1) to give health care; (2) to protect your health and the health of others; and (3) for the security of the institution.
- ☐ **To Our Business Associates.** To give you services, if needed. These are companies that provide services to us. They agree to protect your HI.
- ☐ **Other Restrictions.** Federal and state laws may further limit our use of the HI listed below. We will follow stricter laws that apply.
  1. Alcohol and Substance Use Disorder
  2. Biometric Information
  3. Child or Adult Abuse or Neglect, including Sexual Assault
  4. Communicable Diseases
  5. Genetic Information
  6. HIV/AIDS
  7. Mental Health
  8. Minors' Information
  9. Prescriptions
  10. Reproductive Health
  11. Sexually Transmitted Diseases

We will only use or share your HI as described in this notice or with your written consent. We will get your written consent to share psychotherapy notes about you, except in certain cases allowed by law. We will get your written consent to sell your HI to other people. We will get your written consent to use your HI in certain marketing mailings. If you give us your consent, you may take it back. To find out how, call the phone number on your health insurance ID card.

## Your Rights

You have the following rights for your medical information.

- ☐ **To ask us to limit** our use or sharing for treatment, payment, or health care operations. You can ask to limit sharing with family members or others that help with your care or pay for your care. We may allow your dependents to ask for limits. **We will try to honor your request, but we do not have to do so.** Your request to limit our use or sharing must be made in writing.
- ☐ **To ask to get confidential communications** in a different way or place. For example, at a P.O. Box instead of your home. We will agree to your request as allowed by state and federal law. We take verbal requests but may ask you to confirm your request in writing. You can change your request. This must be in writing. Mail it to the address below.
- ☐ **To see or get a copy** of certain HI. You must ask in writing. Mail it to the address below. If we keep these records in electronic form, you can request an electronic copy. We may send you a summary. We may charge for copies. We may deny your request. If we deny your request, you may have the denial reviewed.
- ☐ **To ask to amend.** If you think your HI is wrong or incomplete you can ask to change it. You must ask in writing. You must give the reasons for the change. We will respond to your request in the time we must do so under the law. Mail this to the address below. If we deny your request, you may add your disagreement to your HI.
- ☐ **To get an accounting** of when we shared your HI in the six years prior to your request. This will not include when we shared HI for the following reasons: (i) For treatment, payment, and health care operations; (ii) With you or with your consent; (iii) With correctional institutions or law enforcement. This will not list the disclosures that federal law does not require us to track.
- ☐ **To get a paper copy of this notice.** You may ask for a paper copy at any time. You may also get a copy at our website.
- ☐ **In certain states, you may have the right to ask that we delete** your HI. Depending on where you live, you may be able to ask us to delete your HI. We will respond to your request in the time we must do so under the law. If we can't, we will tell you. If we can't, you can write us, noting why you disagree and send us the correct information.

## Using Your Rights

- ☐ **To Contact your Health Plan.** If you have questions about this notice, or you want to use your rights, **call the phone number on your ID card.** Or you may contact the UnitedHealth Group Call Center at 1-844-368-7149, or TTY/RTT 711.
- ☐ **To Submit a Written Request.** Mail to:

UnitedHealthcare Privacy Office  
MN017-E300  
PO Box 1459  
Minneapolis MN 55440

- ☐ **To File a Complaint.** If you think your privacy rights have been violated, you may send a complaint at the address above.

**You may also notify the Secretary of the U.S. Department of Health and Human Services.** We will not take any action against you for filing a complaint.

<sup>1</sup> This Medical Information Notice of Privacy Practices applies to health plans that are affiliated with UnitedHealth Group. For a current list of health plans subject to this notice go to <https://www.uhc.com/privacy/entities-fn-v2>.

## **FINANCIAL INFORMATION PRIVACY NOTICE**

**THIS NOTICE SAYS HOW YOUR FINANCIAL INFORMATION MAY BE USED AND SHARED. REVIEW IT CAREFULLY.**

Effective January 1, 2024

We<sup>2</sup> protect your “personal financial information” (“FI”). FI is non-health information. FI identifies you and is generally not public.

### **Information We Collect**

- ☐ We get FI from your applications or forms. This may be name, address, age and social security number.
- ☐ We get FI from your transactions with us or others. This may be premium payment data.

### **Sharing of FI**

We will only share FI as permitted by law.

We may share your FI to run our business. We may share your FI with our Affiliates. We do not need your consent to do so.

- ☐ We may share your FI to process transactions.
- ☐ We may share your FI to maintain your account(s).
- ☐ We may share your FI to respond to court orders and legal investigations.
- ☐ We may share your FI with companies that prepare our marketing materials.

### **Confidentiality and Security**

We limit employee and service provider access to your FI. We have safeguards in place to protect your FI.

### **Questions About This Notice**

Please **call the toll-free member phone number on health plan ID card** or contact the UnitedHealth Group Customer Call Center at 1-844-368-7149, or TTY/RTT 711.

<sup>2</sup> For purposes of this Financial Information Privacy Notice, “we” or “us” refers to health plans affiliated with UnitedHealth Group, and the following UnitedHealthcare affiliates: ACN Group of California, Inc.; AmeriChoice Corporation.; Benefitter Insurance Solutions, Inc.; Claims Management Systems, Inc.; Dental Benefit Providers, Inc.; Ear Professional International Corporation; Excelsior Insurance Brokerage, Inc.; gethealthinsurance.com Agency, Inc. Golden Outlook, Inc.; Golden Rule Insurance Company; HealthMarkets Insurance Agency; Healthplex of CT, Inc.; Healthplex of NJ, Inc.; Healthplex, Inc.; HealthSCOPE Benefits, Inc.; International Healthcare Services, Inc.; Level2 Health IPA, LLC; Level2 Health Holdings, Inc.; Level2 Health Management, LLC; Managed Physical Network, Inc.; Optum Care Networks, Inc.; Optum Health Care Solutions, Inc.; Optum Health Networks, Inc.; Oxford Benefit Management, Inc.; Oxford Health Plans LLC; Physician Alliance of the Rockies, LLC; POMCO Network, Inc.; POMCO, Inc.; Real Appeal, LLC; Solstice Administrators of Alabama, Inc.; Solstice Administrators of Missouri, Inc.; Solstice Administrators of North Carolina, Inc.; Solstice Administrators, Inc.; Solstice Benefit Services, Inc.; Solstice of Minnesota, Inc.; Solstice of New York, Inc.; Spectera, Inc.; Three Rivers Holdings, Inc.; UHIC Holdings, Inc.; UMR, Inc.; United Behavioral Health; United Behavioral Health of New York I.P.A., Inc.; UnitedHealthcare, Inc.; United HealthCare Services, Inc.; UnitedHealth Advisors, LLC; UnitedHealthcare Service LLC; Urgent Care MSO, LLC; USHEALTH Administrators, LLC; and USHEALTH Group, Inc.; and Vivify Health, Inc. This Financial Information Privacy Notice only applies where required by law. Specifically, it does not apply to (1) health care insurance products offered in Nevada by Health Plan of Nevada, Inc. and Sierra Health and Life Insurance Company, Inc.; or (2) other UnitedHealth Group health plans in states that provide exceptions. For a current list of health plans subject to this notice go to <https://www.uhc.com/privacy/entities-fn-v2>.

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## **Section 1.4      We must give you information about the plan, its network of providers, and your covered services**

As a member of our plan, you have the right to get several kinds of information from us. We may also call you occasionally to let you know about other Medicare products and services we offer. Call Customer Service if you want to opt out of receiving these calls or want any of the following kinds of information:

- ☐ **Information about our plan.** This includes, for example, information about the plan’s financial condition.
- ☐ **Information about our network providers and pharmacies.**

- ☐ You have the right to get information about the qualifications of the providers and pharmacies in our network and how we pay the providers in our network.
- ☐ **Information about your coverage and the rules you must follow when using your coverage.** Chapters 3 and 4 provide information regarding medical services. Chapters 5 and 6 provide information about Part D prescription drug coverage.
- ☐ **Information about why something is not covered and what you can do about it.** Chapter 9 provides information on asking for a written explanation on why a medical service or Part D drug is not covered or if your coverage is restricted. Chapter 9 also provides information on asking us to change a decision, also called an appeal.

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**Section 1.5      You have a right to participate with practitioners in making decisions about your health care. We must support your right to make decisions about your care and a candid discussion of appropriate or medically necessary treatment options for your conditions, regardless of cost or benefit coverage.**

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**You have the right to know your treatment options and participate in decisions about your health care**

You have the right to get full information from your doctors and other health care providers. Your providers must explain your medical condition and your treatment choices **in a way that you can understand**.

You also have the right to participate fully in decisions about your health care. To help you make decisions with your doctors about what treatment is best for you, your rights include the following:

- ☐ **To know about all of your choices.** You have the right to be told about all of the treatment options that are recommended for your condition, no matter what they cost or whether they are covered by our plan. It also includes being told about programs our plan offers to help members manage their medications and use drugs safely.
- ☐ **To know about the risks.** You have the right to be told about any risks involved in your care. You must be told in advance if any proposed medical care or treatment is part of a research experiment. You always have the choice to refuse any experimental treatments.
- ☐ **The right to say “no.”** You have the right to refuse any recommended treatment. This includes the right to leave a hospital or other medical facility, even if your doctor advises you not to leave. You also have the right to stop taking your medication. Of course, if you refuse treatment or stop taking medication, you accept full responsibility for what happens to your body as a result.

**You have the right to give instructions about what is to be done if you are not able to make medical decisions for yourself**

Sometimes people become unable to make health care decisions for themselves due to accidents or serious illness. You have the right to say what you want to happen if you are in this situation. This means that, **if you want to**, you can:

- ☐ Fill out a written form to give **someone the legal authority to make medical decisions for you** if you ever become unable to make decisions for yourself.

- ☐ **Give your doctors written instructions** about how you want them to handle your medical care if you become unable to make decisions for yourself.

The legal documents that you can use to give your directions in advance in these situations are called “**advance directives**.” There are different types of advance directives and different names for them. Documents called “**living will**” and “**power of attorney for health care**” are examples of advance directives.

If you want to use an “advance directive” to give your instructions, here is what to do:

- ☐ **Get the form.** You can get an advance directive form from your lawyer, from a social worker, or from some office supply stores. You can sometimes get advance directive forms from organizations that give people information about Medicare. You can also contact Customer Service for assistance in locating an advanced directive form.
- ☐ **Fill it out and sign it.** Regardless of where you get this form, keep in mind that it is a legal document. You should consider having a lawyer help you prepare it.
- ☐ **Give copies to appropriate people.** You should give a copy of the form to your doctor and to the person you name on the form who can make decisions for you if you can’t. You may want to give copies to close friends or family members. Keep a copy at home.

If you know ahead of time that you are going to be hospitalized, and you have signed an advance directive, **take a copy with you to the hospital.**

- ☐ The hospital will ask you whether you have signed an advance directive form and whether you have it with you.
- ☐ If you have not signed an advance directive form, the hospital has forms available and will ask if you want to sign one.

**Remember, it is your choice whether you want to fill out an advance directive** (including whether you want to sign one if you are in the hospital). According to law, no one can deny you care or discriminate against you based on whether or not you have signed an advance directive.

#### **What if your instructions are not followed?**

If you have signed an advance directive, and you believe that a doctor or hospital did not follow the instructions in it, you may file a complaint with the appropriate state-specific agency, for example, your State Department of Health. See Chapter 2, Section 3 for contact information regarding your state-specific agency.

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### **Section 1.6      You have a right to voice complaints or appeals about the organization or the care it provides. You have the right to make complaints and to ask us to reconsider decisions we have made**

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If you have any problems, concerns, or complaints and need to request coverage, or make an appeal, Chapter 9 of this document tells what you can do.

Whatever you do – ask for a coverage decision, make an appeal, or make a complaint – **we are required to treat you fairly.**



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## **Section 1.7      What can you do if you believe you are being treated unfairly or your rights are not being respected?**

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### **If it is about discrimination, call the Office for Civil Rights**

If you believe you have been treated unfairly or your rights have not been respected due to your race, disability, religion, sex, health, ethnicity, creed (beliefs), age, sexual orientation, or national origin, you should call the Department of Health and Human Services' **Office for Civil Rights** at 1-800-368-1019 or TTY 1-800-537-7697, or call your local Office for Civil Rights.

### **Is it about something else?**

If you believe you have been treated unfairly or your rights have not been respected, **and it's not** about discrimination, you can get help dealing with the problem you are having:

- ☐ You can **call Customer Service**.
- ☐ You can **call the SHIP**. For details, go to Chapter 2, Section 3.
- ☐ Or, **you can call Medicare** at 1-800-MEDICARE (1-800-633-4227), 24 hours a day, 7 days a week (TTY 1-877-486-2048).

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## **Section 1.8      You have a right to make recommendations regarding the organization's member rights and responsibilities policy. How to get more information about your rights**

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There are several places where you can get more information about your rights:

- ☐ You can **call Customer Service**.
- ☐ For information on the quality program for your specific health plan, call Customer Service. You may also access this information via the website ([uhcmedicaresolutions.com/resources/mapdp-information-forms.html](https://uhcmedicaresolutions.com/resources/mapdp-information-forms.html)). Select, "Commitment to Quality."
- ☐ You can **call the SHIP**. For details, go to Chapter 2, Section 3.
- ☐ You can contact **Medicare**.
  - ☐ You can visit the Medicare website to read or download the publication "Medicare Rights & Protections." (The publication is available at: [medicare.gov/Pubs/pdf/11534-Medicare-Rights-and-Protections.pdf](https://medicare.gov/Pubs/pdf/11534-Medicare-Rights-and-Protections.pdf))
  - ☐ Or, you can call 1-800-MEDICARE (1-800-633-4227), 24 hours a day, 7 days a week (TTY 1-877-486-2048).

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## **Section 2      You have some responsibilities as a member of the plan**

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Things you need to do as a member of the plan are listed below. If you have any questions, please call Customer Service.

- ☐ **Get familiar with your covered services and the rules you must follow to get these covered services.** Use this **Evidence of Coverage** to learn what is covered for you and the rules you need to follow to get your covered services.
  - ☐ Chapters 3 and 4 give the details about your medical services.



- ☐ Chapters 5 and 6 give the details about your Part D prescription drug coverage.
- ☐ **If you have any other health insurance coverage or prescription drug coverage in addition to our plan, you are required to tell us.** Chapter 1 tells you about coordinating these benefits.
- ☐ **Tell your doctor and other health care providers that you are enrolled in our plan.** Show your UnitedHealthcare member ID card and Medicaid card whenever you get your medical care or Part D prescription drugs.
- ☐ **Help your doctors and other providers help you by giving them information, asking questions, and following through on your care.**
  - ☐ To help get the best care, tell your doctors and other health providers about your health problems. Follow the treatment plans and instructions that you and your doctors agree upon.
  - ☐ Make sure your doctors know all of the drugs you are taking, including over-the-counter drugs, vitamins, and supplements.
  - ☐ If you have any questions, be sure to ask and get an answer you can understand.
- ☐ **Be considerate.** We expect all our members to respect the rights of other patients. We also expect you to act in a way that helps the smooth running of your doctor's office, hospitals, and other offices.
- ☐ **Pay what you owe.** As a plan member, you are responsible for these payments:
  - ☐ In order to be eligible for our plan, you must have Medicare Part A and Medicare Part B. For most Plan members, Medicaid pays for your Part A premium (if you don't qualify for it automatically) and for your Part B premium. If Medicaid is not paying your Medicare premiums for you, you must continue to pay your Medicare premiums to remain a member of the plan.
  - ☐ For most of your medical services covered by the plan, you must pay your share of the cost when you get the service.
  - ☐ If you are required to pay a late enrollment penalty, you must pay the penalty to keep your prescription drug coverage.
  - ☐ If you are required to pay the extra amount for Part D because of your higher income (as reported on your last tax return), you must continue to pay the extra amount directly to the government to remain a member of the plan.
  - ☐ **If you move outside of our plan service area, you cannot remain a member of our plan.**
  - ☐ **If you move within our plan service area, we need to know** so we can keep your membership record up to date and know how to contact you.
  - ☐ If you move, it is also important to tell Social Security (or the Railroad Retirement Board).

## **Chapter 9**

What to do if you have a problem or complaint (coverage decisions, appeals, complaints)

## **Section 1 Introduction**

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### **Section 1.1 What to do if you have a problem or concern**

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This chapter explains the processes for handling problems and concerns. The process you use to handle your problem depends on two things:

1. Whether your problem is about benefits covered by **Medicare** or **Medicaid**. If you would like help deciding whether to use the Medicare process or the Medicaid process, or both, please contact Customer Service.
2. The type of problem you are having:
  - ☐ For some problems, you need to use the **process for coverage decisions and appeals**.
  - ☐ For other problems, you need to use the **process for making complaints**; also called grievances.

These processes have been approved by Medicare. Each process has a set of rules, procedures, and deadlines that must be followed by us and by you.

The guide in Section 3 will help you identify the right process to use and what you should do.

### **Section 1.2 What about the legal terms?**

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There are legal terms for some of the rules, procedures, and types of deadlines explained in this chapter. Many of these terms are unfamiliar to most people and can be hard to understand. To make things easier, this chapter:

- ☐ Uses simpler words in place of certain legal terms. For example, this chapter generally says “making a complaint” rather than “filing a grievance,” “coverage decision” rather than “organization determination” or “coverage determination” or “at-risk determination” and “independent review organization” instead of “Independent Review Entity.”
- ☐ It also uses abbreviations as little as possible.

However, it can be helpful – and sometimes quite important – for you to know the correct legal terms. Knowing which terms to use will help you communicate more accurately to get the right help or information for your situation. To help you know which terms to use, we include legal terms when we give the details for handling specific types of situations.

## **Section 2 Where to get more information and personalized assistance**

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We are always available to help you. Even if you have a complaint about our treatment of you, we are obligated to honor your right to complain. Therefore, you should always reach out to customer service for help. But in some situations, you may also want help or guidance from someone who is not connected with us. Below are two entities that can assist you.

### **State Health Insurance Assistance Program (SHIP)**

Each state has a government program with trained counselors. The program is not connected with us or with any insurance company or health plan. The counselors at this program can help you

understand which process you should use to handle a problem you are having. They can also answer your questions, give you more information, and offer guidance on what to do.

The services of SHIP counselors are free. You will find phone numbers in Chapter 2, Section 3 of this document.

### **Medicare**

You can also contact Medicare to get help. To contact Medicare:

- ☐ You can call 1-800-MEDICARE (1-800-633-4227), 24 hours a day, 7 days a week. TTY users should call 1-877-486-2048.
- ☐ You can also visit the Medicare website ([www.medicare.gov](http://www.medicare.gov)).

### **You can get help and information from Nebraska Department of Health and Human Services (Medicaid)**

For more information and help in handling a problem, you can also contact Nebraska Department of Health and Human Services (Medicaid). Here are two ways to get information directly from Nebraska Department of Health and Human Services (Medicaid):

- ☐ You can call 1-402-471-3121. TTY users should call 1-800-471-7352.
- ☐ You can visit the Nebraska Department of Health and Human Services (Medicaid) website ([dhhs.ne.gov/Pages/medicaid-and-long-term-care.aspx](http://dhhs.ne.gov/Pages/medicaid-and-long-term-care.aspx)).

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## **Section 3      To deal with your problem, which process should you use?**

Because you have Medicare and get assistance from Medicaid, you have different processes that you can use to handle your problem or complaint. Which process you use depends on whether the problem is about Medicare benefits or Medicaid benefits. If your problem is about a benefit covered by Medicare, then you should use the Medicare process. If your problem is about a benefit covered by Medicaid, then you should use the Medicaid process. If you would like help deciding whether to use the Medicare process or the Medicaid process, please contact Customer Service.

The Medicare process and Medicaid process are described in different parts of this chapter. To find out which part you should read, use the chart below.

### **Is your problem about Medicare benefits or Medicaid benefits?**

If you would like help deciding whether your problem is about Medicare benefits or Medicaid benefits, please contact Customer Service.

My problem is about **Medicare** benefits.

Go to the next section of this chapter, **Section 4, “Handling problems about your Medicare benefits.”**

My problem is about **Medicaid** coverage.

Skip ahead to **Section 12** of this chapter, **“Handling problems about your Medicaid benefits.”**

## Problems about your Medicare benefits

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### Section 4 Handling problems about your Medicare benefits

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#### Section 4.1 Should you use the process for coverage decisions and appeals? Or should you use the process for making complaints?

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If you have a problem or concern, you only need to read the parts of this chapter that apply to your situation. The chart below will help you find the right section of this chapter for problems or complaints about **benefits covered by Medicare**.

To figure out which part of this chapter will help with your problem or concern, about your **Medicare** benefits, use this chart

#### Is your problem or concern about your benefits or coverage?

This includes problems about whether medical care (medical items, services and/or Part B prescription drugs) are covered or not, the way they are covered, and problems related to payment for medical care.

#### Yes.

Go on to the next section of this chapter, **Section 5, “A guide to the basics of coverage decisions and appeals.”**

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#### No.

Skip ahead to **Section 11** at the end of this chapter: **“How to make a complaint about quality of care, waiting times, customer service or other concerns.”**

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### Section 5 A guide to the basics of coverage decisions and appeals

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#### Section 5.1 Asking for coverage decisions and making appeals: the big picture

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Coverage decisions and appeals deal with problems related to your benefits and coverage for your medical care (services, items and Part B prescription drugs, including payment). To keep things simple, we generally refer to medical items, services and Medicare Part B prescription drugs as medical care. You use the coverage decision and appeals process for issues such as whether something is covered or not and the way in which something is covered.

### **Asking for coverage decisions prior to receiving benefits**

A coverage decision is a decision we make about your benefits and coverage or about the amount we will pay for your medical care. For example, if your plan network doctor refers you to a medical specialist not inside the network, this referral is considered a favorable coverage decision unless either your network doctor can show that you received a standard denial notice for this medical specialist, or the Evidence of Coverage makes it clear that the referred service is never covered under any condition. You or your doctor can also contact us and ask for a coverage decision if your doctor is unsure whether we will cover a particular medical service or refuses to provide medical care you think that you need. In other words, if you want to know if we will cover medical care before you receive it, you can ask us to make a coverage decision for you. In limited circumstances a request for a coverage decision will be dismissed, which means we won't review the request. Examples of when a request will be dismissed include if the request is incomplete, if someone makes the request on your behalf but isn't legally authorized to do so or if you ask for your request to be withdrawn. If we dismiss a request for a coverage decision, we will send a notice explaining why the request was dismissed and how to ask for a review of the dismissal.

We are making a coverage decision for you whenever we decide what is covered for you and how much we pay. In some cases, we might decide medical care is not covered or is no longer covered by Medicare for you. If you disagree with this coverage decision, you can make an appeal.

### **Making an appeal**

If we make a coverage decision, whether before or after a benefit is received and you are not satisfied, you can "appeal" the decision. An appeal is a formal way of asking us to review and change a coverage decision we have made.

Under certain circumstances, which we discuss later, you can request an expedited or "fast appeal" of a coverage decision. Your appeal is handled by different reviewers than those who made the original decision. When you appeal a decision for the first time, this is called a Level 1 appeal. In this appeal, we review the coverage decision we made to check to see if we were properly following the rules.

When we have completed the review, we give you our decision. In limited circumstances a request for a Level 1 appeal will be dismissed, which means we won't review the request. Examples of when a request will be dismissed include if the request is incomplete, if someone makes the request on your behalf but isn't legally authorized to do so or if you ask for your request to be withdrawn. If we dismiss a request for a Level 1 appeal, we will send a notice explaining why the request was dismissed and how to ask for a review of the dismissal.

If we say no to all or part of your Level 1 appeal for medical care, your appeal will automatically go on to a Level 2 appeal conducted by an independent review organization that is not connected to us.

- ☐ You do not need to do anything to start a Level 2 appeal. Medicare rules require we automatically send your appeal for medical care to Level 2 if we do not fully agree with your Level 1 appeal.
- ☐ See Section 6.4 of this chapter for more information about Level 2 appeals.

- ☐ For Part D drug appeals, if we say no to all or part of your appeal, you will need to ask for a Level 2 appeal. Part D appeals are discussed further in Section 7 of this chapter.

If you are not satisfied with the decision at the Level 2 appeal, you may be able to continue through additional levels of appeal (Section 10 in this chapter explains the Level 3, 4, and 5 appeals processes).

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## Section 5.2      **How to get help when you are asking for a coverage decision or making an appeal**

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Here are resources if you decide to ask for any kind of coverage decision or appeal a decision:

- ☐ You **can call us at Customer Service**.
- ☐ You can get free help from your State Health Insurance Assistance Program.
- ☐ **Your doctor can make a request for you.** If your doctor helps with an appeal past Level 2, they will need to be appointed as your representative. Please call Customer Service and ask for the “Appointment of Representative” form. (The form is also available on Medicare’s website at [www.cms.gov/Medicare/CMS-Forms/CMS-Forms/downloads/cms1696.pdf](http://www.cms.gov/Medicare/CMS-Forms/CMS-Forms/downloads/cms1696.pdf).)
  - ☐ For medical care, your doctor can request a coverage decision or a Level 1 appeal on your behalf. If your appeal is denied at Level 1, it will be automatically forwarded to Level 2.
  - ☐ For Part D prescription drugs, your doctor or other prescriber can request a coverage decision or a Level 1 appeal on your behalf. If your Level 1 appeal is denied your doctor or prescriber can request a Level 2 appeal.
- ☐ **You can ask someone to act on your behalf.** If you want to, you can name another person to act for you as your “representative” to ask for a coverage decision or make an appeal.
  - ☐ If you want a friend, relative, or another person to be your representative, call Customer Service and ask for the “Appointment of Representative” form. (The form is also available on Medicare’s website at [www.cms.gov/Medicare/CMS-Forms/CMS-Forms/downloads/cms1696.pdf](http://www.cms.gov/Medicare/CMS-Forms/CMS-Forms/downloads/cms1696.pdf).) The form gives that person permission to act on your behalf. It must be signed by you and by the person who you would like to act on your behalf. You must give us a copy of the signed form.
  - ☐ While we can accept an appeal request without the form, we cannot begin or complete our review until we receive it. If we do not receive the form within 44 calendar days after receiving your appeal request (our deadline for making a decision on your appeal), your appeal request will be dismissed. If this happens, we will send you a written notice explaining your right to ask the independent review organization to review our decision to dismiss your appeal.
- ☐ **You also have the right to hire a lawyer.** You may contact your own lawyer, or get the name of a lawyer from your local bar association or other referral service. There are also groups that will give you free legal services if you qualify. However, **you are not required to hire a lawyer** to ask for any kind of coverage decision or appeal a decision.

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## Section 5.3      **Which section of this chapter gives the details for your situation?**

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There are four different situations that involve coverage decisions and appeals. Since each situation has different rules and deadlines, we give the details for each one in a separate section:

- **Section 6** of this chapter: “Your medical care: How to ask for a coverage decision or make an appeal”
- **Section 7** of this chapter: “Your Part D prescription drugs: How to ask for a coverage decision or make an appeal of a coverage decision”
- **Section 8** of this chapter: “How to ask us to cover a longer inpatient hospital stay if you think you are being discharged too soon”
- **Section 9** of this chapter: “How to ask us to keep covering certain medical services if you think your coverage is ending too soon” (**Applies only to these services:** home health care, skilled nursing facility care, and Comprehensive Outpatient Rehabilitation Facility (CORF) services)

If you’re not sure which section you should be using, please call Customer Service. You can also get help or information from government organizations such as your State Health Insurance Assistance Program.

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## **Section 6      Your medical care: How to ask for a coverage decision or make an appeal of a coverage decision**

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### **Section 6.1      This section tells what to do if you have problems getting coverage for medical care or if you want us to pay you back for our share of the cost of your care**

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This section is about your benefits for medical care. These benefits are described in Chapter 4 of this document: **Medical Benefits Chart (what is covered and what you pay)**. In some cases, different rules apply to a request for a Part B prescription drug. In those cases, we will explain how the rules for Part B prescription drugs are different from the rules for medical items and services.

This section tells what you can do if you are in any of the five following situations:

1. You are not getting certain medical care you want, and you believe that this care is covered by our plan. **Ask for a coverage decision. Section 6.2.**
2. Our plan will not approve the medical care your doctor or other medical provider wants to give you, and you believe that this care is covered by the plan. **Ask for a coverage decision. Section 6.2.**
3. You have received medical care that you believe should be covered by the plan, but we have said we will not pay for this care. **Make an appeal. Section 6.3.**
4. You have received and paid for medical care that you believe should be covered by the plan, and you want to ask our plan to reimburse you for this care. **Send us the bill. Section 6.5.**

5. You are being told that coverage for certain medical care you have been getting that we previously approved will be reduced or stopped, and you believe that reducing or stopping this care could harm your health. **Make an appeal. Section 6.3.**

**Note:** If the coverage that will be stopped is for hospital care, home health care, skilled nursing facility care, or Comprehensive Outpatient Rehabilitation Facility (CORF) services, you need to read Sections 8 and 9 of this Chapter. Special rules apply to these types of care.

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## Section 6.2      Step-by-step: How to ask for a coverage decision

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### Legal Terms

When a coverage decision involves your medical care, it is called an **“organization determination.”**

A “fast coverage decision” is called an **“expedited determination.”**

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**Step 1: Decide if you need a “standard coverage decision” or a “fast coverage decision”.**

**A “standard coverage decision” is usually made within 14 days or 72 hours for Part B drugs. A “fast coverage decision” is generally made within 72 hours, for medical services, or 24 hours for Part B drugs. In order to get a fast coverage decision, you must meet two requirements:**

- ☐ You may **only ask** for coverage for medical items and/or services (not requests for payment for items and/or services already received).
- ☐ You can get a fast coverage decision **only** if using the standard deadlines **could cause serious harm to your health or hurt your ability to function.**
- ☐ **If your doctor tells us that your health requires a “fast coverage decision,” we will automatically agree to give you a fast coverage decision.**
- ☐ **If you ask for a fast coverage decision on your own, without your doctor’s support, we will decide whether your health requires that we give you a fast coverage decision.** If we do not approve a fast coverage decision, we will send you a letter that:
  - ☐ Explains that we will use the standard deadlines.
  - ☐ Explains if your doctor asks for the fast coverage decision, we will automatically give you a fast coverage decision.
  - ☐ Explains that you can file a “fast complaint” about our decision to give you a standard coverage decision instead of the fast coverage decision you requested.



**Step 2: Ask our plan to make a coverage decision or fast coverage decision.**

Start by calling, writing, or faxing our plan to make your request for us to authorize or provide coverage for the medical care you want. You, your doctor, or your representative can do this. Chapter 2 has contact information.



**Step 3: We consider your request for medical care coverage and give you our answer.**

**For standard coverage decisions we use the standard deadlines.**

**This means we will give you an answer within 14 calendar days** after we receive your request **for a medical item or service**. If your request is for a **Medicare Part B prescription drug**, we will give you an answer **within 72 hours** after we receive your request.

- ☐ **However**, if you ask for more time, or if we need more information that may benefit you **we can take up to 14 more days** if your request is for a medical item or service. If we take extra days, we will tell you in writing. We can't take extra time to make a decision if your request is for a Medicare Part B prescription drug.
- ☐ If you believe we should not take extra days, you can file a "fast complaint". We will give you an answer to your complaint as soon as we make the decision. (The process for making a complaint is different from the process for coverage decisions and appeals. See Section 11 of this chapter for information on complaints.)

**For Fast Coverage decisions we use an expedited timeframe**

**A fast coverage decision means we will answer within 72 hours if your request is for a medical item or service. If your request is for a Medicare Part B prescription drug, we will answer within 24 hours.**

- ☐ **However**, if you ask for more time, or if we need more information that may benefit you **we can take up to 14 more days**. If we take extra days, we will tell you in writing. We can't take extra time to make a decision if your request is for a Medicare Part B prescription drug.
- ☐ If you believe we should **not** take extra days, you can file a "fast complaint." (See Section 11 of this chapter for information on complaints.) We will call you as soon as we make the decision.
- ☐ **If our answer is no to part or all of what you requested**, we will send you a written statement that explains why we said no.



**Step 4: If we say no to your request for coverage for medical care, you can appeal.**

- ☐ If we say no, you have the right to ask us to reconsider this decision by making an appeal. This means asking again to get the medical care coverage you want. If you make an appeal, it means you are going on to Level 1 of the appeals process.

### Section 6.3 Step-by-step: How to make a Level 1 appeal

#### Legal Terms

An appeal to the plan about a medical care coverage decision is called a plan “**reconsideration**.”

A “fast appeal” is also called an “**expedited reconsideration**.”



#### Step 1: Decide if you need a “standard appeal” or a “fast appeal.”

**A standard appeal is usually made within 30 days or 7 days for Part B drugs. A fast appeal is generally made within 72 hours.**

- ☐ If you are appealing a decision we made about coverage for care that you have not yet received, you and/or your doctor will need to decide if you need a “fast appeal.” If your doctor tells us that your health requires a “fast appeal,” we will give you a fast appeal.
- ☐ The requirements for getting a “fast appeal” are the same as those for getting a “fast coverage decision” in Section 6.2 of this chapter.



#### Step 2: Ask our plan for an appeal or a fast appeal

- ☐ **If you are asking for a standard appeal, submit your standard appeal in writing.** Chapter 2 has contact information.
- ☐ **If you are asking for a fast appeal, make your appeal in writing or call us.** Chapter 2 has contact information.
- ☐ **You must make your appeal request within 60 calendar days** from the date on the written notice we sent to tell you our answer on the coverage decision. If you miss this deadline and have a good reason for missing it, explain the reason your appeal is late when you make your appeal. We may give you more time to make your appeal. Examples of good cause may include a serious illness that prevented you from contacting us or if we provided you with incorrect or incomplete information about the deadline for requesting an appeal.
- ☐ **You can ask for a copy of the information regarding your medical decision. You and your doctor may add more information to support your appeal.**



### Step 3: We consider your appeal and we give you our answer.

- ☐ When we are reviewing your appeal, we take a careful look at all of the information. We check to see if we were following all the rules when we said no to your request.
- ☐ We will gather more information if needed, possibly contacting you or your doctor.

#### Deadlines for a “fast appeal”

- ☐ For fast appeals, we must give you our answer **within 72 hours after we receive your appeal**. We will give you our answer sooner if your health requires us to.
  - ☐ However, if you ask for more time, or if we need more information that may benefit you, we **can take up to 14 more calendar days** if your request is for a medical item or service. If we take extra days, we will tell you in writing. We can’t take extra time if your request is for a Medicare Part B prescription drug.
  - ☐ If we do not give you an answer within 72 hours (or by the end of the extended time period if we took extra days), we are required to automatically send your request on to Level 2 of the appeals process, where it will be reviewed by an independent review organization. Section 6.4 explains the Level 2 appeal process.
- ☐ **If our answer is yes to part or all of what you requested**, we must authorize or provide the coverage we have agreed to provide within 72 hours after we receive your appeal.
- ☐ **If our answer is no to part or all of what you requested**, we will send you our decision in writing and automatically forward your appeal to the independent review organization for a Level 2 appeal. The independent review organization will notify you in writing when it receives your appeal.

#### Deadlines for a “standard appeal”

- ☐ For standard appeals, we must give you our answer **within 30 calendar days** after we receive your appeal. If your request is for a Medicare Part B prescription drug you have not yet received, we will give you our answer **within 7 calendar days** after we receive your appeal. We will give you our decision sooner if your **health** condition requires us to.
  - ☐ However, if you ask for more time, or if we need more information that may benefit you, **we can take up to 14 more calendar days** if your request is for a medical item or service. If we take extra days, we will tell you in writing. We can’t take extra time to make a decision if your request is for a Medicare Part B prescription drug.
  - ☐ If you believe we should **not** take extra days, you can file a “fast complaint”. When you file a fast complaint, we will give you an answer to your complaint within 24 hours. (See Section 11 of this chapter for information on complaints.)
  - ☐ If we do not give you an answer by the deadline (or by the end of the extended time period), we will send your request to a Level 2 appeal, where an independent review organization will review the appeal. Section 6.4 explains the Level 2 appeal process.

- ☐ **If our answer is yes to part or all of what you requested**, we must authorize or provide the coverage within 30 calendar days if your request is for a medical item or service, or **within 7 calendar days** if your request is for a Medicare Part B prescription drug.
- ☐ If our plan says no to part or all of your appeal, we will automatically send your appeal to the independent review organization for a Level 2 appeal.

## Section 6.4 Step-by-step: How a Level 2 appeal is done

|                   |                                                                                                                                               |
|-------------------|-----------------------------------------------------------------------------------------------------------------------------------------------|
| <b>Legal Term</b> | The formal name for the “independent review organization” is the <b>“Independent Review Entity.”</b> It is sometimes called the <b>“IRE.”</b> |
|-------------------|-----------------------------------------------------------------------------------------------------------------------------------------------|

The **independent review organization** is an independent organization hired by Medicare. It is not connected with us and is not a government agency. This organization decides whether the decision we made is correct or if it should be changed. Medicare oversees its work.



### Step 1: The independent review organization reviews your appeal.

- ☐ We will send the information about your appeal to this organization. This information is called your “case file.” **You have the right to ask us for a copy of your case file.**
- ☐ You have a right to give the independent review organization additional information to support your appeal.
- ☐ Reviewers at the independent review organization will take a careful look at all of the information related to your appeal.

### If you had a “fast” appeal at Level 1, you will also have a “fast” appeal at Level 2

- ☐ For the “fast appeal” the review organization must give you an answer to your Level 2 appeal within 72 hours of when it receives your appeal.
- ☐ However, if your request is for a medical item or service and the independent review organization needs to gather more information that may benefit you, **it can take up to 14 more calendar days**. The independent review organization can’t take extra time to make a decision if your request is for a Medicare Part B prescription drug.

### If you had a “standard” appeal at Level 1, you will also have a “standard” appeal at Level 2

- ☐ For a “standard appeal” if your request is for a medical item or service, the review organization must give you an answer to your Level 2 Appeal **within 30 calendar days** of when it receives your appeal. If your request is for a Medicare Part B prescription drug, the review organization must give you an answer to your Level 2 appeal within 7 calendar days of when it receives your appeal.



- ☐ However, if your request is for a medical item or service and the independent review organization needs to gather more information that may benefit you, **it can take up to 14 more calendar days**. The independent review organization can't take extra time to make a decision if your request is for a Medicare Part B prescription drug.



### **Step 2: The independent review organization gives you their answer.**

The independent review organization will tell you its decision in writing and explain the reasons for it.

- ☐ **If the independent review organization says yes to part or all of a request for a medical item or service**, we must authorize the medical care coverage within 72 hours or provide the service within 14 calendar days after we receive the independent review organization's decision for standard requests or provide the service within 72 hours from the date the plan receives the independent review organization's decision for expedited requests.
- ☐ **If the independent review organization says yes to part or all of a request for a Medicare Part B prescription drug**, we must authorize or provide the Medicare Part B prescription drug **within 72 hours** after we receive the independent review organization's decision for **standard requests** or within 24 hours from the date we receive the independent review organization's decision for **expedited requests**.
- ☐ **If this organization says no to part or all of your appeal**, it means they agree with us that your request (or part of your request) for coverage for medical care should not be approved. (This is called "upholding the decision" or "turning down your appeal.") In this case, the independent review organization will send you a letter:
  - ☐ Explaining its decision.
  - ☐ Notifying you of the right to a Level 3 appeal if the dollar value of the medical care coverage meets a certain minimum. The written notice you get from the independent review organization will tell you the dollar amount you must meet to continue the appeals process.
  - ☐ Telling you how to file a Level 3 appeal.



### **Step 3: If your case meets the requirements, you choose whether you want to take your appeal further.**

- ☐ There are three additional levels in the appeals process after Level 2 (for a total of five levels of appeal). If you want to go to a Level 3 appeal the details on how to do this are in the written notice you get after your Level 2 appeal.
- ☐ The Level 3 appeal is handled by an Administrative Law Judge or attorney adjudicator. Section 10 in this chapter explains the Level 3, 4, and 5 appeals processes.



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## **Section 6.5      What if you are asking us to pay you back for our share of a bill you have received for medical care?**

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Chapter 7 describes when you may need to ask for reimbursement or to pay a bill you have received from a provider. It also tells how to send us the paperwork that asks us for payment.

### **Asking for reimbursement is asking for a coverage decision from us**

If you send us the paperwork asking for reimbursement, you are asking for a coverage decision. To make this decision, we will check to see if the medical care you paid for is covered. We will also check to see if you followed all the rules for using your coverage for medical care.

- ☐ **If we say yes to your request:** If the medical care is covered and you followed all the rules, we will send you the payment for our share of the cost within 60 calendar days after we receive your request. If you haven't paid for the medical care, we will send the payment directly to the provider.
- ☐ **If we say no to your request:** If the medical care is **not** covered, or you did **not** follow all the rules, we will not send payment. Instead, we will send you a letter that says we will not pay for the medical care and the reasons why.

If you do not agree with our decision to turn you down, **you can make an appeal**. If you make an appeal, it means you are asking us to change the coverage decision we made when we turned down your request for payment.

**To make this appeal, follow the process for appeals that we describe in Section 6.3.** For appeals concerning reimbursement, please note:

- ☐ We must give you our answer within 60 calendar days after we receive your appeal. If you are asking us to pay you back for medical care you have already received and paid for, you are not allowed to ask for a fast appeal.
- ☐ If the independent review organization decides we should pay, we must send you or the provider the payment within 30 calendar days. If the answer to your appeal is yes at any stage of the appeals process after Level 2, we must send the payment you requested to you or to the provider within 60 calendar days.

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## **Section 7      Your Part D prescription drugs: How to ask for a coverage decision or make an appeal**

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### **Section 7.1      This section tells you what to do if you have problems getting a Part D drug or you want us to pay you back for a Part D drug**

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Your benefits include coverage for many prescription drugs. To be covered, the drug must be used for a medically accepted indication. (See Chapter 5 for more information about a medically accepted indication.) For details about Part D drugs, rules, restrictions, and costs please see Chapters 5 and 6. **This section is about your Part D drugs only.** To keep things simple, we

generally say “drug” in the rest of this section, instead of repeating “covered outpatient prescription drug” or “Part D drug” every time. We also use the term “drug list” instead of “List of Covered Drugs” or “Formulary.”

- ☐ If you do not know if a drug is covered or if you meet the rules, you can ask us. Some drugs require that you get approval from us before we will cover it.
- ☐ If your pharmacy tells you that your prescription cannot be filled as written, the pharmacy will give you a written notice explaining how to contact us to ask for a coverage decision.

## Part D coverage decisions and appeals

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| <b>Legal Term</b> | An initial coverage decision about your Part D drugs is called a <b>“coverage determination.”</b> |
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A coverage decision is a decision we make about your benefits and coverage or about the amount we will pay for your drugs. This section tells what you can do if you are in any of the following situations:

- ☐ Asking to cover a Part D drug that is not on the plan’s **List of Covered Drugs. Ask for an exception. Section 7.2**
- ☐ Asking to waive a restriction on the plan’s coverage for a drug (such as limits on the amount of the drug you can get). **Ask for an exception. Section 7.2**
- ☐ Asking to get pre-approval for a drug. **Ask for a coverage decision. Section 7.4**
- ☐ Pay for a prescription drug you already bought. **Ask us to pay you back. Section 7.4**

If you disagree with a coverage decision we have made, you can appeal our decision.

This section tells you both how to ask for coverage decisions and how to request an appeal.

## Section 7.2 What is an exception?

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| <b>Legal Terms</b> | Asking for coverage of a drug that is not on the Drug List is sometimes called asking for a <b>“formulary exception.”</b> |
|                    | Asking for removal of a restriction on coverage for a drug is sometimes called asking for a <b>“formulary exception.”</b> |
|                    | Asking to pay a lower price for a covered non-preferred drug is sometimes called asking for a <b>“tiering exception.”</b> |

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If a drug is not covered in the way you would like it to be covered, you can ask us to make an “exception.” An exception is a type of coverage decision.

For us to consider your exception request, your doctor or other prescriber will need to explain the medical reasons why you need the exception approved. Here are two examples of exceptions that you or your doctor or other prescriber can ask us to make:

- 1. Covering a Part D drug for you that is not on our Drug List.** If we agree to cover a drug not on the Drug List, you will need to pay the cost-sharing amount that applies to all of our drugs. You cannot ask for an exception to the cost-sharing amount we require you to pay for the drug.
- 2. Removing a restriction for a covered drug.** Chapter 5 describes the extra rules or restrictions that apply to certain drugs on our **Drug List**.

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### Section 7.3      Important things to know about asking for exceptions

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#### Your doctor must tell us the medical reasons

Your doctor or other prescriber must give us a statement that explains the medical reasons for requesting an exception. For a faster decision, include this medical information from your doctor or other prescriber when you ask for the exception.

Typically, our Drug List includes more than one drug for treating a particular condition. These different possibilities are called “alternative” drugs. If an alternative drug would be just as effective as the drug you are requesting and would not cause more side effects or other health problems, we will generally **not** approve your request for an exception.

#### We can say yes or no to your request

- ☐ If we approve your request for an exception, our approval usually is valid until the end of the plan year. This is true as long as your doctor continues to prescribe the drug for you and that drug continues to be safe and effective for treating your condition.
- ☐ If we say no to your request, you can ask for another review by making an appeal.

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### Section 7.4      Step-by-step: How to ask for a coverage decision, including an exception

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| <b>Legal Term</b> | A “fast coverage decision” is called an “ <b>expedited coverage determination</b> .” |
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**Step 1: Decide if you need a “standard coverage decision” or a “fast coverage decision.”**

**“Standard coverage decisions”** are made **within 72 hours** after we receive your doctor’s statement. **“Fast coverage decisions”** are made within **24 hours** after we receive your doctor’s statement.

**If your health requires it, ask us to give you a “fast coverage decision.” To get a fast coverage decision, you must meet two requirements:**

- ☐ You must be asking for a **drug you have not yet received**. (You cannot ask for fast coverage decision to be paid back for a drug you have already bought.)
- ☐ Using the standard deadlines could **cause serious harm to your health or hurt your ability to function**.
- ☐ **If your doctor or other prescriber tells us that your health requires a “fast coverage decision,” we will automatically give you a fast coverage decision.**
- ☐ If you ask for a fast coverage decision on your own, without your doctor or prescriber’s support, we will decide whether your health requires that we give you a fast coverage decision. If we do not approve a fast coverage decision, we will send you a letter that:
  - ☐ Explains that we will use the standard deadlines.
  - ☐ Explains if your doctor or other prescriber asks for the fast coverage decision, we will automatically give you a fast coverage decision.
  - ☐ Tells you how you can file a “fast complaint” about our decision to give you a standard coverage decision instead of the fast coverage decision you requested. We will answer your complaint within 24 hours of receipt.



## **Step 2: Request a “standard coverage decision” or a “fast coverage decision.”**

Start by calling, writing, or faxing our plan to make your request for us to authorize or provide coverage for the medical care you want. You can also access the coverage decision process through our website. We must accept any written request, including a request submitted on the CMS Model Coverage Determination Request Form, which is available on our website. Chapter 2 has contact information. To assist us in processing your request, please be sure to include your name, contact information, and information identifying which denied claim is being appealed. You, your doctor, (or other prescriber) or your representative can do this. You can also have a lawyer act on your behalf. Section 4 of this chapter tells how you can give written permission to someone else to act as your representative.

- ☐ If you are requesting an exception, provide the “supporting statement,” which is the medical reasons for the exception. Your doctor or other prescriber can fax or mail the statement to us. Or your doctor or other prescriber can tell us on the phone and follow up by faxing or mailing a written statement if necessary.



### Step 3: We consider your request and give you our answer.

#### Deadlines for a “fast” coverage decision

- ☐ We must generally give you our answer **within 24 hours** after we receive your request.
  - ☐ For exceptions, we will give you our answer within 24 hours after we receive your doctor’s supporting statement. We will give you our answer sooner if your health requires us to.
  - ☐ If we do not meet this deadline, we are required to send your request on to Level 2 of the appeals process, where it will be reviewed by an independent review organization.
- ☐ **If our answer is yes to part or all of what you requested**, we must provide the coverage we have agreed to provide within 24 hours after we receive your request or doctor’s statement supporting your request.
- ☐ **If our answer is no to part or all of what you requested**, we will send you a written statement that explains why we said **no**. We will also tell you how you can appeal.

#### Deadlines for a “standard” coverage decision about a drug you have not yet received

- ☐ We must generally give you our answer **within 72 hours** after we receive your request.
  - ☐ For exceptions, we will give you our answer within 72 hours after we receive your doctor’s supporting statement. We will give you our answer sooner if your health requires us to.
  - ☐ If we do not meet this deadline, we are required to send your request on to Level 2 of the appeals process, where it will be reviewed by an independent review organization.
- ☐ **If our answer is yes to part or all of what you requested**, we must **provide the coverage** we have agreed to provide **within 72 hours** after we receive your request or doctor’s statement supporting your request.
- ☐ **If our answer is no to part or all of what you requested**, we will send you a written statement that explains why we said **no**. We will also tell you how you can appeal.

#### Deadlines for a “standard” coverage decision about payment for a drug you have already bought

- ☐ We must give you our answer **within 14 calendar days** after we receive your request.
- ☐ If we do not meet this deadline, we are required to send your request on to Level 2 of the appeals process, where it will be reviewed by an independent review organization.
- ☐ **If our answer is yes to part or all of what you requested**, we are also required to make payment to you within 14 calendar days after we receive your request.
- ☐ **If our answer is no to part or all of what you requested**, we will send you a written statement that explains why we said **no**. We will also tell you how you can appeal.



### Step 4: If we say no to your coverage request, you can make an appeal.

- ☐ If we say no, you have the right to ask us to reconsider this decision by making an appeal. This means asking again to get the drug coverage you want. If you make an appeal, it means you are going on to Level 1 of the appeals process.

## Section 7.5 Step-by-step: How to make a Level 1 appeal

### Legal Terms

An appeal to the plan about a Part D drug coverage decision is called a plan **“redetermination.”**

A “fast appeal” is also called an **“expedited redetermination.”**



### Step 1: Decide if you need a “standard appeal” or a “fast appeal.”

**A “standard appeal” is usually made within 7 days. A “fast appeal” is generally made within 72 hours. If your health requires it, ask for a “fast appeal”**

- ☐ If you are appealing a decision we made about a drug you have not yet received, you and your doctor or other prescriber will need to decide if you need a “fast appeal.”
- ☐ The requirements for getting a “fast appeal” are the same as those for getting a “fast coverage decision” in Section 7.4 of this chapter.



### Step 2: You, your representative, doctor or other prescriber must contact us and make your Level 1 appeal. If your health requires a quick response, you must ask for a “fast appeal.”

- ☐ **For standard appeals, submit a written request.** Chapter 2 has contact information.
- ☐ **For fast appeals either submit your appeal in writing or call us at 1-844-368-7149.** Chapter 2 has contact information.
- ☐ **We must accept any written request**, including a request submitted on the CMS Model Coverage Determination Request Form, which is available on our website. Please be sure to include your name, contact information, and information regarding your claim to assist us in processing your request.
- ☐ **You must make your appeal request within 60 calendar days** from the date on the written notice we sent to tell you our answer on the coverage decision. If you miss this deadline and have a good reason for missing it, explain the reason your appeal is late when you make your appeal. We may give you more time to make your appeal. Examples of good cause may include



a serious illness that prevented you from contacting us or if we provided you with incorrect or incomplete information about the deadline for requesting an appeal.

- ☐ **You can ask for a copy of the information in your appeal and add more information.** You and your doctor may add more information to support your appeal.



### **Step 3: We consider your appeal and we give you our answer.**

- ☐ When we are reviewing your appeal, we take another careful look at all of the information about your coverage request. We check to see if we were following all the rules when we said no to your request. We may contact you or your doctor or other prescriber to get more information.

#### **Deadlines for a “fast appeal”**

- ☐ For fast appeals, we must give you our answer **within 72 hours after we receive your appeal**. We will give you our answer sooner if your health requires us to.
  - ☐ If we do not give you an answer within 72 hours, we are required to send your request on to Level 2 of the appeals process, where it will be reviewed by an independent review organization. Section 7.6 explains the Level 2 appeals process.
- ☐ **If our answer is yes to part or all of what you requested**, we must provide the coverage we have agreed to provide within 72 hours after we receive your appeal.
- ☐ **If our answer is no to part or all of what you requested**, we will send you a written statement that explains why we said no and how you can appeal our decision.

#### **Deadlines for a “standard” appeal for a drug you have not yet received**

- ☐ For standard appeals, we must give you our answer **within 7 calendar days** after we receive your appeal. We will give you our decision sooner if you have not received the drug yet and your health condition requires us to do so.
  - ☐ If we do not give you a decision within 7 calendar days, we are required to send your request on to Level 2 of the appeals process, where it will be reviewed by an independent review organization. Section 7.6 explains the Level 2 appeal process.
- ☐ **If our answer is yes to part or all of what you requested**, we must **provide the coverage** as quickly as your health requires, but **no later than 7 calendar days** after we receive your appeal.
- ☐ **If our answer is no to part or all of what you requested**, we will send you a written statement that explains why we said no and how you can appeal our decision.

#### **Deadlines for a “standard appeal” about payment for a drug you have already bought**

- ☐ We must give you our answer **within 14 calendar days** after we receive your request.
  - ☐ If we do not meet this deadline, we are required to send your request on to Level 2 of the appeals process, where it will be reviewed by an independent review organization.
- ☐ **If our answer is yes to part or all of what you requested**, we are also required to make payment to you within 30 calendar days after we receive your request.



- ☐ **If our answer is no to part or all of what you requested**, we will send you a written statement that explains why we said no. We will also tell you how you can appeal.



**Step 4: If we say no to your appeal, you decide if you want to continue with the appeals process and make another appeal.**

- ☐ If you decide to make another appeal, it means your appeal is going on to Level 2 of the appeals process.

## Section 7.6 Step-by-step: How to make a Level 2 appeal

### Legal Term

The formal name for the “Independent Review Organization” is the “Independent Review Entity.” It is sometimes called the “IRE.”

The **independent review organization is an independent organization hired by Medicare**. It is not connected with us and is not a government agency. This organization decides whether the decision we made is correct or if it should be changed. Medicare oversees its work.



**Step 1: You (or your representative or your doctor or other prescriber) must contact the independent review organization and ask for a review of your case.**

- ☐ If our plan says no to your Level 1 appeal, the written notice we send you will include **instructions on how to make a Level 2 appeal** with the independent review organization. These instructions will tell who can make this Level 2 appeal, what deadlines you must follow, and how to reach the review organization. If, however, we did not complete our review within the applicable timeframe, or make an unfavorable decision regarding “at-risk” determination under our drug management program, we will automatically forward your claim to the IRE.
- ☐ We will send the information about your appeal to this organization. This information is called your “case file.” **You have the right to ask us for a copy of your case file.**
- ☐ You have a right to give the independent review organization additional information to support your appeal.



**Step 2: The independent review organization reviews your appeal.**

Reviewers at the independent review organization will take a careful look at all of the information related to your appeal.

### Deadlines for “fast” appeal

- ☐ If your health requires it, ask the independent review organization for a “fast appeal.”
- ☐ If the organization agrees to give you a “fast appeal,” the organization must give you an answer to your Level 2 appeal **within 72 hours** after it receives your appeal request.

### Deadlines for “standard” appeal

- ☐ For standard appeals, the review organization must give you an answer to your Level 2 appeal **within 7 calendar days** after it receives your appeal if it is for a drug you have not yet received. If you are requesting that we pay you back for a drug you have already bought, the review organization must give you an answer to your Level 2 appeal **within 14 calendar days** after it receives your request.



### Step 3: The independent review organization gives you their answer.

#### For “fast” appeals

- ☐ If the independent review organization says yes to part or all of what you requested, we must provide the drug coverage that was approved by the review organization **within 24 hours** after we receive the decision from the review organization.

#### For “standard” appeals

- ☐ If the independent review organization says yes to part or all of your request for coverage, we must provide the drug coverage that was approved by the review organization **within 72 hours** after we receive the decision from the review organization.
- ☐ If the independent review organization says yes to part or all of your request to pay you back for a drug you already bought, we are required to **send payment to you within 30 calendar days** after we receive the decision from the review organization.

#### What if the review organization says no to your appeal?

If this organization says no to part or all of your appeal, it means they agree with our decision not to approve your request (or part of your request). (This is called “upholding the decision.” It is also called “turning down your appeal.”) In this case, the independent review organization will send you a letter:

- ☐ Explaining its decision.
- ☐ Notifying you of the right to a Level 3 appeal if the dollar value of the drug coverage you are requesting meets a certain minimum. If the dollar value of the drug coverage you are requesting is too low, you cannot make another appeal and the decision at Level 2 is final.
- ☐ Telling you the dollar value that must be in dispute to continue with the appeals process.



**Step 4: If your case meets the requirements, you choose whether you want to take your appeal further.**

- ☐ There are three additional levels in the appeals process after Level 2 (for a total of five levels of appeal).
- ☐ If you want to go on to a Level 3 appeal the details on how to do this are in the written notice you get after your Level 2 appeal decision.
- ☐ The Level 3 appeal is handled by an Administrative Law Judge or attorney adjudicator. Section 10 in this chapter tells more about Levels 3, 4, and 5 of the appeals process.

## **Section 8                      How to ask us to cover a longer inpatient hospital stay if you think you are being discharged too soon**

When you are admitted to a hospital, you have the right to get all of your covered hospital services that are necessary to diagnose and treat your illness or injury.

During your covered hospital stay, your doctor and the hospital staff will be working with you to prepare for the day when you will leave the hospital. They will help arrange for care you may need after you leave.

- ☐ The day you leave the hospital is called your “**discharge date.**”
- ☐ When your discharge date is decided, your doctor or the hospital staff will tell you.
- ☐ If you think you are being asked to leave the hospital too soon, you can ask for a longer hospital stay and your request will be considered.

### **Section 8.1                      During your inpatient hospital stay, you will get a written notice from Medicare that tells about your rights**

Within two days of being admitted to the hospital, you will be given a written notice called **An Important Message from Medicare about Your Rights**. Everyone with Medicare gets a copy of this notice. If you do not get the notice from someone at the hospital (for example, a caseworker or nurse), ask any hospital employee for it. If you need help, please call Customer Service or 1-800-MEDICARE (1-800-633-4227), 24 hours a day, 7 days a week (TTY 1-877-486-2048).

**1. Read this notice carefully and ask questions if you don’t understand it.** It tells you:

- ☐ Your right to receive Medicare-covered services during and after your hospital stay, as ordered by your doctor. This includes the right to know what these services are, who will pay for them, and where you can get them.
- ☐ Your right to be involved in any decisions about your hospital stay.
- ☐ Where to report any concerns you have about the quality of your hospital care.

- ☐ Your right to **request an immediate review** of the decision to discharge you if you think you are being discharged from the hospital too soon. This is a formal, legal way to ask for a delay in your discharge date so that we will cover your hospital care for a longer time.

**2. You will be asked to sign the written notice to show that you received it and understand your rights.**

- ☐ You or someone who is acting on your behalf will be asked to sign the notice.
- ☐ Signing the notice shows **only** that you have received the information about your rights. The notice does not give your discharge date. Signing the notice **does not mean** you are agreeing on a discharge date.

**3. Keep your copy** of the notice handy so you will have the information about making an appeal (or reporting a concern about quality of care) if you need it.

- ☐ If you sign the notice more than two days before your discharge date, you will get another copy before you are scheduled to be discharged.
- ☐ To look at a copy of this notice in advance, you can call Customer Service or 1-800 MEDICARE (1-800-633-4227), 24 hours a day, 7 days a week. TTY users should call 1-877-486-2048. You can also see the notice online at [www.cms.gov/Medicare/Medicare-General-Information/BNH/HospitalDischargeAppealNotices.html](http://www.cms.gov/Medicare/Medicare-General-Information/BNH/HospitalDischargeAppealNotices.html).

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## **Section 8.2                      Step-by-step: How to make a Level 1 appeal to change your hospital discharge date**

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If you want to ask for your inpatient hospital services to be covered by us for a longer time, you will need to use the appeals process to make this request. Before you start, understand what you need to do and what the deadlines are:

- ☐ **Follow the process.**
- ☐ **Meet the deadlines.**
- ☐ **Ask for help if you need it.** If you have questions or need help at any time, please call Customer Service. Or, call your State Health Insurance Assistance Program, a government organization that provides personalized assistance.

**During a Level 1 appeal, the Quality Improvement Organization reviews your appeal.** It checks to see if your planned discharge date is medically appropriate for you.

**The Quality Improvement Organization** is a group of doctors and other health care professionals paid by the Federal government to check on and help improve the quality of care for people with Medicare. This includes reviewing hospital discharge dates for people with Medicare. These experts are not part of our plan.



**Step 1: Contact the Quality Improvement Organization for your state and ask for an immediate review of your hospital discharge. You must act quickly.**

### How can you contact this organization?

- ☐ The written notice you received (**An Important Message from Medicare About Your Rights**) tells you how to reach this organization. Or, find the name, address, and phone number of the Quality Improvement Organization for your state in Chapter 2.

### Act quickly:

- ☐ To make your appeal, you must contact the Quality Improvement Organization **before** you leave the hospital and **no later than midnight the day of your discharge**.
  - ☐ If you meet this deadline, you may stay in the hospital **after** your discharge date **without paying for it** while you wait to get the decision from the Quality Improvement Organization.
  - ☐ If you do **not** meet this deadline, and you decide to stay in the hospital after your planned discharge date, **you may have to pay all of the costs** for hospital care you receive after your planned discharge date.
- ☐ If you miss the deadline for contacting the Quality Improvement Organization, and you still wish to appeal, you must make an appeal directly to our plan instead. For details about this other way to make your appeal, see Section 8.4.

Once you request an immediate review of your hospital discharge the Quality Improvement Organization will contact us. By noon of the day after we are contacted, we will give you a **Detailed Notice of Discharge**. This notice gives your planned discharge date and explains in detail the reasons why your doctor, the hospital, and we think it is right (medically appropriate) for you to be discharged on that date.

You can get a sample of the **Detailed Notice of Discharge** by calling Customer Service or 1-800-MEDICARE (1-800-633-4227), 24 hours a day, 7 days a week. (TTY users should call 1-877-486-2048.) Or you can see a sample notice online at [www.cms.gov/Medicare/Medicare-General-Information/BNI/HospitalDischargeAppealNotices](http://www.cms.gov/Medicare/Medicare-General-Information/BNI/HospitalDischargeAppealNotices).



### **Step 2: The Quality Improvement Organization conducts an independent review of your case.**

- ☐ Health professionals at the Quality Improvement Organization (we will call them “the reviewers”) will ask you (or your representative) why you believe coverage for the services should continue. You don’t have to prepare anything in writing, but you may do so if you wish.
- ☐ The reviewers will also look at your medical information, talk with your doctor, and review information that the hospital and we have given to them.
- ☐ By noon of the day after the reviewers told us of your appeal, you will get a written notice from us that gives your planned discharge date. This notice also explains in detail the reasons why your doctor, the hospital, and we think it is right (medically appropriate) for you to be discharged on that date.



**Step 3: Within one full day after it has all the needed information, the Quality Improvement Organization will give you its answer to your appeal.**

**What happens if the answer is yes?**

- ☐ If the review organization says **yes**, **we must keep providing your covered inpatient hospital services for as long as these services are medically necessary.**
- ☐ You will have to keep paying your share of the costs (such as deductibles or copayments, if these apply). In addition, there may be limitations on your covered hospital services.

**What happens if the answer is no?**

- ☐ If the review organization says **no**, they are saying that your planned discharge date is medically appropriate. If this happens, **our coverage for your inpatient hospital services will end** at noon on the day **after** the Quality Improvement Organization gives you its answer to your appeal.
- ☐ If the review organization says **no** to your appeal and you decide to stay in the hospital, then **you may have to pay the full cost** of hospital care you receive after noon on the day after the Quality Improvement Organization gives you its answer to your appeal.



**Step 4: If the answer to your Level 1 appeal is no, you decide if you want to make another appeal.**

- ☐ If the Quality Improvement Organization has said **no** to your appeal, **and** you stay in the hospital after your planned discharge date, then you can make another appeal. Making another appeal means you are going on to Level 2 of the appeals process.

**Section 8.3**

**Step-by-step: How to make a Level 2 appeal to change your hospital discharge date**

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During a Level 2 appeal, you ask the Quality Improvement Organization to take another look at their decision on your first appeal. If the Quality Improvement Organization turns down your Level 2 appeal, you may have to pay the full cost for your stay after your planned discharge date.



**Step 1: Contact the Quality Improvement Organization again and ask for another review.**

- ☐ You must ask for this review **within 60 calendar days** after the day the Quality Improvement Organization said **no** to your Level 1 appeal. You can ask for this review only if you stay in the hospital after the date that your coverage for the care ended.



**Step 2: The Quality Improvement Organization does a second review of your situation.**

- ☐ Reviewers at the Quality Improvement Organization will take another careful look at all of the information related to your appeal.



**Step 3: Within 14 calendar days of receipt of your request for a Level 2 appeal, the reviewers will decide on your appeal and tell you their decision.**

**If the review organization says yes:**

- ☐ **We must reimburse you** for our share of the costs of hospital care you have received since noon on the day after the date your first appeal was turned down by the Quality Improvement Organization. **We must continue providing coverage for your inpatient hospital care for as long as it is medically necessary.**
- ☐ You must continue to pay your share of the costs and coverage limitations may apply.

**If the review organization says no:**

- ☐ It means they agree with the decision they made on your Level 1 appeal.
- ☐ The notice you get will tell you in writing what you can do if you wish to continue with the review process.



**Step 4: If the answer is no, you will need to decide whether you want to take your appeal further by going on to Level 3.**

- ☐ There are three additional levels in the appeals process after Level 2 (for a total of five levels of appeal). If you want to go to a Level 3 appeal, the details on how to do this are in the written notice you get after your Level 2 appeal decision.
- ☐ The Level 3 appeal is handled by an Administrative Law Judge or attorney adjudicator. Section 10 in this chapter tells more about Levels 3, 4, and 5 of the appeals process.

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**Section 8.4      What if you miss the deadline for making your Level 1 appeal to change your hospital discharge date?**

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**Legal Term**

A “fast review” (or “fast appeal”) is also called an “**expedited appeal.**”

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### **You can appeal to us instead**

As explained above, you must act quickly to start your Level 1 appeal of your hospital discharge date. If you miss the deadline for contacting the Quality Improvement Organization, there is another way to make your appeal.

If you use this other way of making your appeal, **the first two levels of appeal are different.**

#### **Step-by-Step: How to make a Level 1 alternate appeal**



##### **Step 1: Contact our plan and ask for a “fast review.”**

- ☐ **Ask for a “fast review.”** This means you are asking us to give you an answer using the “fast” deadlines rather than the “standard” deadlines. Chapter 2 has contact information.



##### **Step 2: We do a “fast” review of your planned discharge date, checking to see if it was medically appropriate.**

- ☐ During this review, we take a look at all of the information about your hospital stay. We check to see if your planned discharge date was medically appropriate. We see if the decision about when you should leave the hospital was fair and followed all the rules.



##### **Step 3: We give you our decision within 72 hours after you ask for a “fast review”.**

- ☐ **If we say yes to your appeal,** it means we have agreed with you that you still need to be in the hospital after the discharge date. We will keep providing your covered inpatient hospital services for as long as they are medically necessary. It also means that we have agreed to reimburse you for our share of the costs of care you have received since the date when we said your coverage would end. (You must pay your share of the costs and there may be coverage limitations that apply.)
- ☐ **If we say no to your appeal,** we are saying that your planned discharge date was medically appropriate. Our coverage for your inpatient hospital services ends as of the day we said coverage would end.
  - ☐ If you stayed in the hospital **after** your planned discharge date, then **you may have to pay the full cost** of hospital care you received after the planned discharge date.



##### **Step 4: If our plan says no to your appeal, your case will automatically be sent on to the next level of the appeals process.**

## Step-by-Step: Level 2 alternate appeal process

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### Legal Term

The formal name for the “Independent Review Organization” is the **“Independent Review Entity.”** It is sometimes called the **“IRE.”**

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**The independent review organization is an independent organization hired by Medicare.** It is not connected with our plan and is not a government agency. This organization decides whether the decision we made is correct or if it should be changed. Medicare oversees its work.



### **Step 1: We will automatically forward your case to the independent review organization.**

- ☐ We are required to send the information for your Level 2 appeal to the independent review organization within 24 hours of when we tell you that we are saying no to your first appeal. (If you think we are not meeting this deadline or other deadlines, you can make a complaint. Section 11 of this chapter tells how to make a complaint.)



### **Step 2: The independent review organization does a “fast review” of your appeal. The reviewers give you an answer within 72 hours.**

- ☐ Reviewers at the Independent review organization will take a careful look at all of the information related to your appeal of your hospital discharge.
- ☐ **If this organization says yes to your appeal,** then we must pay you back for our share of the costs of hospital care you received since the date of your planned discharge. We must also continue the plan’s coverage of your inpatient hospital services for as long as it is medically necessary. You must continue to pay your share of the costs. If there are coverage limitations, these could limit how much we would reimburse or how long we would continue to cover your services.
- ☐ **If this organization says no to your appeal,** it means they agree that your planned hospital discharge date was medically appropriate.
  - ☐ The written notice you get from the independent review organization will tell how to start a Level 3 appeal with the review process, which is handled by an Administrative Law Judge or attorney adjudicator.



### **Step 3: If the independent review organization turns down your appeal, you choose whether you want to take your appeal further.**

- ☐ There are three additional levels in the appeals process after Level 2 (for a total of five levels of appeal). If reviewers say no to your Level 2 appeal, you decide whether to accept their decision or go on to Level 3 appeal.
- ☐ Section 10 in this chapter tells more about Levels 3, 4, and 5 of the appeals process.

---

## **Section 9                      How to ask us to keep covering certain medical services if you think your coverage is ending too soon**

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### **Section 9.1                      This section is only about three services: Home health care, skilled nursing facility care, and Comprehensive Outpatient Rehabilitation Facility (CORF) services**

---

When you are getting covered **home health services, skilled nursing care, or rehabilitation care (Comprehensive Outpatient Rehabilitation Facility)**, you have the right to keep getting your services for that type of care for as long as the care is needed to diagnose and treat your illness or injury.

When we decide it is time to stop covering any of the three types of care for you, we are required to tell you in advance. When your coverage for that care ends, **we will stop paying our share of the cost for your care.**

If you think we are ending the coverage of your care too soon, **you can appeal our decision.** This section tells you how to ask for an appeal.

---

### **Section 9.2                      We will tell you in advance when your coverage will be ending**

---

|                   |                                                                                                                                                                                                                                           |
|-------------------|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <b>Legal Term</b> | <b>“Notice of Medicare Non-Coverage.”</b> It tells you how you can request a <b>“fast-track appeal.”</b> Requesting a fast-track appeal is a formal, legal way to request a change to our coverage decision about when to stop your care. |
|-------------------|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|

---

**1. You receive a notice in writing** at least two days before our plan is going to stop covering your care. The notice tells you:

- ☐ The date when we will stop covering the care for you.
- ☐ How to request a “fast track appeal” to request us to keep covering your care for a longer period of time.

**2. You, or someone who is acting on your behalf, will be asked to sign the written notice to show that you received it.** Signing the notice shows **only** that you have received the information about when your coverage will stop. **Signing it does not mean you agree** with the plan’s decision to stop care.

---

## Section 9.3      **Step-by-step: How to make a Level 1 appeal to have our plan cover your care for a longer time**

---

If you want to ask us to cover your care for a longer period of time, you will need to use the appeals process to make this request. Before you start, understand what you need to do and what the deadlines are.

- ☐ **Follow the process.**
- ☐ **Meet the deadlines.**
- ☐ **Ask for help if you need it.** If you have questions or need help at any time, please call Customer Service. Or call your State Health Insurance Assistance Program, a government organization that provides personalized assistance.

**During a Level 1 appeal, the Quality Improvement Organization reviews your appeal.** It decides if the end date for your care is medically appropriate.



**Step 1: Make your Level 1 appeal: contact the Quality Improvement Organization and ask for a fast-track appeal. You must act quickly.**

### **How can you contact this organization?**

- ☐ The written notice you received (Notice of Medicare Non-Coverage) tells you how to reach this organization. Or find the name, address, and phone number of the Quality Improvement Organization for your state in Chapter 2.

### **Act quickly:**

- ☐ You must contact the Quality Improvement Organization to start your appeal by noon of the day before the effective date on the Notice of Medicare Non-Coverage.
- ☐ If you miss the deadline for contacting the Quality Improvement Organization, and you still wish to file an appeal, you must make an appeal directly to us instead. For details about this other way to make your appeal, see Section 9.5 of this chapter.



**Step 2: The Quality Improvement Organization conducts an independent review of your case.**

---

### **Legal Term**

**“Detailed Explanation of Non-Coverage.”** Notice that provides details on reasons for ending coverage.

---

### What happens during this review?

- ☐ Health professionals at the Quality Improvement Organization (“the reviewers”) will ask you, or your representative, why you believe coverage for the services should continue. You don’t have to prepare anything in writing, but you may do so if you wish.
- ☐ The review organization will also look at your medical information, talk with your doctor, and review the information that our plan has given to them.
- ☐ By the end of the day the reviewers tell us of your appeal, you will get the **Detailed Explanation of Non-Coverage** from us that explains in detail our reasons for ending our coverage for your services.



**Step 3: Within one full day after they have all the information they need; the reviewers will tell you their decision.**

### What happens if the reviewers say yes?

- ☐ If the reviewers say **yes** to your appeal, then **we must keep providing your covered services for as long as it is medically necessary.**
- ☐ You will have to keep paying your share of the costs (such as deductibles or copayments, if these apply). There may be limitations on your covered services.

### What happens if the reviewers say no?

- ☐ If the reviewers say **no**, then **your coverage will end on the date we have told you.**
- ☐ If you decide to keep getting the home health care, or skilled nursing facility care, or Comprehensive Outpatient Rehabilitation Facility (CORF) services **after** this date when your coverage ends, then **you will have to pay the full cost** of this care yourself.



**Step 4: If the answer to your Level 1 appeal is no, you decide if you want to make another appeal.**

- ☐ If reviewers say **no** to your Level 1 appeal – **and** you choose to continue getting care after your coverage for the care has ended – then you can make a Level 2 appeal.

---

## Section 9.4      Step-by-step: How to make a Level 2 appeal to have our plan cover your care for a longer time

---

During a Level 2 appeal, you ask the Quality Improvement Organization to take another look at the decision on your first appeal. If the Quality Improvement Organization turns down your Level 2 appeal, you may have to pay the full cost for your home health care, or skilled nursing facility care, or Comprehensive Outpatient Rehabilitation Facility (CORF) services **after** the date when we said your coverage would end.



**Step 1: Contact the Quality Improvement Organization again and ask for another review.**

- ☐ You must ask for this review **within 60 days** after the day when the Quality Improvement Organization said **no** to your Level 1 appeal. You can ask for this review only if you continued getting care after the date that your coverage for the care ended.



**Step 2: The Quality Improvement Organization does a second review of your situation.**

- ☐ Reviewers at the Quality Improvement Organization will take another careful look at all of the information related to your appeal.



**Step 3: Within 14 days of receipt of your appeal request, reviewers will decide on your appeal and tell you their decision.**

**What happens if the review organization says yes?**

- ☐ **We must reimburse you** for our share of the costs of care you have received since the date when we said your coverage would end. **We must continue providing coverage** for the care for as long as it is medically necessary.
- ☐ You must continue to pay your share of the costs and there may be coverage limitations that apply.

**What happens if the review organization says no?**

- ☐ It means they agree with the decision made to your Level 1 appeal.
- ☐ The notice you get will tell you in writing what you can do if you wish to continue with the review process. It will give you the details about how to go on to the next level of appeal, which is handled by an Administrative Law Judge or attorney adjudicator.



**Step 4: If the answer is no, you will need to decide whether you want to take your appeal further.**

- ☐ There are three additional levels of appeal after Level 2, for a total of five levels of appeal. If you want to go on to a Level 3 appeal, the details on how to do this are in the written notice you get after your Level 2 appeal decision.

- ☐ The Level 3 appeal is handled by an Administrative Law Judge or attorney adjudicator. Section 10 in this chapter tells more about Levels 3, 4, and 5 of the appeals process.

---

## Section 9.5      What if you miss the deadline for making your Level 1 appeal?

---

### You can appeal to us instead

As explained above, you must act quickly to contact the Quality Improvement Organization to start your first appeal (within a day or two, at the most). If you miss the deadline for contacting this organization, there is another way to make your appeal. If you use this other way of making your appeal, **the first two levels of appeal are different.**

### Step-by-Step: How to make a Level 1 Alternate Appeal

---

|                   |                                                                                 |
|-------------------|---------------------------------------------------------------------------------|
| <b>Legal Term</b> | A “fast” review (or “fast appeal”) is also called an <b>“expedited appeal.”</b> |
|-------------------|---------------------------------------------------------------------------------|

---



#### Step 1: Contact us and ask for a “fast review.”

- ☐ **Ask for a “fast review.”** This means you are asking us to give you an answer using the “fast” deadlines rather than the “standard” deadlines. Chapter 2 has contact information.



#### Step 2: We do a “fast” review of the decision we made about when to end coverage for your services.

- ☐ During this review, we take another look at all of the information about your case. We check to see if we were following all the rules when we set the date for ending the plan’s coverage for services you were receiving.



#### Step 3: We give you our decision within 72 hours after you ask for a “fast review”.

- ☐ **If we say yes to your appeal,** it means we have agreed with you that you need services longer, and will keep providing your covered services for as long as it is medically necessary. It also means that we have agreed to reimburse you for our share of the costs of care you have received since the date when we said your coverage would end. (You must pay your share of the costs and there may be coverage limitations that apply.)



- ☐ **If we say no to your appeal**, then your coverage will end on the date we told you and we will not pay any share of the costs after this date.
- ☐ If you continued to get home health care, or skilled nursing facility care, or Comprehensive Outpatient Rehabilitation Facility (CORF) services **after** the date when we said your coverage would end, then **you will have to pay the full cost** of this care.



**Step 4: If we say no to your fast appeal, your case will automatically go on to the next level of the appeals process.**

---

**Legal Term**

The formal name for the “independent review organization” is the **“Independent Review Entity.”** It is sometimes called the **“IRE.”**

---

**Step-by-Step: Level 2 Alternate Appeal Process**

During the Level 2 appeal, the **independent review organization** reviews the decision we made to your “fast appeal.” This organization decides whether the decision should be changed. **The independent review organization is an independent organization that is hired by Medicare.** This organization is not connected with our plan and it is not a government agency. This organization is a company chosen by Medicare to handle the job of being the independent review organization. Medicare oversees its work.



**Step 1: We automatically forward your case to the independent review organization.**

- ☐ We are required to send the information for your Level 2 appeal to the independent review organization within 24 hours of when we tell you that we are saying no to your first appeal. (If you think we are not meeting this deadline or other deadlines, you can make a complaint. Section 11 of this chapter tells how to make a complaint.)



**Step 2: The independent review organization does a “fast review” of your appeal. The reviewers give you an answer within 72 hours.**

- ☐ Reviewers at the independent review organization will take a careful look at all of the information related to your appeal.
- ☐ **If this organization says yes to your appeal**, then we must pay you back for our share of the costs of care you have received since the date when we said your coverage would end. We must also continue to cover the care for as long as it is medically necessary. You must continue

to pay your share of the costs. If there are coverage limitations, these could limit how much we would reimburse or how long we would continue to cover services.

- ☐ **If this organization says no to your appeal**, it means they agree with the decision our plan made to your first appeal and will not change it.
- ☐ The notice you get from the independent review organization will tell you in writing what you can do if you wish to go on to a Level 3 appeal.



**Step 3: If the independent review organization says no to your appeal, you choose whether you want to take your appeal further.**

- ☐ There are three additional levels of appeal after Level 2, for a total of five levels of appeal. If you want to go on to a Level 3 appeal, the details on how to do this are in the written notice you get after your Level 2 appeal decision.
- ☐ A Level 3 appeal is reviewed by an Administrative Law Judge or attorney adjudicator. Section 10 in this chapter tells more about Levels 3, 4, and 5 of the appeals process.

---

## **Section 10      Taking your appeal to Level 3 and beyond**

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### **Section 10.1      Appeal Levels 3, 4, and 5 for Medical Service Requests**

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This section may be appropriate for you if you have made a Level 1 appeal and a Level 2 appeal, and both of your appeals have been turned down.

If the dollar value of the item or medical service you have appealed meets certain minimum levels, you may be able to go on to additional levels of appeal. If the dollar value is less than the minimum level, you cannot appeal any further. The written response you receive to your Level 2 appeal will explain how to make a Level 3 appeal.

For most situations that involve appeals, the last three levels of appeal work in much the same way. Here is who handles the review of your appeal at each of these levels.

---

**Level 3 appeal: An Administrative Law Judge or an attorney adjudicator who works for the Federal government** will review your appeal and give you an answer.

---

- ☐ **If the Administrative Law Judge or attorney adjudicator says yes to your appeal, the appeals process may or may not be over.** Unlike a decision at a Level 2 appeal, we have the right to appeal a Level 3 decision that is favorable to you. If we decide to appeal, it will go to a Level 4 appeal.
- ☐ If we decide **not** to appeal, we must authorize or provide you with the medical care within 60 calendar days after receiving the Administrative Law Judge's or attorney adjudicator's decision.

- ☐ If we decide to appeal the decision, we will send you a copy of the Level 4 appeal request with any accompanying documents. We may wait for the Level 4 appeal decision before authorizing or providing the medical care in dispute.
- ☐ **If the Administrative Law Judge or attorney adjudicator says no to your appeal, the appeals process may or may not be over.**
  - ☐ If you decide to accept this decision that turns down your appeal, the appeals process is over.
  - ☐ If you do not want to accept the decision, you can continue to the next level of the review process. The notice you get will tell you what to do for a Level 4 appeal.

---

**Level 4 appeal:** The Medicare **Appeals Council** (Council) will review your appeal and give you an answer. The Council is part of the Federal government.

---

- ☐ **If the answer is yes, or if the Council denies our request to review a favorable Level 3 appeal decision, the appeals process may or may not be over.** Unlike a decision at Level 2, we have the right to appeal a Level 4 decision that is favorable to you. We will decide whether to appeal this decision to Level 5.
  - ☐ If we decide **not** to appeal the decision, we must authorize or provide you with the medical care within 60 calendar days after receiving the Council's decision.
  - ☐ If we decide to appeal the decision, we will let you know in writing.
- ☐ **If the answer is no or if the Council denies the review request, the appeals process may or may not be over.**
  - ☐ If you decide to accept this decision that turns down your appeal, the appeals process is over.
  - ☐ If you do not want to accept the decision, you may be able to continue to the next level of the review process. If the Council says no to your appeal, the notice you get will tell you whether the rules allow you to go on to a Level 5 appeal and how to continue with a Level 5 appeal.

---

**Level 5 appeal:** A judge at the **Federal District Court** will review your appeal.

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- ☐ A judge will review all of the information and decide yes or no to your request. This is a final answer. There are no more appeal levels after the Federal District Court.

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## **Section 10.2      Appeal Levels 3, 4, and 5 for Part D Drug Requests**

---

This section may be appropriate for you if you have made a Level 1 appeal and a Level 2 appeal, and both of your appeals have been turned down.

If the value of the drug you have appealed meets a certain dollar amount, you may be able to go on to additional levels of appeal. If the dollar amount is less, you cannot appeal any further. The written response you receive to your Level 2 appeal will explain who to contact and what to do to ask for a Level 3 appeal.

For most situations that involve appeals, the last three levels of appeal work in much the same way. Here is who handles the review of your appeal at each of these levels.

---

**Level 3 appeal: An Administrative Law Judge or an attorney adjudicator who works for the Federal government** will review your appeal and give you an answer.

---

- ☐ **If the answer is yes, the appeals process is over.** We must **authorize or provide the drug coverage** that was approved by the Administrative Law Judge or attorney adjudicator **within 72 hours (24 hours for expedited appeals) or make payment no later than 30 calendar days** after we receive the decision.
- ☐ **If the answer is no, the appeals process may or may not be over.**
  - ☐ If you decide to accept this decision that turns down your appeal, the appeals process is over.
  - ☐ If you do not want to accept the decision, you can continue to the next level of the review process. The notice you get will tell you what to do for a Level 4 appeal.

---

**Level 4 appeal:** The Medicare **Appeals Council** (Council) will review your appeal and give you an answer. The Council is part of the Federal government.

---

- ☐ **If the answer is yes, the appeals process is over.** We must **authorize or provide the drug coverage** that was approved by the Council **within 72 hours (24 hours for expedited appeals) or make payment no later than 30 calendar days** after we receive the decision.
- ☐ **If the answer is no, the appeals process may or may not be over.**
  - ☐ If you decide to accept this decision that turns down your appeal, the appeals process is over.
  - ☐ If you do not want to accept the decision, you may be able to continue to the next level of the review process. If the Council says no to your appeal or denies your request to review the appeal, the notice will tell you whether the rules allow you to go on to a Level 5 appeal. It will also tell you who to contact and what to do next if you choose to continue with your appeal.

---

**Level 5 appeal:** A judge at the **Federal District Court** will review your appeal.

---

- ☐ A judge will review all of the information and decide yes or no to your request. This is a final answer. There are no more appeal levels after the Federal District Court.

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## **Section 11      How to make a complaint about quality of care, waiting times, customer service, or other concerns**

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## Section 11.1 What kinds of problems are handled by the complaint process?

The complaint process is only used for certain types of problems. This includes problems related to quality of care, waiting times, and the customer service. Here are examples of the kinds of problems handled by the complaint process.

| Complaint                                                                                                                                       | Example                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                              |
|-------------------------------------------------------------------------------------------------------------------------------------------------|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <b>Quality of your medical care</b>                                                                                                             | <input type="checkbox"/> Are you unhappy with the quality of the care you have received (including care in the hospital)?                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                            |
| <b>Respecting your privacy</b>                                                                                                                  | <input type="checkbox"/> Did someone not respect your right to privacy or share confidential information?                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                            |
| <b>Disrespect, poor customer service, or other negative behaviors</b>                                                                           | <input type="checkbox"/> Has someone been rude or disrespectful to you?<br><input type="checkbox"/> Are you unhappy with our Customer Service?<br><input type="checkbox"/> Do you feel you are being encouraged to leave the plan?                                                                                                                                                                                                                                                                                                                                                                                                                                                   |
| <b>Waiting times</b>                                                                                                                            | <input type="checkbox"/> Are you having trouble getting an appointment, or waiting too long to get it?<br><input type="checkbox"/> Have you been kept waiting too long by doctors, pharmacists, or other health professionals? Or by Customer Service or other staff at our plan?<br><input type="checkbox"/> Examples include waiting too long on the phone, in the waiting or exam room, or getting a prescription.                                                                                                                                                                                                                                                                |
| <b>Cleanliness</b>                                                                                                                              | <input type="checkbox"/> Are you unhappy with the cleanliness or condition of a clinic, hospital, or doctor's office?                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                |
| <b>Information you get from us</b>                                                                                                              | <input type="checkbox"/> Did we fail to give you a required notice?<br><input type="checkbox"/> Is our written information hard to understand?                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                       |
| <b>Timeliness</b> (These types of complaints are all related to the <b>timeliness</b> of our actions related to coverage decisions and appeals) | <p>If you have asked us for a coverage decision or made an appeal, and you think that we are not responding quickly enough, you can also make a complaint about our slowness. Here are examples:</p> <input type="checkbox"/> You asked us for a "fast coverage decision" or a "fast appeal," and we have said no; you can make a complaint.<br><input type="checkbox"/> You believe we are not meeting the deadlines for coverage decisions or appeals; you can make a complaint.<br><input type="checkbox"/> You believe we are not meeting deadlines for covering or reimbursing you for certain medical items or services or drugs that were approved; you can make a complaint. |

| Complaint | Example                                                                                                                                                              |
|-----------|----------------------------------------------------------------------------------------------------------------------------------------------------------------------|
|           | <input type="checkbox"/> You believe we failed to meet required deadlines for forwarding your case to the independent review organization; you can make a complaint. |

## Section 11.2 How to make a complaint

### Legal Terms

A “**complaint**” is also called a “**grievance**.”

“**Making a complaint**” is also called “**filing a grievance**.”

“**Using the process for complaints**” is also called “**using the process for filing a grievance**.”

A “**fast complaint**” is also called an “**expedited grievance**.”

## Section 11.3 Step-by-step: Making a complaint



### Step 1: Contact us promptly – either by phone or in writing.

- ☐ **Usually, calling Customer Service is the first step.** If there is anything else you need to do, Customer Service will let you know.
  - ☐ **If you do not wish to call (or you called and were not satisfied), you can put your complaint in writing and send it to us.** If you put your complaint in writing, we will respond to your complaint in writing.
  - ☐ We must receive your complaint within 60 calendar days of the event or incident you are complaining about. If something kept you from filing your complaint (you were sick, we provided incorrect information, etc.) let us know and we might be able to accept your complaint past 60 days. We will address your complaint as quickly as possible but no later than 30 days after receiving it. Sometimes we need additional information, or you may wish to provide additional information. If that occurs, we may take an additional 14 days to respond to your complaint. If the additional 14 days is taken, you will receive a letter letting you know.
- If your complaint is because we took 14 extra days to respond to your request for a coverage determination or appeal or because we decided you didn’t need a fast coverage decision or a fast appeal, you can file a fast complaint. We will respond to you within 24 hours of receiving your complaint. The address and fax numbers for filing complaints are located in Chapter 2 under “How to contact us when you are making a complaint about your medical care” or for

Part D prescription drug complaints “How to contact us when you are making a complaint about your Part D prescription drugs.”

- ☐ The **deadline** for making a complaint is 60 calendar days from the time you had the problem you want to complain about.



**Step 2: We look into your complaint and give you our answer.**

- ☐ **If possible, we will answer you right away.** If you call us with a complaint, we may be able to give you an answer on the same phone call.
- ☐ **Most complaints are answered within 30 calendar days.** If we need more information and the delay is in your best interest or if you ask for more time, we can take up to 14 more calendar days (44 calendar days total) to answer your complaint. If we decide to take extra days, we will tell you in writing.
- ☐ **If you are making a complaint because we denied your request for a “fast coverage decision” or a “fast appeal,” we will automatically give you a “fast complaint”.** If you have a “fast complaint,” it means we will give you **an answer within 24 hours**.
- ☐ **If we do not agree** with some or all of your complaint or don’t take responsibility for the problem you are complaining about, we will include our reasons in our response to you.

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**Section 11.4      You can also make complaints about quality of care to the Quality Improvement Organization**

---

When your complaint is about **quality of care**, you also have two extra options:

- ☐ **You can make your complaint directly to the Quality Improvement Organization.**
- ☐ The Quality Improvement Organization is a group of practicing doctors and other health care experts paid by the Federal government to check and improve the care given to Medicare patients. Chapter 2 has contact information.

**Or**

- ☐ **You can make your complaint to both the Quality Improvement Organization and us at the same time.**

---

**Section 11.5      You can also tell Medicare about your complaint**

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You can submit a complaint about UHC Dual Complete NE-V001 (HMO-POS D-SNP) directly to Medicare. To submit a complaint to Medicare, go to [www.medicare.gov/MedicareComplaintForm/home.aspx](http://www.medicare.gov/MedicareComplaintForm/home.aspx). You may also call 1-800-MEDICARE (1-800-633-4227). TTY/TDD users can call 1-877-486-2048.



## **Problems about your Medicaid benefits**

### **Section 12      Handling problems about your Medicaid benefits**

---

You can get help and information from Nebraska Department of Health and Human Services (Medicaid).

For more information and help in handling a problem, you can also contact Nebraska Department of Health and Human Services (Medicaid).

If you have Medicare and Medicaid, some of your plan services may also be covered by your State Nebraska Department of Health and Human Services (Medicaid) program. Therefore, if you believe that we improperly denied you a service or payment for a service, you may also have the right to ask your State Nebraska Department of Health and Human Services (Medicaid) program to pay for the service. You may also have appeals and grievances related to Medicaid-covered services. Please see your Medicaid Handbook for more information, or contact your State Nebraska Department of Health and Human Services (Medicaid) agency at the contact information listed in Chapter 2, Section 6 of this booklet.

The Plan will provide reasonable assistance determined by your needs. This may include, but not limited to, helping you complete forms, reviewing your Medicaid benefit and addressing claims questions, complaints and/or appeals.

# **Chapter 10**

Ending your membership in the  
plan

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## Section 1 Introduction to ending your membership in our plan

---

Ending your membership in the plan may be **voluntary** (your own choice) or **involuntary** (not your own choice):

- ☐ You might leave our plan because you have decided that you **want** to leave. Sections 2 and 3 provide information on ending your membership voluntarily.
- ☐ There are also limited situations where you do not choose to leave, but we are required to end your membership. Section 5 tells you about situations when we must end your membership.

If you are leaving our plan, our plan must continue to provide your medical care and you will continue to pay your cost share until your membership ends.

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## Section 2 When can you end your membership in our plan?

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### Section 2.1 You may be able to end your membership because you have Medicare and Medicaid

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Most people with Medicare can end their membership only during certain times of the year. Because you have Medicaid, you may be able to end your membership in our plan or switch to a different plan one time during each of the following Special Enrollment Periods:

- ☐ January to March
- ☐ April to June
- ☐ July to September

If you joined our plan during one of these periods, you'll have to wait for the next period to end your membership or switch to a different plan. You can't use this Special Enrollment Period to end your membership in our plan between October and December. However, all people with Medicare can make changes from October 15 – December 7 during the Annual Enrollment Period. Section 2.2 tells you more about the Annual Enrollment Period.

- ☐ Choose any of the following types of Medicare plans:
  - ☐ Another Medicare health plan, with or without prescription drug coverage
  - ☐ Original Medicare **with** a separate Medicare prescription drug plan
  - ☐ Original Medicare without a separate Medicare prescription drug plan
    - ☐ If you choose this option, Medicare may enroll you in a drug plan, unless you have opted out of automatic enrollment.

**Note:** If you disenroll from Medicare prescription drug coverage and go without “creditable” prescription drug coverage for a continuous period of 63 days or more, you may have to pay a Part D late enrollment penalty if you join a Medicare drug plan later.

Contact your State Medicaid Office to learn about your Medicaid plan options (telephone numbers are in Chapter 2, Section 6 of this document).

- ☐ When will your membership end? Your membership will usually end on the first day of the month after we receive your request to change your plans. Your enrollment in your new plan will also begin on this day.

---

## Section 2.2      You can end your membership during the Annual Enrollment Period

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You can end your membership in our plan during the **Annual Enrollment Period** (also known as the “Annual Open Enrollment Period”). During this time, review your health and drug coverage and decide about coverage for the upcoming year.

- ☐ The **Annual Enrollment Period** is from **October 15 to December 7**.
- ☐ **Choose to keep your current coverage or make changes to your coverage for the upcoming year.** If you decide to change to a new plan, you can choose any of the following types of plans:
  - ☐ Another Medicare health plan, with or without prescription drug coverage.
  - ☐ Original Medicare **with** a separate Medicare prescription drug plan.
  - ☐ Original Medicare without a separate Medicare prescription drug plan.
  - ☐ **Your membership will end in our plan** when your new plan’s coverage begins on January 1.

**If you receive “Extra Help” from Medicare to pay for your prescription drugs:** If you switch to Original Medicare and do not enroll in a separate Medicare prescription drug plan, Medicare may enroll you in a drug plan, unless you have opted out of automatic enrollment.

**Note:** If you disenroll from Medicare prescription drug coverage and go without creditable prescription drug coverage for 63 days or more in a row, you may have to pay a Part D late enrollment penalty if you join a Medicare drug plan later.

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## Section 2.3      You can end your membership during the Medicare Advantage Open Enrollment Period

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You have the opportunity to make **one** change to your health coverage during the **Medicare Advantage Open Enrollment Period**.

- ☐ The **annual Medicare Advantage Open Enrollment Period** is from January 1 to March 31.
- ☐ **During the annual Medicare Advantage Open Enrollment Period** you can:
  - ☐ Switch to another Medicare Advantage Plan with or without prescription drug coverage.
  - ☐ Disenroll from our plan and obtain coverage through Original Medicare. If you choose to switch to Original Medicare during this period, you can also join a separate Medicare prescription drug plan at that time.
- ☐ **Your membership will end** on the first day of the month after you enroll in a different Medicare Advantage plan or we get your request to switch to Original Medicare. If you also choose to

enroll in a Medicare prescription drug plan, your membership in the drug plan will begin the first day of the month after the drug plan gets your enrollment request.

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## **Section 2.4                      In certain situations, you can end your membership during a Special Enrollment Period**

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In certain situations, you may be eligible to end your membership at other times of the year. This is known as a **Special Enrollment Period**.

**You may be eligible to end your membership during a Special Enrollment Period** if any of the following situations apply to you. These are just examples, for the full list you can contact the plan, call Medicare, or visit the Medicare website (medicare.gov):

- ☐ Usually, when you have moved.
- ☐ If you have Medicaid.
- ☐ If you are eligible for “Extra Help” with paying for your Medicare prescriptions.
- ☐ If we violate our contract with you.
- ☐ If you are getting care in an institution, such as a nursing home or long-term care (LTC) hospital.

**Note:** If you’re in a drug management program, you may not be able to change plans. Chapter 5, Section 10 tells you more about drug management programs.

**Note:** Section 2.1 tells you more about the special enrollment period for people with Medicaid.

**The enrollment time periods vary** depending on your situation.

**To find out if you are eligible for a Special Enrollment Period**, please call Medicare at 1-800-MEDICARE (1-800-633-4227), 24 hours a day, 7 days a week. TTY users call 1-877-486-2048. If you are eligible to end your membership because of a special situation, you can choose to change both your Medicare health coverage and prescription drug coverage. You can choose:

- ☐ Another Medicare health plan with or without prescription drug coverage.
- ☐ Original Medicare with a separate Medicare prescription drug plan.
- ☐ – **or–** Original Medicare **without** a separate Medicare prescription drug plan.

**Note:** If you disenroll from Medicare prescription drug coverage and go without creditable prescription drug coverage for 63 days or more in a row, you may have to pay a Part D late enrollment penalty if you join a Medicare drug plan later.

**Your membership will usually end** on the first day of the month after your request to change your plan is received.

**If you receive “Extra Help” from Medicare to pay for your prescription drugs:** If you switch to Original Medicare and do not enroll in a separate Medicare prescription drug plan, Medicare may enroll you in a drug plan, unless you have opted out of automatic enrollment.

**Note:** Sections 2.1 and 2.2 tell you more about the special enrollment period for people with Medicaid and Extra Help.

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## **Section 2.5                      Where can you get more information about when you can end your membership?**

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If you have any questions about ending your membership you can:

- ☐ **Call Customer Service.**
- ☐ Find the information in the **Medicare & You 2024** handbook.
- ☐ Contact **Medicare** at 1-800-MEDICARE (1-800-633-4227), 24 hours a day, 7 days a week. (TTY 1-877-486-2048).

### Section 3 How do you end your membership in our plan?

The table below explains how you should end your membership in our plan.

| If you would like to switch from our plan to:                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                             | This is what you should do:                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                           |
|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <input type="checkbox"/> Another Medicare health plan.                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                    | <input type="checkbox"/> Enroll in the new Medicare health plan. Your new coverage will begin on the first day of the following month. You will automatically be disenrolled from our plan when your new plan's coverage begins.                                                                                                                                                                                                                                                                                                                                      |
| <input type="checkbox"/> Original Medicare <b>with</b> a separate Medicare prescription drug plan.                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                        | <input type="checkbox"/> Enroll in the new Medicare prescription drug plan. Your new coverage will begin on the first day of the following month. You will automatically be disenrolled from our plan when your new plan's coverage begins.                                                                                                                                                                                                                                                                                                                           |
| <input type="checkbox"/> Original Medicare without a separate Medicare prescription drug plan. <ul style="list-style-type: none"> <li><input type="checkbox"/> If you switch to Original Medicare and do not enroll in a separate Medicare prescription drug plan, Medicare may enroll you in a drug plan, unless you have opted out of automatic enrollment.</li> <li><input type="checkbox"/> If you disenroll from Medicare prescription drug coverage and go 63 days or more in a row without creditable prescription drug coverage, you may have to pay a late enrollment penalty if you join a Medicare drug plan later.</li> </ul> | <input type="checkbox"/> <b>Send us a written request to disenroll</b> or visit our website to disenroll online. Contact Customer Service if you need more information on how to do this. <ul style="list-style-type: none"> <li><input type="checkbox"/> You can also contact <b>Medicare</b> at 1-800-MEDICARE (1-800-633-4227), 24 hours a day, 7 days a week, and ask to be disenrolled. TTY users should call 1-877-486-2048.</li> <li><input type="checkbox"/> You will be disenrolled from our plan when your coverage in Original Medicare begins.</li> </ul> |

**Note:** If you disenroll from Medicare prescription drug coverage and go without creditable prescription drug coverage for 63 days or more in a row, you may have to pay a Part D late enrollment penalty if you join a Medicare drug plan later.

For questions about your Nebraska Department of Health and Human Services (Medicaid) benefits, contact Nebraska Department of Health and Human Services (Medicaid), at 1-402-471-3121, 8 a.m. - 5 p.m. CT, Monday - Friday. TTY users should call 1-800-471-7352. Ask how joining another plan

or returning to Original Medicare affects how you get your Nebraska Department of Health and Human Services (Medicaid) coverage.

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## **Section 4            Until your membership ends, you must keep getting your medical items, services and drugs through our plan**

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Until your membership ends, and your new Medicare and Medicaid coverage begins, you must continue to get your medical items, services and prescription drugs through our plan.

- ☐ **Continue to use our network providers to receive medical care.**
- ☐ **Continue to use our network pharmacies or mail order to get your prescriptions filled.**
- ☐ **If you are hospitalized on the day that your membership ends, your hospital stay will be covered by our plan until you are discharged (even if you are discharged after your new health coverage begins).**

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## **Section 5            We must end your membership in the plan in certain situations**

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### **Section 5.1            When must we end your membership in the plan?**

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**We must end your membership in the plan if any of the following happen:**

- ☐ If you no longer have Medicare Part A and Part B.
- ☐ If you are no longer eligible for Medicaid. As stated in Chapter 1, section 2.1, our plan is for people who are eligible for both Medicare and Medicaid. We must notify you in writing that you have a 6 month grace period to regain eligibility before you are disenrolled. For more information on the grace period and how it may affect your costs under this plan, please see Chapter 4, Section 1.1.
- ☐ If you do not pay your medical spend down, if applicable.
- ☐ If you move out of our service area.
- ☐ If you are away from our service area for more than 6 months.
  - ☐ If you move or take a long trip, call Customer Service to find out if the place you are moving or traveling to is in our plan's area .
- ☐ If you become incarcerated (go to prison).
- ☐ If you are no longer a United States citizen or lawfully present in the United States.
- ☐ If you lie or withhold information about other insurance you have that provides prescription drug coverage.
- ☐ If you intentionally give us incorrect information when you are enrolling in our plan and that information affects your eligibility for our plan. (We cannot make you leave our plan for this reason unless we get permission from Medicare first.)
- ☐ If you continuously behave in a way that is disruptive and makes it difficult for us to provide medical care for you and other members of our plan. (We cannot make you leave our plan for this reason unless we get permission from Medicare first.)



- ☐ If you let someone else use your UnitedHealthcare member ID card to get medical care. (We cannot make you leave our plan for this reason unless we get permission from Medicare first.)
  - ☐ If we end your membership because of this reason, Medicare may have your case investigated by the Inspector General.
- ☐ If you are required to pay the extra Part D amount because of your income and you do not pay it, Medicare will disenroll you from our plan.

**Where can you get more information?**

If you have questions or would like more information on when we can end your membership call Customer Service.

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**Section 5.2            We cannot ask you to leave our plan for any health-related reason**

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Our plan is not allowed to ask you to leave our plan for any health-related reason.

**What should you do if this happens?**

If you feel that you are being asked to leave our plan because of a health-related reason, call Medicare at 1-800-MEDICARE (1-800-633-4227) 24 hours a day, 7 days a week (TTY 1-877-486-2048).

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**Section 5.3            You have the right to make a complaint if we end your membership in our plan**

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If we end your membership in our plan, we must tell you our reasons in writing for ending your membership. We must also explain how you can file a grievance or make a complaint about our decision to end your membership.

# **Chapter 11**

## Legal notices

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## **Section 1            Notice about governing law**

The principal law that applies to this **Evidence of Coverage** document is Title XVIII of the Social Security Act and the regulations created under the Social Security Act by the Centers for Medicare & Medicaid Services, or CMS. In addition, other Federal laws may apply and, under certain circumstances, the laws of the state you live in. This may affect your rights and responsibilities even if the laws are not included or explained in this document.

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## **Section 2            Notice about non-discrimination**

We don't discriminate based on race, ethnicity, national origin, color, religion, sex, gender, age, sexual orientation, mental or physical disability, health status, claims experience, medical history, genetic information, evidence of insurability, or geographic location within the service area. All organizations that provide Medicare Advantage plans, like our plan, must obey Federal laws against discrimination, including Title VI of the Civil Rights Act of 1964, the Rehabilitation Act of 1973, the Age Discrimination Act of 1975, the Americans with Disabilities Act, Section 1557 of the Affordable Care Act, all other laws that apply to organizations that get Federal funding, and any other laws and rules that apply for any other reason.

If you want more information or have concerns about discrimination or unfair treatment, please call the Department of Health and Human Services' Office for Civil Rights at 1-800-368-1019 (TTY 1-800-537-7697) or your local Office for Civil Rights. You can also review information from the Department of Health and Human Services' Office for Civil Rights at [ocrportal.hhs.gov/ocr/portal/lobby.jsf](https://ocrportal.hhs.gov/ocr/portal/lobby.jsf).

If you have a disability and need help with access to care, please call us at Customer Service. If you have a complaint, such as a problem with wheelchair access, Customer Service can help.

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## **Section 3            Notice about Medicare Secondary Payer subrogation rights**

We have the right and responsibility to collect for covered Medicare services for which Medicare is not the primary payer. According to CMS regulations at 42 CFR sections 422.108 and 423.462, our plan, as a Medicare Advantage Organization, will exercise the same rights of recovery that the Secretary exercises under CMS regulations in subparts B through D of part 411 of 42 CFR and the rules established in this section supersede any State laws.

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## **Section 4            Third party liability and subrogation**

If you suffer an illness or injury for which any third party is alleged to be liable or responsible due to any negligent or intentional act or omission causing illness or injury to you, you must promptly notify us of the illness or injury. We will send you a statement of the amounts we paid for services provided in connection with the illness or injury. If you recover any sums from any third party, we shall be reimbursed out of any such recovery from any third party for the payments we made on your behalf, subject to the limitations in the following paragraphs.

- 1) **Our payments are less than the recovery amount.** If our payments are less than the total recovery amount from any third party (the “recovery amount”), then our reimbursement is computed as follows:
  - a) **First:** Determine the ratio of the procurement costs to the recovery amount (the term “procurement costs” means the attorney fees and expenses incurred in obtaining a settlement or judgment).
  - b) **Second:** Apply the ratio calculated above to our payment. The result is our share of procurement costs.
  - c) **Third:** Subtract our share of procurement costs from our payments. The remainder is our reimbursement amount.
- 2) **Our payments equal or exceed the recovery amount.** If our payments equal or exceed the recovery amount, our reimbursement amount is the total recovery amount minus the total procurement costs.
- 3) **We incur procurement costs because of opposition to our reimbursement.** If we must bring suit against the party that received the recovery amount because that party opposes our reimbursement, our reimbursement amount is the lower of the following:
  - a) Our payments made on your behalf for services; or
  - b) the recovery amount, minus the party’s total procurement cost.

Subject to the limitations stated above, you agree to grant us an assignment of, and a claim and a lien against, any amounts recovered through settlement, judgment or verdict. You may be required by us and you agree to execute documents and to provide information necessary to establish the assignment, claim, or lien to ascertain our right to reimbursement.

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## Section 5 Member liability

In the event we fail to reimburse provider’s charges for covered services, you will not be liable for any sums owed by us. Neither the plan nor Medicare will pay for non-covered services except for the following eligible expenses:

- ☐ Emergency services
- ☐ Urgently needed services
- ☐ Out-of-area and routine travel dialysis (must be received in a Medicare Certified Dialysis Facility within the United States)
- ☐ Post-stabilization services

If you enter into a private contract with a provider, neither the plan nor Medicare will pay for those services.

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## Section 6 Medicare-covered services must meet requirement of reasonable and necessary

In determining coverage, services must meet the reasonable and necessary requirements under Medicare in order to be covered under your plan, unless otherwise listed as a covered service. A service is “reasonable and necessary” if the service is:

- ☐ Safe and effective;
- ☐ Not experimental or investigational; and
- ☐ Appropriate, including the duration and frequency that is considered appropriate for the service, in terms of whether it is:
  1. Furnished in accordance with accepted standards of medical practice for the diagnosis or treatment of the patient’s condition or to improve the function of a malformed body member;
  2. Furnished in a setting appropriate to the patient’s medical needs and condition;
  3. Ordered and furnished by qualified personnel;
  4. One that meets, but does not exceed, the patient’s medical need; and
  5. At least as beneficial as an existing and available medically appropriate alternative.

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## **Section 7                      Non duplication of benefits with automobile, accident or liability coverage**

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If you are receiving benefits as a result of other automobile, accident or liability coverage, we will not duplicate those benefits. It is your responsibility to take whatever action is necessary to receive payment under automobile, accident, or liability coverage when such payments may reasonably be expected, and to notify us of such coverage when available. If we happen to duplicate benefits to which you are entitled under other automobile, accident or liability coverage, we may seek reimbursement of the reasonable value of those benefits from you, your insurance carrier, or your health care provider to the extent permitted under State and/or federal law. We will provide benefits over and above your other automobile, accident or liability coverage, if the cost of your health care services exceeds such coverage. **You are required to cooperate with us in obtaining payment from your automobile, accident or liability coverage carrier. Your failure to do so may result in termination of your plan membership.**

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## **Section 8                      Acts beyond our control**

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If, due to a natural disaster, war, riot, civil insurrection, complete or partial destruction of a facility, ordinance, law or decree of any government or quasi-governmental agency, labor dispute (when said dispute is not within our control), or any other emergency or similar event not within the control of us, providers may become unavailable to arrange or provide health services pursuant to this Evidence of Coverage and Disclosure Information, then we shall attempt to arrange for covered services insofar as practical and according to our best judgment. Neither we nor any provider shall have any liability or obligation for delay or failure to provide or arrange for covered services if such delay is the result of any of the circumstances described above.

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## **Section 9                      Contracting medical providers and network hospitals are independent contractors**

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The relationships between the plan and network providers and network hospitals are independent contractor relationships. None of the network providers or network hospitals or their physicians or employees are employees or agents of the plan. An agent would be anyone authorized to act on the plan's behalf.

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## **Section 10      Technology assessment**

We regularly review new procedures, devices and drugs to determine whether or not they are safe and efficacious for members. New procedures and technology that are safe and efficacious are eligible to become Covered Services. If the technology becomes a Covered Service, it will be subject to all other terms and conditions of the plan, including medical necessity and any applicable member copayments, coinsurance, deductibles or other payment contributions.

In determining whether to cover a service, we use proprietary technology guidelines to review new devices, procedures and drugs, including those related to behavioral/mental health. When clinical necessity requires a rapid determination of the safety and efficacy of a new technology or new application of an existing technology for an individual member, one of our Medical Directors makes a medical necessity determination based on individual member medical documentation, review of published scientific evidence, and, when appropriate, relevant specialty or professional opinion from an individual who has expertise in the technology.

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## **Section 11      Member statements**

In the absence of fraud, all statements made by you will be deemed representations and not warranties. No such representation will void coverage or reduce covered services under this Evidence of Coverage or be used in defense of a legal action unless it is contained in a written application.

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## **Section 12      Information upon request**

As a plan member, you have the right to request information on the following:

- ☐ General coverage and comparative plan information
- ☐ Utilization control procedures
- ☐ Quality improvement programs
- ☐ Statistical data on grievances and appeals
- ☐ The financial condition of UnitedHealthcare Insurance Company or one of its affiliates

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## **Section 13      2024 Enrollee Fraud & Abuse Communication**

2024 Enrollee Fraud & Abuse Communication  
**How you can fight healthcare fraud**

Our company is committed to preventing fraud, waste, and abuse in Medicare benefit programs and we're asking for your help. If you identify a potential case of fraud, please report it to us immediately.

Here are some examples of potential Medicare fraud cases:

- ☐ A health care provider - such as a physician, pharmacy, or medical device company - bills for services you never got;
- ☐ A supplier bills for equipment different from what you got;
- ☐ Someone uses another person's Medicare card to get medical care, prescriptions, supplies or equipment;
- ☐ Someone bills for home medical equipment after it has been returned;
- ☐ A company offers a Medicare drug or health plan that hasn't been approved by Medicare; or
- ☐ A company uses false information to mislead you into joining a Medicare drug or health plan.

To report a potential case of fraud in a Medicare benefit program, call UHC Dual Complete NE-V001 (HMO-POS D-SNP) Customer Service at 1-844-368-7149 (TTY 711), 8 a.m.-8 p.m.: 7 Days Oct-Mar; M-F Apr-Sept.

This hotline allows you to report cases anonymously and confidentially. We will make every effort to maintain your confidentiality. However, if law enforcement needs to get involved, we may not be able to guarantee your confidentiality. Please know that our organization will not take any action against you for reporting a potential fraud case in good faith.

You may also report potential medical or prescription drug fraud cases to the Medicare Drug Integrity Contractor (MEDIC) at 1-877-7SafeRx (1-877-772-3379) or to the Medicare program directly at (1-800-633-4227). The Medicare fax number is 1-717-975-4442 and the website is [medicare.gov](https://www.medicare.gov).

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## **Section 14      Commitment of Coverage Decisions**

UnitedHealthcare's Clinical Services Staff and Physicians make decisions on the health care services you receive based on the appropriateness of care and service and existence of coverage. Clinical Staff and Physicians making these decisions: 1. Do not specifically receive reward for issuing non-coverage (denial) decisions; 2. Do not offer incentives to physicians or other health care professionals to encourage inappropriate underutilization of care or services; and 3. Do not hire, promote, or terminate physicians or other individuals based upon the likelihood or the perceived likelihood that the individual will support or tend to support the denial of benefits.

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## **Section 15      Renew Active® Terms and Conditions**

### **Eligibility requirements**

- ☐ Only members enrolled in a participating Medicare Plan insured by UnitedHealthcare Insurance Company ("UnitedHealthcare") and affiliates are eligible for the Renew Active program ("Program"), which includes, without limitation, access to standard fitness memberships at



participating gyms/fitness locations, online fitness and cognitive providers, digital communities, in-person and virtual events, clubs, classes and discounts for meal delivery at no additional cost.

- ☐ By enrolling in the Program, you hereby accept and agree to be bound by these terms and conditions.

### **Enrollment requirements**

- ☐ Membership and participation in the Program is voluntary.
- ☐ You must enroll in the Program according to the instructions provided on this website. Once enrolled, you must obtain your confirmation code and provide it when requested to sign up for any Program services. Provide your confirmation code when requested when visiting a participating gym/fitness location to receive standard membership access at no additional cost, registering with an online fitness and/or cognitive providers, joining the Fitbit® Community for Renew Active, and to gain access to included discounts. Please note, that by using your confirmation code, you are electing to disclose that you are a Renew Active member with a participating UnitedHealthcare Medicare plan.
- ☐ Program enrollment is on an individual basis and the Program's waived monthly membership rate for standard membership services at participating gyms and fitness locations is only applicable to individual memberships.
- ☐ You are responsible for any and all non-covered services and/or similar fee-based products and services offered by Program service providers (including, without limitation, gym/fitness centers, digital fitness offerings, digital cognitive providers, Fitbit, and other third party service offerings made available through the Program), including, without limitation, fees associated with personal training sessions, specialized classes, enhanced facility membership levels beyond the basic or standard membership level, and meal delivery.

Fitness membership equipment, classes, personalized fitness plans, caregiver access and events may vary by location. Access to gym and fitness location network may vary by location and plan.

### **Liability waiver**

- ☐ Always seek the advice of a doctor prior to beginning an exercise program or making changes to your lifestyle or health care routine.
- ☐ Certain services, discounts, classes, events, and online fitness offerings are provided by affiliates of UnitedHealthcare or other third parties not affiliated with UnitedHealthcare. Participation in these third-party services are subject to your acceptance of their respective terms and policies. UnitedHealthcare and its respective subsidiaries are not responsible for the services or information provided by third parties. The information provided through these services is for informational purposes only and is not a substitute for the advice of a doctor. UnitedHealthcare and its respective subsidiaries and affiliates do not endorse and are not responsible for the services or information provided by third parties, the content on any linked site, or for any injuries you may sustain while participating in any activities under the Program.

### **Other requirements**

- ☐ You must verify that the individual gym/fitness location or service provider participates in the Program before enrolling.
- ☐ If a Program service provider you use, including a gym or fitness location, ceases to participate in the Program, your Program participation and waived monthly membership rate with such service provider through the Program will be discontinued until you join another service offered by a participating service provider. You will be responsible for paying the standard membership rates of the service provider should you elect to continue to receive services from a service provider once that service provider ceases to participate in our Program. If you wish to cancel your membership with such service provider, you can opt to do so per the cancellation policy of the applicable service provider, including the applicable gym or fitness location. You should review your termination rights with a service provider when you initially elect to sign up with such service provider.

### **Data requirements**

- ☐ Optum (the Program administrator) and/or your service provider will collect and electronically send and/or receive the minimum amount of your personal information required in order to facilitate the Program in accordance with the requirements of applicable laws, including privacy laws. Such required personal information includes, but is not limited to, program confirmation code, gym/fitness location/provider membership ID, activity year and month, and monthly visit count. By enrolling in the Program, you authorize Optum and your service provider to request and/or provide such personal information.

# **Chapter 12**

Definitions of important words

## Chapter 12

### Definitions of important words

**Ambulatory Surgical Center** – An Ambulatory Surgical Center is an entity that operates exclusively for the purpose of furnishing outpatient surgical services to patients not requiring hospitalization and whose expected stay in the center does not exceed 24 hours.

**Appeal** – An appeal is something you do if you disagree with our decision to deny a request for coverage of health care services or prescription drugs or payment for services or drugs you already received. You may also make an appeal if you disagree with our decision to stop services that you are receiving.

**Balance Billing** – When a provider (such as a doctor or hospital) bills a patient more than the plan's allowed cost-sharing amount. As a member of UHC Dual Complete NE-V001 (HMO-POS D-SNP), you only have to pay our plan's allowed cost-sharing amounts when you get services covered by our plan. We do not allow network providers to "balance bill" or otherwise charge you more than the amount of cost-sharing your plan says you must pay.

**Benefit period** – The way that Original Medicare measures your use of hospital and skilled nursing facility (SNF) services. A benefit period begins the day you go into a hospital or skilled nursing facility. The benefit period ends when you have not received any inpatient hospital care (or skilled care in a SNF) for 60 days in a row. If you go into a hospital or a skilled nursing facility after one benefit period has ended, a new benefit period begins. You must pay the inpatient hospital deductible for each benefit period. There is no limit to the number of benefit periods.

**Biological Product** – A prescription drug that is made from natural and living sources like animal cells, plant cells, bacteria, or yeast. Biological products are more complex than other drugs and cannot be copied exactly, so alternative forms are called biosimilars. Biosimilars generally work just as well, and are as safe, as the original biological products.

**Brand Name Drug** – A prescription drug that is manufactured and sold by the pharmaceutical company that originally researched and developed the drug. Brand name drugs have the same active-ingredient formula as the generic version of the drug. However, generic drugs are manufactured and sold by other drug manufacturers and are generally not available until after the patent on the brand name drug has expired.

**Catastrophic Coverage Stage** – The stage in the Part D Drug Benefit that begins when you (or other qualified parties on your behalf) have spent \$8,000 for Part D covered drugs during the covered year. During this payment stage, the plan pays the full cost for your covered Part D drugs. You pay nothing.

**Centers for Medicare & Medicaid Services (CMS)** – The Federal agency that administers Medicare.

**Chronic-Care Special Needs Plan** – C-SNPs are SNPs that restrict enrollment to MA eligible individuals who have one or more severe or disabling chronic conditions, as defined under 42 CFR 422.2, including restricting enrollment based on the multiple commonly co-morbid and clinically-linked condition groupings specified in 42 CFR 422.4(a)(1)(iv).

**Clinical Research Study** – A clinical research study is a way that doctors and scientists test new types of medical care, like how well a new cancer drug works. They test new medical care procedures or drugs by asking for volunteers to help with the study. This kind of study is one of the final stages of a research process that helps doctors and scientists see if a new approach works and if it is safe.

**Coinsurance** – An amount you may be required to pay, expressed as a percentage (for example 20%) as your share of the cost for services or prescription drugs. Coinsurance for in-network services is based upon contractually negotiated rates (when available for the specific covered service to which the coinsurance applies) or Medicare Allowable Cost, depending on our contractual arrangements for the service.

**Compendia** – Medicare-recognized reference books for drug information and medically accepted indications for Part D coverage.

**Complaint** – The formal name for “making a complaint” is “filing a grievance.” The complaint process is used only for certain types of problems. This includes problems related to quality of care, waiting times, and the customer service you receive. It also includes complaints if your plan does not follow the time periods in the appeal process.

**Comprehensive Outpatient Rehabilitation Facility (CORF)** – A facility that mainly provides rehabilitation services after an illness or injury, including physical therapy, social or psychological services, respiratory therapy, occupational therapy and speech-language pathology services, and home environment evaluation services.

**Copayment (or “copay”)** – An amount you may be required to pay as your share of the cost for a medical service or supply, like a doctor’s visit, hospital outpatient visit, or a prescription drug. A copayment is a set amount (for example \$10), rather than a percentage.

**Cost-Sharing** – Cost-sharing refers to amounts that a member has to pay when services or drugs are received. (This is in addition to the plan’s monthly premium.) Cost-sharing includes any combination of the following three types of payments: (1) any deductible amount a plan may impose before services or drugs are covered; (2) any fixed “copayment” amount that a plan requires when a specific service or drug is received; or (3) any “coinsurance” amount, a percentage of the total amount paid for a service or drug that a plan requires when a specific service or drug is received.

**Coverage Determination** – A decision about whether a drug prescribed for you is covered by the plan and the amount, if any, you are required to pay for the prescription. In general, if you bring your prescription to a pharmacy and the pharmacy tells you the prescription isn’t covered under your plan, that isn’t a coverage determination. You need to call or write to your plan to ask for a formal decision about the coverage. Coverage determinations are called “coverage decisions” in this document.

**Covered Drugs** – The term we use to mean all of the prescription drugs covered by our plan.

**Covered Services** – The term we use in this EOC to mean all of the health care services and supplies that are covered by our plan.

**Creditable Prescription Drug Coverage** – Prescription drug coverage (for example, from an employer or union) that is expected to pay, on average, at least as much as Medicare’s standard prescription drug coverage. People who have this kind of coverage when they become eligible for

Medicare can generally keep that coverage without paying a penalty, if they decide to enroll in Medicare prescription drug coverage later.

**Custodial Care** – Custodial care is personal care provided in a nursing home, hospice, or other facility setting when you do not need skilled medical care or skilled nursing care. Custodial care, provided by people who do not have professional skills or training, includes help with activities of daily living like bathing, dressing, eating, getting in or out of a bed or chair, moving around, and using the bathroom. It may also include the kind of health-related care that most people do themselves, like using eye drops. Medicare doesn't pay for custodial care.

**Customer Service** – A department within our plan responsible for answering your questions about your membership, benefits, grievances, and appeals.

**Daily cost-sharing rate** – A “daily cost-sharing rate” may apply when your doctor prescribes less than a full month's supply of certain drugs for you and you are required to pay a copayment. A daily cost-sharing rate is the copayment divided by the number of days in a month's supply. Here is an example: If your copayment for a one-month supply of a drug is \$30, and a one-month's supply in your plan is 30 days, then your “daily cost-sharing rate” is \$1 per day.

Daily Cost Share applies only if the drug is in the form of a solid oral dose (e.g., tablet or capsule) when dispensed for less than a one-month supply under applicable law. The Daily Cost Share requirements do not apply to either of the following:

1. Solid oral doses of antibiotics.
2. Solid oral doses that are dispensed in their original container or are usually dispensed in their original packaging to assist patients with compliance.

**Deductible** – The amount you must pay for health care or prescriptions before our plan pays.

**Disenroll or Disenrollment** – The process of ending your membership in our plan.

**Dispensing Fee** – A fee charged each time a covered drug is dispensed to pay for the cost of filling a prescription, such as the pharmacist's time to prepare and package the prescription.

**Dual Eligible Individual** – A person who qualifies for Medicare and Medicaid coverage.

**Dual Eligible Special Needs Plans (D-SNP)** – D-SNPs enroll individuals who are entitled to both Medicare (title XVIII of the Social Security Act) and medical assistance from a state plan under Medicaid (title XIX). States cover some Medicare costs, depending on the state and the individual's eligibility.

**Durable Medical Equipment (DME)** – Certain medical equipment that is ordered by your doctor for medical reasons. Examples include walkers, wheelchairs, crutches, powered mattress systems, diabetic supplies, IV infusion pumps, speech generating devices, oxygen equipment, nebulizers, or hospital beds ordered by a provider for use in the home.

**Emergency** – A medical emergency is when you, or any other prudent layperson with an average knowledge of health and medicine, believe that you have medical symptoms that require immediate medical attention to prevent loss of life (and, if you are a pregnant woman, loss of an unborn child), loss of a limb, or loss of function of a limb, or loss of or serious impairment to a bodily function. The medical symptoms may be an illness, injury, severe pain, or a medical condition that is quickly getting worse.

**Emergency Care** – Covered services that are: 1) provided by a provider qualified to furnish



emergency services; and 2) needed to treat, evaluate, or stabilize an emergency medical condition.

**Evidence of Coverage (EOC) and Disclosure Information** – This document, along with your enrollment form and any other attachments, riders, or other optional coverage selected, which explains your coverage, what we must do, your rights, and what you have to do as a member of our plan.

**Exception** – A type of coverage decision that, if approved, allows you to get a drug that is not on our formulary (a formulary exception), or get a non-preferred drug at a lower cost-sharing level (a tiering exception). You may also request an exception if our plan requires you to try another drug before receiving the drug you are requesting, or if our plan limits the quantity or dosage of the drug you are requesting (a formulary exception).

**Extra Help** – A Medicare or a state program to help people with limited income and resources pay Medicare prescription drug program costs, such as premiums, deductibles, and coinsurance.

**Generic Drug** – A prescription drug that is approved by the Food and Drug Administration (FDA) as having the same active ingredient(s) as the brand name drug. Generally, a “generic” drug works the same as a brand name drug and usually costs less.

**Grievance** – A type of complaint you make about our plan, providers, or pharmacies, including a complaint concerning the quality of your care. This does not involve coverage or payment disputes.

**Home Health Aide** – A person who provides services that do not need the skills of a licensed nurse or therapist, such as help with personal care (e.g., bathing, using the toilet, dressing, or carrying out the prescribed exercises).

**Home Health Care** – Skilled nursing care and certain other health care services that you get in your home for the treatment of an illness or injury. Covered services are listed in the Benefits Chart in Chapter 4, Section 2.1 under the heading “Home health agency care.” If you need home health care services, our plan will cover these services for you provided the Medicare coverage requirements are met. Home health care can include services from a **home health aide** if the services are part of the home health plan of care for your illness or injury. They aren’t covered unless you are also getting a covered skilled service. Home health services don’t include the services of housekeepers, food service arrangements, or full-time nursing care at home.

**Hospice** – A benefit that provides special treatment for a member who has been medically certified as terminally ill, meaning having a life expectancy of 6 months or less. We, your plan, must provide you with a list of hospices in your geographic area. If you elect hospice and continue to pay premiums you are still a member of our plan. You can still obtain all medically necessary services as well as the supplemental benefits we offer.

**Hospice Care** – A special way of caring for people who are terminally ill and providing counseling for their families. Hospice care is physical care and counseling that is given by a team of people who are part of a Medicare-certified public agency or private company. Depending on the situation, this care may be given in the home, a hospice facility, a hospital, or a nursing home. Care from a hospice is meant to help patients in the last months of life by giving comfort and relief from pain. The focus is on care, not cure. For more information on hospice care visit [medicare.gov](https://www.medicare.gov) and under “Search Tools” choose “Find a Medicare Publication” to view or download the publication “Medicare Hospice Benefits.” Or, call (1-800-633-4227). TTY users should call 1-877-486-2048. You may call 24 hours a day/7 days a week.



**Hospital Inpatient Stay** – A hospital stay when you have been formally admitted to the hospital for skilled medical services. Even if you stay in the hospital overnight, you might still be considered an “outpatient.”

**Income Related Monthly Adjustment Amount (IRMAA)** – If your modified adjusted gross income as reported on your IRS tax return from 2 years ago is above a certain amount, you’ll pay the standard premium amount and an Income Related Monthly Adjustment Amount, also known as IRMAA. IRMAA is an extra charge added to your premium. Less than 5% of people with Medicare are affected, so most people will not pay a higher premium.

**Initial Coverage Limit** – The maximum limit of coverage under the Initial Coverage Stage.

**Initial Coverage Stage** – This is the stage before your total drug costs including amounts you have paid and what your plan has paid on your behalf for the year have reached \$5,030.

**Initial Enrollment Period** – When you are first eligible for Medicare, the period of time when you can sign up for Medicare Part A and Part B. If you’re eligible for Medicare when you turn 65, your Initial Enrollment Period is the 7-month period that begins 3 months before the month you turn 65, includes the month you turn 65, and ends 3 months after the month you turn 65.

**Institutional Special Needs Plan (SNP)** – A plan that enrolls eligible individuals who continuously reside or are expected to continuously reside for 90 days or longer in a long-term care (LTC) facility. These LTC facilities may include a skilled nursing facility (SNF); nursing facility (NF); (SNF/NF); an Intermediate Care Facility for Individuals with Intellectual Disabilities (ICF/IID); an inpatient psychiatric facility, and/or facilities approved by CMS that furnishes similar long-term, healthcare services that are covered under Medicare Part A, Medicare Part B, or Medicaid; and whose residents have similar needs and healthcare status to the other named facility types. An institutional Special Needs Plan must have a contractual arrangement with (or own and operate) the specific LTC facility(ies).

**Institutional Equivalent Special Needs Plan (SNP)** – A plan that enrolls eligible individuals living in the community but requiring an institutional level of care based on the State assessment. The assessment must be performed using the same respective State level of care assessment tool and administered by an entity other than the organization offering the plan. This type of Special Needs Plan may restrict enrollment to individuals that reside in a contracted assisted living facility (ALF) if necessary to ensure uniform delivery of specialized care.

**List of Covered Drugs (Formulary or “Drug List”)** – A list of prescription drugs covered by the plan.

**Low Income Subsidy (LIS)** – See “Extra Help.”

**Maximum Out-of-Pocket Amount** – The most that you pay out-of-pocket during the calendar year for in-network covered Part A and Part B services. Amounts you pay for your plan premiums, Medicare Part A and Part B premiums, and prescription drugs do not count toward the maximum out-of-pocket amount. If you are eligible for Medicare cost-sharing assistance under Medicaid, you are not responsible for paying any out-of-pocket costs toward the maximum out-of-pocket amount for covered Part A and Part B services. (Note: Because our members also get assistance from Medicaid, very few members ever reach this out-of-pocket maximum.) See Chapter 4, Section 1.2 for information about your maximum out-of-pocket amount

**Medicaid (or Medical Assistance)** – A joint Federal and State program that helps with medical

costs for some people with low incomes and limited resources. State Medicaid programs vary, but most health care costs are covered if you qualify for both Medicare and Medicaid.

**Medical Emergency** – A medical emergency is when you, or any other prudent layperson with an average knowledge of health and medicine, believe that you have medical symptoms that require immediate medical attention to prevent loss of life, loss of a limb, or loss of function of a limb. The medical symptoms may be an illness, injury, severe pain, or a medical condition that is quickly getting worse.

**Medically Accepted Indication** – A use of a drug that is either approved by the Food and Drug Administration or supported by certain reference books.

**Medically Necessary** – Services, supplies, or drugs that are needed for the prevention, diagnosis, or treatment of your medical condition and meet accepted standards of medical practice.

**Medicare** – The Federal health insurance program for people 65 years of age or older, some people under age 65 with certain disabilities, and people with End-Stage Renal Disease (generally those with permanent kidney failure who need dialysis or a kidney transplant).

**Medicare Advantage Open Enrollment Period** – The time period from January 1 until March 31 when members in a Medicare Advantage plan can cancel their plan enrollment and switch to another Medicare Advantage plan, or obtain coverage through Original Medicare. If you choose to switch to Original Medicare during this period, you can also join a separate Medicare prescription drug plan at that time. The Medicare Advantage Open Enrollment Period is also available for a 3-month period after an individual is first eligible for Medicare.

**Medicare Advantage (MA) Plan** – Sometimes called Medicare Part C. A plan offered by a private company that contracts with Medicare to provide you with all your Medicare Part A and Part B benefits. A Medicare Advantage Plan can be an i) HMO, ii) PPO, a iii) Private Fee-for-Service (PFFS) plan, or a iv) Medicare Medical Savings Account (MSA) plan. Besides choosing from these types of plans, a Medicare Advantage HMO or PPO plan can also be a Special Needs Plan (SNP). In most cases, Medicare Advantage Plans also offer Medicare Part D (prescription drug coverage). These plans are called **Medicare Advantage Plans with Prescription Drug Coverage**.

**Medicare Allowable Cost** – The maximum price of a service for reimbursement purposes under Original Medicare.

**Medicare Assignment** – In Original Medicare, a doctor or supplier "accepts assignment" when he or she agrees to accept the Medicare-approved amount as full payment for covered services.

**Medicare Coverage Gap Discount Program** – A program that provides discounts on most covered Part D brand name drugs to Part D members who have reached the Coverage Gap Stage and who are not already receiving "Extra Help." Discounts are based on agreements between the Federal government and certain drug manufacturers.

**Medicare-Covered Services** – Services covered by Medicare Part A and Part B. All Medicare health plans must cover all of the services that are covered by Medicare Part A and B. The term Medicare-Covered Services does not include the extra benefits, such as vision, dental or hearing, that a Medicare Advantage plan may offer.

**Medicare Health Plan** – A Medicare health plan is offered by a private company that contracts with Medicare to provide Part A and Part B benefits to people with Medicare who enroll in the plan. This term includes all Medicare Advantage Plans, Medicare Cost Plans, Special Needs Plans,

Demonstration/Pilot Programs, and Programs of All-inclusive Care for the Elderly (PACE).

**Medicare Prescription Drug Coverage (Medicare Part D)** – Insurance to help pay for outpatient prescription drugs, vaccines, biologicals, and some supplies not covered by Medicare Part A or Part B.

**“Medigap” (Medicare Supplement Insurance) Policy** – Medicare supplement insurance sold by private insurance companies to fill “gaps” in Original Medicare. Medigap policies only work with Original Medicare. (A Medicare Advantage Plan is not a Medigap policy.)

**Member (Member of our plan, or “Plan Member”)** – A person with Medicare who is eligible to get covered services, who has enrolled in our plan, and whose enrollment has been confirmed by the Centers for Medicare & Medicaid Services (CMS).

**Network** – The doctors and other health care professionals, medical groups, hospitals, and other health care facilities or providers that have an agreement with us to provide covered services to our members and to accept our payment and any plan cost-sharing as payment in full. (See Chapter 1, Section 3.2)

**Network Pharmacy** – A pharmacy that contracts with our plan where members of our plan can get their prescription drug benefits. In most cases, your prescriptions are covered only if they are filled at one of our network pharmacies.

**Network Provider** – “Provider” is the general term for doctors, other health care professionals, hospitals, and other health care facilities that are licensed or certified by Medicare and by the State to provide health care services. “Network providers” have an agreement with our plan to accept our payment as payment in full, and in some cases to coordinate as well as provide covered services to members of our plan. Network providers are also called “plan providers.”

**Organization Determination** – A decision our plan makes about whether items or services are covered or how much you have to pay for covered items or services. Organization determinations are called “coverage decisions” in this document.

**Original Medicare** (“Traditional Medicare” or “Fee-for-service” Medicare) – Original Medicare is offered by the government, and not a private health plan like Medicare Advantage Plans and prescription drug plans. Under Original Medicare, Medicare services are covered by paying doctors, hospitals, and other health care providers payment amounts established by Congress. You can see any doctor, hospital, or other health care provider that accepts Medicare. You must pay the deductible. Medicare pays its share of the Medicare-approved amount, and you pay your share. Original Medicare has two parts: Part A (Hospital Insurance) and Part B (Medical Insurance) and is available everywhere in the United States.

**Out-of-Network Pharmacy** – A pharmacy that does not have a contract with our plan to coordinate or provide covered drugs to members of our plan. Most drugs you get from out-of-network pharmacies are not covered by our plan unless certain conditions apply.

**Out-of-Network Provider or Out-of-Network Facility** – A provider or facility that does not have a contract with our plan to coordinate or provide covered services to members of our plan. Out-of-network providers are providers that are not employed, owned, or operated by our plan.

**Out-of-Pocket Costs** – See the definition for “cost-sharing” above. A member’s cost-sharing requirement to pay for a portion of services or drugs received is also referred to as the member’s “out-of-pocket” cost requirement.

**PACE plan** – A PACE (Program of All-Inclusive Care for the Elderly) plan combines medical, social, and long-term care services for frail people to help people stay independent and living in their community (instead of moving to a nursing home) as long as possible, while getting the high quality care they need. People enrolled in PACE plans receive both their Medicare and Medicaid benefits through the plan. PACE is not available in all states. If you would like to know if PACE is available in your state, please contact Customer Service.

**Part C** – see “**Medicare Advantage (MA) Plan.**”

**Part D** – The voluntary Medicare Prescription Drug Benefit Program.

**Part D Drugs** – Drugs that can be covered under Part D. We may or may not offer all Part D drugs. Certain categories of drugs have been excluded as covered Part D drugs by Congress. Certain categories of Part D drugs must be covered by every plan.

**Part D Late Enrollment Penalty** – An amount added to your monthly premium for Medicare drug coverage if you go without creditable coverage (coverage that is expected to pay, on average, at least as much as standard Medicare prescription drug coverage) for a continuous period of 63 days or more after you are first eligible to join a Part D plan. If you lose Extra Help, you may be subject to the late enrollment penalty if you go 63 days or more in a row without Part D or other creditable prescription drug coverage.

**Point of Service (POS) Plan** – As a member of this Point of Service (POS) plan you may receive covered services from network providers. You may also receive covered routine dental services from providers who are not contracted with UnitedHealthcare.

**Premium** – The periodic payment to Medicare, an insurance company, or a health care plan for health or prescription drug coverage.

**Prescription Drug Benefit Manager** – Third party prescription drug organization responsible for processing and paying prescription drug claims, developing and maintaining the drug list (formulary), and negotiating discounts and rebates with drug manufacturers.

**Primary Care Provider (PCP)** – The doctor or other provider you see first for most health problems. In many Medicare health plans, you must see your primary care provider before you see any other health care provider.

**Prior Authorization** – For medical services it means a process where your PCP or treating provider must receive approval in advance before certain medical services will be provided or payable. For certain drugs it means a process where you or your provider must receive approval in advance before certain drugs will be provided or payable. Covered services that need prior authorization are marked in the Medical Benefits Chart in Chapter 4. Covered drugs that need prior authorization are marked in the formulary.

**Prosthetics and Orthotics** – Medical devices including, but are not limited to, arm, back and neck braces; artificial limbs; artificial eyes; and devices needed to replace an internal body part or function, including ostomy supplies and enteral and parenteral nutrition therapy.

**Provider** – The doctor or other provider you see first for most health problems. In many Medicare health plans, you must see your primary care provider before you see any other health care provider.

**Quality Improvement Organization (QIO)** – A group of practicing doctors and other health care

experts paid by the Federal government to check and improve the care given to Medicare patients.

**Quantity Limits** – A management tool that is designed to limit the use of selected drugs for quality, safety, or utilization reasons. Limits may be on the amount of the drug that we cover per prescription or for a defined period of time.

**Real-Time Benefit Tool** – A portal or computer application in which enrollees can look up complete, accurate, timely, clinically appropriate, enrollee-specific formulary and benefit information. This includes cost sharing amounts, alternative formulary medications that may be used for the same health condition as a given drug, and coverage restrictions (Prior Authorization, Step Therapy, Quantity Limits) that apply to alternative medications.

**Rehabilitation Services** – These services include physical therapy, speech and language therapy, and occupational therapy.

**Retail Walk-In Clinic** – A provider location that generally does not require appointments and may be a standalone location or located in a retail store, supermarket or pharmacy. Walk-In Clinic Services are subject to the same cost-sharing as Urgent Care Centers. (See the Benefit Chart in Chapter 4)

**Service Area** – A geographic area where you must live to join a particular health plan. For plans that limit which doctors and hospitals you may use, it's also generally the area where you can get routine (non-emergency) services. The plan may disenroll you if you permanently move out of the plan's service area.

**Skilled Nursing Facility (SNF) Care** – Skilled nursing care and rehabilitation services provided on a continuous, daily basis, in a skilled nursing facility. Examples of care include physical therapy or intravenous injections that can only be given by a registered nurse or doctor.

**Special Needs Plan** – A special type of Medicare Advantage Plan that provides more focused health care for specific groups of people, such as those who have both Medicare and Medicaid, who reside in a nursing home, or who have certain chronic medical conditions.

**Step Therapy** – A utilization tool that requires you to first try another drug to treat your medical condition before we will cover the drug your physician may have initially prescribed.

**Supplemental Security Income (SSI)** – A monthly benefit paid by Social Security to people with limited income and resources who are disabled, blind, or age 65 and older. SSI benefits are not the same as Social Security benefits.

**Urgently Needed Services** – Covered services that are not emergency services, provided when the network providers are temporarily unavailable or inaccessible or when the enrollee is out of the service area. For example, you need immediate care during the weekend. Services must be immediately needed and medically necessary.



# UHC Dual Complete NE-V001 (HMO-POS D-SNP) Customer Service:



Call **1-844-368-7149**

Calls to this number are free. 8 a.m.-8 p.m.: 7 Days Oct-Mar; M-F Apr-Sept. Customer Service also has free language interpreter services available for non-English speakers.

**TTY 711**

Calls to this number are free. 8 a.m.-8 p.m.: 7 Days Oct-Mar; M-F Apr-Sept.



Write: **P.O. Box 30770**  
**Salt Lake City, UT 84130-0770**



**[myuhc.com/communityplan](http://myuhc.com/communityplan)**

## State Health Insurance Assistance Program

State Health Insurance Assistance Program is a state program that gets money from the Federal government to give free local health insurance counseling to people with Medicare.

You can call the SHIP in your state at the number listed in Chapter 2 Section 3 of the Evidence of Coverage.

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